Hard Calls, Clear Notes

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The NP Casebook of Defensible Documentation

Dana Ellery

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Laws, regulations, employer policies and standards of care differ from state to state and from one organization to the next, and they change over time. Nothing in this book states what the law or the standard of care requires where you practice. For your own situation, ask your organization's risk manager, your professional liability insurer, your state board of nursing and, where needed, an attorney of your own choosing.

No documentation approach prevents a malpractice claim, a licensing board complaint, an employer review or any other sanction, or guarantees how one will end. In this book, “defensible” describes a note that makes the clinician's reasoning visible; it does not describe a legal result.

Every patient, clinician, organization, event and note in this book is constructed for teaching. None is a real case or a real court decision, and any resemblance to real people or organizations is coincidental. The sample notes are models of wording, not templates to copy; their doses, thresholds and clinical choices belong to constructed patients and are not recommendations. Clinical decision rules and guidelines are summarized and may have been updated. Check drugs, doses and clinical decisions against the original sources, current labeling and your local protocols.

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Contents

Introduction 1

Part I — Writing for the Later Reader 5

1 What a Chart Becomes After a Bad Outcome 7

2 The Provider Note Records a Decision 13

3 Where a Claim, a Complaint, and a Review Begin 19

4 Writing the Differential You Actually Ran 25

5 The Working Diagnosis and What Would Change It 31

6 Facts, Quotes, and What Never Goes in the Chart 37

7 Templates, Copy-Forward, and the Late Addendum 43

Part II — Orders, Waits, and Pushback 49

8 The Test You Order and the Test You Don't 51

9 Closing the Loop on the Abnormal Result 57

10 The Delayed Scan and the Bed That Never Opens 63

11 The Consultant Who Never Comes 69

12 Why This Drug, This Dose, This Patient 75

13 When You Say No to an Antibiotic or a Scan 81

14 Hostile Encounters in Neutral Words 87

Part III — Choices, Continuity, and Aftermath 93

15 The Capacity Question, Decision by Decision 95

16 Informed Refusal and the Partial Yes 101

17 The Patient Leaving Against Medical Advice 107

18 The Discharge and the Patient Who Comes Back 113

19 Handoffs, Cross-Cover, and the Night Call 119

20 The Telehealth Visit and the Portal Message 125

21 Missed Visits and Referrals That Go Nowhere 131

22 The Adverse Event and the Records Request 137

Part IV — The Mid-Shift Reference 143

23 Phrasebook of Weak and Defensible Wording 145

24 Scenario Finder and What Each Note Must Contain 153

25 Glossary of Legal Terms in Plain English 165

About the Author 171

One request 173

Introduction

The note at the end of the shift

Leah Okafor, a former medical-surgical nurse, is four months into her first provider job, at an urgent care clinic. Late on a Thursday she sees Marcus, forty-one, with a headache that has built slowly over two days. Neither of them exists; every case in this book is constructed.

She asks how it started, because she knows what a sudden, worst-ever headache can mean. She checks his neck, his temperature, his pupils, his strength and his gait. She considers bleeding and infection, finds nothing that points to either, and settles on a tension headache. Before he leaves she tells him exactly what should bring him back tonight: a sudden change, vomiting, confusion, fever, a stiff neck.

She types her note before calling the next patient:

> HA x 2 days. Neuro intact. Likely tension HA. Ibuprofen. Return PRN.

The questions she asked about onset are not there, the dangerous diagnoses she thought of and set aside are not there, and the warnings she gave Marcus have shrunk to "Return PRN." Nothing in the note shows that his neck was supple and his temperature normal, the two findings that argued against infection. If his headache turns out to be something else, the person who reads that note months later will see a clinician who wrote down a conclusion. They will have no way of seeing the clinician who reached it.

The problem nobody taught you

That gap between what was done and what was written is the problem this book is about. It is not a problem of competence. Leah's thinking was sound; her note simply did not carry it. Nursing school and years at the bedside trained her to chart what she observed, and she does that well. Nobody trained her to write down a judgment, the alternatives she weighed and the reasons she chose one, because until four months ago that was not her job.

Now it is. If you have just moved from the RN role into the provider role, you now sign H&Ps, progress notes, urgent-care and emergency visits and discharge notes, and each records a decision that is yours. Final-year NP students on rotation are learning to write the same notes, usually under a preceptor's review. Much of what has been written about defensible charting is built around the nursing note: flowsheets, narrative entries, handoff formats. That material is useful, but it does not reach the moments where your note is most exposed: the night page about a patient you have never met, the result that arrives after the patient has gone home, the colleague who disagrees with your plan.

What this book gives you

This book is built from those moments. For each hard call an NP makes—the test you don't order, the abnormal result, the consultant who never comes, the capacity question, the departure against medical advice, the discharge, the telehealth visit, the prescription—you will see three things. First, the questions the record will be asked later. Second, a weak note set beside a defensible rewrite of the same encounter, so the difference is visible line by line. Third, the short list of elements that note has to contain.

The defensible version is never more defensive, and the reasoning it adds is short. Leah's note needed perhaps three more sentences. With them, it reads:

> HA x 2 days, gradual onset; denies sudden or worst-ever HA. Neuro intact. Considered bleed (onset not sudden) and CNS infection (afebrile, neck supple). Likely tension HA. Ibuprofen. Told to return tonight for sudden change, vomiting, confusion, fever or stiff neck.

That is two minutes of writing, and it is the kind of writing this book teaches: in your own words, in the time a shift allows. Each scenario chapter shows the whole note, so its rewrite runs longer, mostly with the history and exam a provider note has anyway. For a full board, the chapter on the differential shows a short version. The goal is a note that shows what you saw, what you considered and why you decided.

That goal has a limit. No note makes anyone immune to a lawsuit, a board complaint or a sanction, and this book does not pretend otherwise. What a good note does is make your reasoning visible on the record, so that a later reader judges the decision you actually made rather than the one a sparse note leaves them to guess at.

The book also leaves some things out on purpose. It is not legal advice and it is not clinical advice: it does not tell you how to diagnose or treat, and the doses, thresholds and decisions in its scenarios belong to constructed patients. It gives no state-by-state rules on scope of practice, collaboration or prescribing; where those rules matter, it tells you what to ask your risk manager, your malpractice insurer and your state board. It does not cover coding, billing or any particular EHR. There are no forms or templates to fill in, and no real court cases: every scenario is constructed and labeled as such.

How the book is built

The book has four parts and moves from principle to practice; each principle is explained once. Part I, *Writing for the Later Reader*, sets out how a provider note is read after something goes wrong, why that reading differs from the way your nursing notes were read, and where claims against NPs tend to begin. It then turns that understanding into a way of writing: the differential you actually ran, uncertainty stated honestly, objective language, and the EHR itself, which fills a note with text you did not type and keeps a trace of every change you make.

The scenarios follow, roughly in the order a patient moves through your care. Part II, *Orders, Waits, and Pushback*, covers the workup and its delays, the prescription and the request you decline, and the visit that turns hostile. Part III, *Choices, Continuity, and Aftermath*, covers the patient's right to choose and to leave, the discharge and the handoff, the encounters away from the bedside, the

patient who drops out of care, and what happens after an adverse event. Each scenario chapter is built the same way and points back to the principles it rests on, so once you know the shape of one you can find your way through any of them mid-shift.

Part IV, *The Mid-Shift Reference*, is for the moment of writing. A phrasebook sets weak wording beside defensible wording, scenario by scenario. A scenario finder collects the required elements of each note, indexed by what is happening right now and by setting: hospital medicine, emergency and urgent care, primary care. A short glossary defines the legal terms in plain English; most are also defined where they first appear.

You do not have to read the scenarios front to back. Read Part I before your next shift if you can, then keep the book wherever you chart. The next time you do what Leah did, from the question about onset to the warning at the door, your note should show it.

PART I

Writing for the Later Reader

CHAPTER 1

What a Chart Becomes After a Bad Outcome

The record, laid end to end

When you write a note, it feels like a single document about a single moment. After a bad outcome it stops being that. Someone exports the entire record for the episode of care, including every note, order, vital sign, medication given, result and message, and sorts it by time. Your note becomes one entry on a line that runs from the patient's arrival to the event that brought the reviewer to the chart. The reviewer studies what each entry says, and also the spaces between the entries.

Here is a constructed example. Nadia Brooks is a hospital medicine NP on a general medical floor. On a weekday afternoon she admits Harold Jensen, seventy-four, with community-acquired pneumonia. Cut down to its key entries, the record later put together for review reads like this:

- 14:20. Admission H&P signed by N. Brooks, NP: pneumonia, oxygen at 2 L by nasal cannula, antibiotics started.
- 19:05. Nursing note: respiratory rate 26, oxygen saturation 89% on 2 L. NP paged.
- 19:12. Order by N. Brooks, NP: increase oxygen to 4 L by nasal cannula.
- 23:50. Nursing note: patient confused, saturation 84% on 4 L. Rapid response called.

- 00:20. Rapid response note: patient transferred to intensive care.

What Nadia actually did between 19:12 and 23:50 is not on that line. She went to the bedside at 19:15 and found Mr. Jensen alert, speaking in full sentences and not using his accessory muscles. By 19:25, ten minutes after she arrived, his saturation was 94% and his respiratory rate 22. She asked the nurse to call her if he needed more than 4 liters, if his breathing rate passed 28, or if his thinking changed. At 21:00 she phoned the nurse and heard that the numbers were holding. Then two new admissions arrived at once, and she never wrote any of it down.

The reviewer sees a page, an order that took one click, and then more than four and a half hours with nothing from the provider before a rapid response. An order proves that someone touched a keyboard. It does not prove that anyone looked at the patient, and nothing on the timeline says that anyone did. The provider's side of the evening exists only in Nadia's memory. Months or years later, that memory will be tested against a record that was made at the time. When your entry is missing, other people's entries tell your part of the story, and they tell it from where they stood.

The timeline shows more than gaps. It also shows the order of events. Did a result post before or after the decision that should have depended on it? Did a reassessment come before the deterioration, or only after it? Does the plan written in a note match the orders placed beside it? A note that is accurate on its own can still look wrong once it sits among the entries around it.

Who opens the chart

Several people may read that timeline, at different times and for different reasons, and each one comes to it with a question of their own.

The first is usually the **risk manager**. This is the person in your organization whose job includes finding out what happened after an adverse event, meaning unintended harm to a patient caused by the care rather than by the illness itself, and how exposed the organization is. Risk managers often read the chart within days, sometimes before anyone has mentioned a lawyer. They look for three things: whether the record is complete, whether it supports the care that was

actually given, and whether it contains anything that will be hard to explain later.

If a claim is filed, two **attorneys** read the same pages from opposite sides. The **plaintiff** is the patient or family bringing the claim. Their attorney looks for the gap, the contradiction and the decision with no visible reason, because each one is a place to build a story of warning signs that were ignored. The defense attorney, who may represent you or your employer, reads for the same things with the opposite hope: material that shows what you saw and why you acted as you did. Your testimony can add to the record. But an account given long afterward from memory carries less weight than a line written that evening, and both sides know it.

On the clinical questions, the opinion that carries the most weight belongs to the **expert witness**. This is a clinician hired by one side to explain what the standard of care required and whether your care met it. Each state sets its own rules on who may serve as an expert in a case against an NP, and it is not always another NP. Whoever it is, the expert reads the chart the way a clinician would, in order, asking at each timestamp what a competent provider would have known and done at that point.

A complaint to your state licensing board brings a different reader, the **board investigator**. The investigator asks whether your practice met the standards your license requires and stayed within your scope of practice. The record itself, and not only the care it describes, can be part of what the board examines.

The question every reader asks

Behind all these readings sits one legal question, and it helps to have it in plain words.

In most cases, a malpractice claim is a claim of **negligence**. In broad terms, it says that a clinician who owed the patient care failed to meet the standard of care, and that this failure harmed the patient. The **standard of care** is not perfection, and it is not the best care imaginable. It is, in the usual formulation, what a reasonably prudent NP, with similar training and in similar circumstances, would have done with the same information at that moment. It is rarely a single right answer. Often several choices would have been reasonable,

and the question is whether yours was one of them. The exact wording varies from state to state, and in some states a court may measure an NP against the physician standard for care that physicians also provide. In every version, the last words are the part your note controls: the same information, at that moment.

Causation is the link between the failure and the harm. Even if an expert decides that something should have been done differently, the claim also has to show that doing it would, more likely than not, have changed what happened to the patient. Some states also allow a claim for a lost chance of a better outcome. In Mr. Jensen's case, the experts on each side would argue over whether a different decision at 19:15 would have changed what happened at 23:50. Both sides build that argument from the timeline, so they build it from what the chart says was known at 19:15.

The words "similar circumstances" matter too. An urgent care clinic at ten at night with no CT scanner is not an emergency department with imaging down the hall. An evening with two admissions arriving at once is not a quiet one. The standard takes the setting into account, but only the setting the reader can see. If the record does not show the circumstances, the reader has to guess at them.

Readers who already know the ending

Every reader in this chapter comes to the chart after the outcome. They know Mr. Jensen went to intensive care before they reach the 19:05 entry, and that knowledge changes how the entry looks. **Hindsight bias** is the name for a familiar tendency: once you know how something ended, the ending seems more predictable than it was, and the early signs seem louder than they sounded at the time.

Read backward from the transfer, a saturation of 89% looks like the first step toward the ICU. Read forward, at 19:05 on a busy evening, it was one number among many, and it improved when the oxygen went up. The standard of care asks the reader to judge the decision forward, using only the information available at the time. Experienced experts know this and try to do it. They are still reading a story whose last page they have already seen.

The only part of the record written before anyone knew the ending is the note. At 19:25, in about two minutes, Nadia could have written something like this:

> Paged by RN for SpO2 89% on 2 L, RR 26. At bedside 19:15: alert, oriented, speaking full sentences, no accessory muscle use. O2 increased to 4 L NC; at 19:25 SpO2 94%, RR 22. Plan: continuous pulse oximetry. RN to call if O2 need exceeds 4 L, RR above 28, or any change in mental status. Will check back by 21:00.

At 21:00, one more line would have recorded what the nurse reported. Neither entry changes what happened at 23:50. What they change is the reader's question. The timeline would no longer show hours of silence. It would show a patient who was seen, a response to treatment, limits set in advance and a check in between. The reader would then have to ask whether that plan was reasonable for the patient described at 19:25. That is a question about a decision, and a decision can be judged on its merits. A gap raises a question about absence, and absence is far harder to defend.

The note also records what was not yet known. At 19:25 Nadia could not know whether Mr. Jensen would keep improving or start to tire. Her record would show a decision based on the patient in front of her, and a plan to find out what happened next.

What a note protects, and in what order

In this book's order of priorities, the patient comes first. The night covering provider and the rapid response team would have read Nadia's 19:25 entry long before any attorney saw it. It would have told them how Mr. Jensen had looked, what had been tried and what limits were set. A note written for the next clinician serves the patient best, and usually holds up best with every other reader.

Only after that does the note serve your license and your career, and it does so in one way: by making your reasoning visible. It cannot make a wrong decision right, or keep anyone from filing a claim or a complaint. A note written mainly to protect its author tends to read that way, crowded with boilerplate and self-justification, and the readers in this chapter spot it quickly. What a careful note offers is narrower, and more reliable. It lets people who arrive already knowing the ending see the moment you were in: what you knew, what you did not know yet, and why you chose what you chose.

CHAPTER 2

The Provider Note Records a Decision

One patient, two notes

Dolores Fitch, seventy-nine, arrives at a community emergency department on a Tuesday afternoon with her daughter, after a fall at home. She takes apixaban, a blood thinner, for atrial fibrillation. She and everyone who treats her are invented for this chapter. Owen Castillo, the nurse who receives her, writes this at 15:05:

> 79-year-old female arrived with daughter after fall at home approx. 12:30. States she caught her foot on the edge of a rug and fell backward, striking the back of her head on a wood floor. Denies loss of consciousness. 3 cm raised hematoma to occiput, skin intact. Alert and oriented x4, pupils equal and reactive. Takes apixaban, last dose 08:00 per patient. BP 148/86, HR 78 irregular, RR 16, SpO2 97% on room air. Pain 3/10. NP Raman notified 14:57.

It is a good nursing note. Priya Raman, the first-year NP who will see Mrs. Fitch, wrote hundreds like it in seven years as an emergency nurse. It records what Owen saw and heard, when he saw it and whom he told. It does not say whether the fall was a trip or a faint, whether Mrs. Fitch needs a scan, or whether she can go home. Those questions are not his to answer, and no reviewer will look for the answers there. What a reviewer asks of his note is whether he saw what was there, wrote it down accurately and told the right person in time. On all three counts the note holds up.

Priya sees Mrs. Fitch at 15:10 and orders a CT scan of the head, which is read at 16:35 as showing no bleeding. She talks with the attending physician and discharges Mrs. Fitch home with her daughter at 17:50. Then she writes:

> 79F s/p fall, head strike, on Eliquis. No LOC. Neuro nonfocal. CT head neg. Dx: fall, scalp contusion. D/C home with daughter. F/u PCP. Return precautions given.

Beside Owen's note, Priya's reads like more of the same: findings, a result, an outcome. That is the problem. A reviewer opens a provider note with a different question. The question is not what the clinician saw but what the clinician concluded, and why. Was this a trip, or a faint that ended on the floor? Why a CT? With a blood thinner on board, why home rather than a night of observation? What were the patient and her daughter told to watch for? Priya answered every one of those questions at the bedside. Her note answers none of them, so a later reader sees the result of a judgment but not the judgment itself.

Written to answer those questions, the same note reads:

> 79F on apixaban for AF (last dose 08:00), fell at home ~12:30. Per patient, a trip: caught foot on rug edge, fell backward, struck occiput. No LOC, recalls entire event. No lightheadedness, palpitations or chest pain beforehand. Exam: GCS 15, 3 cm occipital hematoma, no midline neck tenderness, pupils equal and reactive, no focal deficit, gait steady with cane. ECG: AF rate 78, unchanged from prior. CT head (read 16:35): no acute hemorrhage or fracture.
>
> Assessment: mechanical fall, scalp contusion. CT obtained because anticoagulation raises the risk of intracranial bleeding even after a minor head strike. Nothing in history, exam or ECG suggests syncope.
>
> Plan: discussed with patient and daughter the small risk of delayed bleeding on apixaban; offered overnight observation or home with someone present. Both prefer home; daughter staying tonight. Told to return at once for worsening headache, vomiting, confusion, unusual sleepiness or new weakness. Apixaban unchanged; PCP visit within 1 week to review falls and anticoagulation. Reviewed in person with Dr. Malik Grant, ED attending, at 17:30; he did not examine the patient and agreed with discharge.

The rewrite does not describe a different afternoon. It describes the same one, and it keeps the reasoning that connects the facts: why the fall was read as a trip, why the scan was ordered, what choice was offered and who made it. Most of the added length is history and exam that belong in any provider note. The reasoning itself takes about four sentences. There is one more difference. Owen charted an irregular heart rate, and the rewrite accounts for it with the ECG and the known atrial fibrillation. Once a nurse records an observation, the provider is expected to have weighed it. If the provider note never mentions it, it can look like something the provider missed.

The parts of a provider note

Whatever headings your template uses, a provider note is built from the same five parts. The SOAP format you may know from nursing school folds exam and data into "objective," and some organizations put the assessment and plan first.

The history is the patient's account as you drew it out: why they came, what happened, their conditions, medications, allergies and circumstances. Your questions shape it, which makes it more revealing than it looks. Priya asked about lightheadedness before the fall, and that question shows she was thinking about a faint before her assessment says so.

The exam is what you found yourself. It includes the findings you looked for and did not find, the *pertinent negatives*, such as "no midline neck tenderness." The data section holds what came from elsewhere: labs, imaging, the ECG, old records, and the nursing observations you relied on. The assessment says what you think is going on, how sure you are, and what you considered and set aside. The plan says what you will do, what the patient agreed to, what happens next and who is responsible for it.

After a bad outcome, the assessment and plan carry the most weight. The standard-of-care question from the previous chapter asks what a reasonable NP would have done with the same information at that moment. History, exam and data are the information. The assessment and plan are the decision, and they are the only place that shows how one led to the other. They are also the only part nobody else can write. The nurse charts the same vital signs, the radiologist writes the CT report, and a template can fill in a normal exam. Only you can say

why all of it pointed toward home rather than admission. A note can have a flawless history and exam and still leave its reader guessing about the one thing they opened it to find.

Four notes, four questions

Most of what a new NP signs falls into four kinds of notes. Each answers a different question on the record, and once you know the question, you know what the assessment and plan have to carry.

The history and physical, or H&P, is the admission note. It answers two questions: what did the admitting provider believe was wrong when the patient arrived, and what did they set in motion? Every later note in the stay is read against it, so its working diagnosis and first plan become the baseline.

The progress note answers whether anything has changed since the last note and whether the plan still fits. A reviewer reading a run of progress notes looks for movement: a patient getting better or worse, and a plan that changes with them. A patient's creatinine climbs for three days while the assessment stays word for word the same. On the record, that reads as three days in which nobody noticed.

The emergency or urgent-care visit note answers one question: why this **disposition**? Disposition is where the patient goes at the end of the visit: home, admission, transfer or another setting. You may never see this patient again, so the note has to stand on its own and show why the destination was safe enough. In Priya's rewrite, the plan is that answer.

The discharge note, or discharge summary, answers what happened during the stay, what is still unresolved, and who is responsible for it now. The next clinician reads it first. Later readers use it to find where a pending result or a follow-up was lost.

Decision-making, not a billing level

You may have met the phrase **medical decision-making**, or MDM, in a coding lecture or a message from your billing department. There it names a grade. The coding rules used for many visits rate how complex a decision was, based on the

problems addressed, the data reviewed and the risk of the management chosen. That rating is one of the ways a visit's billing level is set.

This book uses the phrase in a plainer sense: the reasoning that took you from the information to the plan, written so the next reader can follow it. The two meanings overlap, but they ask different questions. Billing asks how complex the decision was. The reader after a bad outcome asks why you made it. A note can contain every element a coder looks for and still never say why Mrs. Fitch went home. A note padded to reach a level gets longer without getting clearer, and the padding buries the three or four sentences a reviewer actually needs. This book leaves coding aside. Questions about billing levels belong with your organization's coding and compliance staff.

Your name at the bottom

When you sign a note, you become its author. Your signature says that you wrote the note, or reviewed and adopted it, and that it is accurate. It covers every line: the sentence a template inserted, the review of systems a scribe typed, the section a student drafted. Whether you typed a line or only accepted it, it is yours.

Many NPs also work with a physician whose name appears in the record. Depending on the state and the setting, an NP may practice independently or under a **collaborative agreement**, an arrangement, usually in writing, that sets out how an NP and a physician work together. Some states use other names, such as practice agreement or protocol, and the required form and content vary. An NP may also practice under **supervision**, in which a physician holds a defined oversight role. This book gives no state rules. Ask your state board what applies to you, and ask your employer what its policy requires.

A second signature tells a reader less than people assume. A **cosignature** is a second clinician's signature on a note someone else wrote. An **attestation** is a statement the second clinician adds about their own role: that they saw the patient, reviewed the note or agreed with the plan. Depending on the policy behind it, a cosignature may mean that the physician examined the patient, that they read the note line by line, or only that they signed it from a queue the next morning. The signature alone does not say which. A reviewer may assume a review that never happened, or doubt one that did.

Your own note can settle the question, because you know what the physician actually did. In this scenario, Priya's department asks the attending to cosign NP notes. Her last line names Dr. Grant and the time, says that he did not examine Mrs. Fitch, and states what he agreed with. When he cosigns the next morning, nobody has to guess what his part was. The rule also works the other way. “Discussed with attending” does not belong in a note when the only contact was a hallway remark that never touched the decision. The record should show who decided, who was consulted and what each of them did.

CHAPTER 3

Where a Claim, a Complaint, and a Review Begin

What the closed-claims reports can tell you

A **closed claim** is a malpractice claim that has reached its end: withdrawn, dismissed, settled or decided at trial. Once a claim closes, the insurer that handled it knows what was alleged, what the record showed and what the claim cost. Insurers study these files in bulk to learn where their policyholders get into trouble, and some publish what they find.

For nurse practitioners, one public series of this kind comes from CNA, an insurer, and NSO, the Nurses Service Organization, which offers malpractice coverage to nurses and NPs. Together they have published claim reports on NPs, drawn from closed claims of NPs insured through NSO. Read the most recent edition they have published, and read the methodology first. This book quotes no figures from it. Percentages and payment amounts change between editions, and a number printed in a book stays wrong after the report is updated.

The methodology sets the limits of what the data can say. The claims come from one insurer's policyholders, not from every NP in the country. Claims often close years after the care, so each report describes practice as it was, not as it is. And a claims report counts only what reached the insurer: a case reviewed inside a clinic and never reported will not appear in it. This book groups the allegations such claims raise under four headings of its own: the diagnosis, the

treatment, the medication and the follow-up. Check how the current edition labels its categories.

One note, three processes

Gary Pruitt, fifty-six, has had a burning feeling high in his abdomen for a week. It comes mostly after meals, but twice it came while he was climbing the stairs at work. At 10:15 on a Tuesday he describes it to Devon Marsh, an NP at a family practice clinic for fourteen months, who thinks of the heart as well as the stomach. Neither man exists; the case, like every other in the book, is constructed.

Devon gets an ECG in the clinic, which is normal, starts omeprazole and orders an exercise stress test, asking the front desk to book it within the week. He tells Mr. Pruitt to call the clinic if nobody has phoned him by Friday, and to call 911 if the burning comes on at rest or with sweating or shortness of breath. The booking request is lost in a staffing change. Twelve days later, with no test booked, Mr. Pruitt has a heart attack at home. He survives, with some damage to his heart.

That one visit can now set off three separate processes, and each one reads Devon's note with a question of its own. The first is a **malpractice claim**: a civil claim for money damages, built on the elements of negligence described earlier. Mr. Pruitt's attorney and an expert witness will ask two things. Would a reasonably prudent NP, knowing what Devon knew that morning, have sent him to the emergency department or made sure the test happened sooner? And would either step have spared his heart? An insurer usually defends the claim, either Devon's own or his employer's. Any payment made on his behalf, by settlement or judgment, would be reported to the **National Practitioner Data Bank**, a federal repository that hospitals and licensing boards can query.

The second is a **licensing board complaint**. Mr. Pruitt's wife writes to the state board that licenses Devon, saying he “brushed off” her husband's chest pain. Anyone can file a complaint, and it needs no injury and no proof that a different decision would have changed the outcome. The board investigator asks whether Devon's assessment, his documentation and his practice met the standards his license requires and stayed within his scope of practice. What is at stake is the license rather than money, and outcomes range from a dismissed

complaint to conditions on practice or formal discipline. Not every malpractice policy pays for a lawyer in a board matter, and an employer's policy may not cover you there at all, so ask your insurer, in writing, what yours does.

The third process is the one new NPs tend to forget, though it is often the first to start: the **employer review**. The clinic learns of the heart attack, an incident report is filed, and the medical director asks for a **peer review**, an internal examination of a clinician's care by colleagues. The reviewer is a physician or senior NP who knows how long the clinic's referrals take and who books them.

The questions are practical. Was the urgency of the test passed on, and to whom? Was Mr. Pruitt told what to do if nobody called? Is this one visit, or a pattern in Devon's charts? The outcome can be a conversation, a focused review of his cases, limits on his practice or the end of the job. Whether such a review can later be used outside the organization depends on state law, so ask your risk manager.

All three processes read the same note, signed once on a Tuesday morning. Devon cannot know which of them will come, or in what order, and he cannot write a version for each. He does not need to. Each one asks, in its own vocabulary, what he saw, what he considered and why he decided.

Four allegations, four places in the note

A claim usually reaches paper as a letter from an attorney, but it begins earlier, at one moment in a visit. That moment has a place in the note, and the answer to the allegation either appears there or it does not. A diagnosis allegation says that a condition was missed or found too late. The moment is the assessment, and the question is whether the dangerous possibility was considered. Devon's note read:

> Epigastric burning x 1 wk, worse after meals. ECG NSR. Likely GERD. Start omeprazole 20 mg daily. Refer stress test. F/u 2 wks.

A rewrite of the same visit goes no further than what Devon asked, found, thought or did:

56M, smoker, father had MI at 60. Epigastric burning x 1 wk, mostly within 30 min of meals; twice while climbing stairs at work, resolved within 3 min of rest. No pain at rest, no dyspnea, diaphoresis or syncope. Abdomen soft, nontender; heart regular, lungs clear. ECG 10:30: NSR, no ischemic changes. Assessment: GERD most likely given timing with meals, but the exertional episodes keep angina possible, and a normal resting ECG does not exclude it. Plan: omeprazole 20 mg daily. Exercise stress test ordered; front desk asked at 10:45 to book within 1 week. Patient to call clinic if not contacted by Friday. Told to call 911 for burning or chest pain at rest, or with sweating or shortness of breath. Recheck 2 wks.

The rewrite does not settle whether Devon was right. An expert may still argue that the test should have happened within days. But the argument is now about a decision anyone can see Devon making, with the danger named and a plan built around it. Nobody has to ask whether he thought of the heart at all.

A treatment allegation says the plan itself was wrong: the wrong level of care, a consultant not called, a worsening patient not escalated. A related allegation says the patient refused the plan, or left, without understanding what that meant. Here the moment is the plan and the disposition. The line that answers it says why this level of care was safe enough for this patient and what would have changed it. When the patient chose otherwise, it also says what they understood when they made that choice. The scenarios on delayed imaging, the absent consultant, capacity, refusal, leaving against medical advice, discharge and the night handoff are built around that line.

A medication allegation says the wrong drug or dose was prescribed, an allergy or interaction was missed, monitoring never happened, or a controlled substance was prescribed without the checks the rules required. The moment is the prescription. The line that answers it records why this drug suited this patient, what risks were discussed and what checks you ran. The chapter on prescribing works that line from the side of the drug you prescribe, and the chapter on the antibiotic or scan you decline works it from the other side.

A follow-up allegation is the one most likely to fall between notes: the result nobody acted on, the referral never completed, the patient never told. In Devon's case this is where the rewrite does the most work. A note cannot make anyone book a test. It can show that the next step had an owner and a deadline,

and that the patient knew what to do if the step never came. The scenarios on abnormal results, on missed visits and lost referrals, and on telehealth and portal messages all turn on that point.

Board complaints can also turn on matters with no injury behind them. A patient may say they were spoken to with contempt. A pharmacist may question a prescription, or an entry may look as if it was changed after the fact. The chapters on objective language, on hostile encounters and on the chart as a document cover those.

What a defensible note is, and what it is not

Devon's rewrite has four properties, and every scenario in this book aims for them. It is **contemporaneous**, meaning written at the time of the care or soon after, before memory starts filling gaps. The rules for entries made later belong to the chapter on templates and addenda. It is specific: times, values and the warning actually given, not "Refer stress test" alone. It is honest about uncertainty: where "Likely GERD" left the heart out, the rewrite says what was still possible and what was being done about it. And it is honest in the plainest sense, because every line records something that happened.

It is also not a shield. Written exactly this way, it would not have stopped the claim, the complaint or the review. What it changes is what all three examine: a decision with its reasons, instead of a conclusion with no reasons at all.

How the scenario chapters are built

Every scenario chapter opens with a constructed scenario: a named clinician, a setting, a patient and the clock. Next comes what the record will be asked: the questions a claim, a complaint or a review would bring to it. Then comes the weak note, as a competent, busy NP might write it: accurate as far as it goes, but thin.

Beside it is the defensible rewrite of the same encounter. It adds nothing the clinician did not do, so the difference lies in the writing, not the care. Last comes the short list of what the note must contain: elements, not sentences, because the note should be in your words. When a patient is walking out or a page is waiting, the list is the part to reach for.

CHAPTER 4

Writing the Differential You Actually Ran

The assessment that stops at the answer

> 33F L chest pain x 2 days. Chest wall tender. ECG NSR, CXR neg. Assessment: MSK chest pain. Ibuprofen, RTC PRN.

Talia Brennan signs that note on a Saturday afternoon as the NP covering the fast-track area of a suburban emergency department. She and her patient are invented; the visit behind the note begins at 13:40, when she sees Alicia Moreno, thirty-three, who has had sharp pain on the left side of her chest for two days. It began the morning after she helped her sister move furniture, and it gets worse when she breathes deeply or twists.

Talia thinks of a pulmonary embolism before anything else, and her questions show it: coughing blood, leg swelling, earlier clots, hormonal contraception, recent surgery or long travel. She asks whether the pain changes with exertion or with position. She listens to both lungs, presses along the ribs, looks at both calves, and gets an ECG and a chest X-ray. Everything points to a strained chest wall, and she sends Ms. Moreno home with ibuprofen.

The assessment in her note is a single line, and it is a conclusion. The ECG and the X-ray suggest that Talia was looking for something, but the note does not say what, or what she found when she looked. Suppose Ms. Moreno comes back four days later with a pulmonary embolism. As the previous chapter showed, the first question a reviewer brings to a diagnosis allegation is whether

the dangerous possibility was considered at all. This note cannot answer it. Talia's reasoning happened in the room and never reached the page.

Rewritten, with the plan left aside because the discharge has a chapter of its own, the same visit reads:

> 33F, L-sided chest pain x 2 days, began the morning after moving furniture. Sharp, worse with deep breath and twisting. Not exertional; no change lying flat or sitting forward. No dyspnea, hemoptysis, fever, leg pain or swelling. Copper IUD, no hormone use. No prior DVT/PE; no surgery, trauma or long travel in past month. Nonsmoker, no diabetes or hypertension, no family history of early heart disease. HR 84, SpO2 98% RA. Pain reproduced exactly by pressure over L 4th–5th costochondral junctions. Breath sounds equal, no rub. Calves symmetric, nontender. ECG 13:55: NSR, no ST changes, no PR depression. CXR: no pneumothorax, no infiltrate.
>
> Assessment: L costochondral strain, most likely given onset after lifting and pain reproduced on palpation. PE considered: judged low risk on clinical grounds, then PERC applied, none of 8 criteria present (age 33, HR 84, SpO2 98% RA; no hemoptysis, hormone use, prior VTE, unilateral leg swelling, or recent surgery or trauma). No further testing for PE. Pneumothorax: breath sounds equal, none on CXR. Pericarditis: no positional change, no rub, ECG as above. Cardiac ischemia: not exertional, no risk factors on history, ECG normal.

Every line of the assessment has the same shape: a diagnosis, the reason it was kept or set aside, and behind that reason a finding the reader can locate higher in the note. A reviewer does not have to agree with Talia's judgment to see that she made one, and on what grounds. Nor does the rewrite describe care she did not give. The question about contraception, the pressure on the ribs and the look at the calves all happened in the room. The weak note left them there.

The diagnosis that had to be thought of

Most complaints carry at least one diagnosis that would do serious harm if it were missed. A reviewer applying the standard of care will ask whether a reasonably prudent NP would have thought of it for that presentation, and then whether you did. For a young woman with pleuritic chest pain, pulmonary

embolism is one such diagnosis. Which diagnoses belong on that list for which complaint is clinical knowledge, and this book assumes you have it. What the book adds is how to get the one you considered onto the page.

First, name it. A reviewer will not infer a pulmonary embolism from a chest X-ray, which could have been ordered to look for pneumonia, a rib fracture or a collapsed lung. Then give it a reason of its own, made of specific findings. “PE unlikely” by itself repeats the weak note's problem one line lower: a conclusion with nothing underneath. The reason also has to belong to that diagnosis. Ms. Moreno's tender ribs make a chest wall strain likely, but tenderness on palpation does not by itself make an embolism unlikely. That is why the rewrite gives the embolism its own grounds instead of leaning on the ribs.

Naming a diagnosis has a consequence. Once it appears in the assessment, the reader expects to find either the reason it was set aside or the step taken because it could not be. A dangerous diagnosis written down and then left alone reads worse than one never mentioned: it proves you thought of it and shows nothing done about it. That is the practical sense of writing the differential you actually ran. A list of ten diagnoses copied from a reference, each followed by nothing, does not make a note look thorough. It hands the reviewer ten questions the note cannot answer.

Pertinent negatives that carry the exclusion

The reasons in Talia's assessment are only as strong as the findings they point to, and most of those findings are negatives. The chapter on the provider note introduced *pertinent negatives*, the findings you looked for and did not find. In a differential they have a specific job: each one is chosen by the diagnosis it argues against. “Copper IUD, no hormone use” and “No prior DVT/PE” sit in the history because Talia was weighing an embolism. “Calves symmetric, nontender” sits in the exam for the same reason. “No PR depression” sits in the ECG line because she was checking for pericarditis. The negatives come from history, exam and data alike, and a reader can trace each one back to the alternative it answers.

Now set beside them the exam a template might produce for the same visit:

> Lungs: clear to auscultation bilaterally. CV: regular rate and rhythm, no murmurs. Extremities: no edema. ROS: all other systems negative.

Every line is normal, and none of it does the work. The same words can appear on any chest-pain patient the template touches, so a reviewer cannot tell which lines were checked for Ms. Moreno and which were defaults. “No edema” is not the same finding as “calves symmetric, nontender.” The first describes a leg; the second describes a leg examined for a clot. “All other systems negative” does not tell anyone that she was asked about coughing blood. A pertinent negative carries an exclusion only when it is specific enough to show it was sought for a reason, and when the assessment points to it. A templated normal exam can be accurate and still argue against nothing. When the template says something the history contradicts, the problem is a different one, taken up in the chapter on templates and copy-forward.

Scores and decision tools

Talia also used a decision rule, which a hurried note often records in two words: “PERC neg.” The Pulmonary Embolism Rule-out Criteria, or PERC, are a list of eight findings. When none is present in a patient the clinician has already judged low risk, the rule supports stopping the workup for embolism without further testing. Those two words give the reader the tool and the result. They leave out everything the reader needs to check the decision.

Whenever you rely on a score or a rule, three things belong in the note. The first is its name, written so a clinician will recognize it. The second is the inputs, meaning the values you actually used. A reviewer will set them beside the triage vital signs, the nursing notes and your own history, and a heart rate or a medication that disagrees with your score will be noticed. Listing the inputs shows that the tool was fed the patient in the chart. The third is the tool's limits, and whether this patient fell inside them. PERC is meant only for patients already judged low risk on clinical grounds. In a patient who was not, a negative result does not carry the same meaning. That is why Talia's note records her judgment first and the rule second.

The same holds for any tool, whether a Wells score, a HEART score or a head CT rule. A bare number such as “HEART 3” cannot be checked, and it

does not show which part of the history or which result produced it. If you depart from what a tool suggests, write why: the score was low, but one finding it does not count made you test anyway. A rule answers a narrow question about one diagnosis, so it supports the differential and does not replace it. And record only the tools you applied at the time. A score calculated afterward describes a calculation that played no part in the decision.

The short version for a full board

Not every visit leaves time for the rewrite in full. If Ms. Moreno had arrived at 19:00, with eleven patients on the fast-track board, the history and exam would still have had to carry the findings, but the assessment could have shrunk to this:

> Assessment: L costochondral strain (onset after lifting, pain reproduced on palpation). PE: low risk clinically, PERC negative, inputs above. Pneumothorax: none on CXR, breath sounds equal. Pericarditis, ischemia: ECG normal, pain not positional or exertional.

That is four sentences, and each alternative still has a name and a reason. The method behind them is short too. Lead with the working diagnosis and the one or two findings that support it. Then list the dangerous alternatives you actually weighed, the ones a reviewer would expect for this complaint, and give each a reason in a few words that points to findings already in the note instead of restating them. Alternatives that share a reason can share a line. Leave out the remote possibilities you never seriously entertained, because they were not part of the differential you ran. You can cut the narration; you cannot cut the reasons. If time leaves room for only one full reason, it goes to the dangerous diagnosis.

The shape works beyond the fast track. An admission H&P, a progress note on day three or a primary care visit for a cough can use the same pattern of name, reason and finding. The person who opens the chart later will not be asking for a longer note. They will be looking for the moment you set each possibility aside, and for the finding that let you do it.

CHAPTER 5

The Working Diagnosis and What Would Change It

Six hours into a stomachache

The urgent care clinic where Erin Kowalski works as an NP closes at 21:00 and has no imaging on site. At 18:20 on a Wednesday she sees Ben Sorensen, nineteen, whose stomach began to ache around noon. The pain sits around his navel and comes in cramps. He vomited once at 15:00 and has eaten almost nothing since breakfast. His roommate was sick with vomiting last week.

His temperature is 99.5°F. His abdomen is soft and mildly tender around the navel, with no guarding and no rebound tenderness. Pressing on the lower right side hurts no more than pressing anywhere else. He climbs on and off the table easily, hops on one foot without wincing and keeps down the water he sips in the room.

Erin thinks a stomach virus is the likeliest explanation. She also knows that appendicitis can begin as vague pain around the navel before it settles in the lower right abdomen, and that six hours in, an exam may not yet tell the two apart. What she does next depends on how she holds that uncertainty. So does her note.

Two ways to write it wrong

Here is one note she could write about this invented patient:

> 19M abd pain x 6 hrs, vomiting x1, roommate w/ similar illness. Abd soft, mild periumbilical tenderness. Dx: viral gastroenteritis. Ondansetron, clear fluids. RTC PRN.

And here is another:

> 19M abd pain x 6 hrs. Ddx includes gastroenteritis, appendicitis, bowel obstruction, pancreatitis, kidney stone. Cannot rule out surgical abdomen. Etiology unclear. Clinical correlation recommended. Advised ED eval if symptoms persist or worsen.

The first note is certain, and the certainty is the problem. Suppose Ben comes back two days later with a ruptured appendix. A reviewer will find a diagnosis stated as fact six hours into the illness, a plan that rests entirely on it, and nothing to show that Erin knew the picture could still change. She did know, but the note gives her no credit for it.

The second note looks careful, and a reviewer distrusts it just as quickly. It names appendicitis and then shows nothing done about it. As the chapter on the differential explained, that reads worse than leaving it out. "Cannot rule out" is true of almost any diagnosis at any visit, so it says nothing about how worried Erin was. "Clinical correlation recommended" belongs in a radiology report, where it asks the treating clinician to weigh a finding; in a provider note, the treating clinician is the author. Like "RTC PRN," "persist or worsen" gives Ben no threshold he could recognize once he is home and the clinic is closed. Taken together, the note shows a clinician who saw danger everywhere and planned for none of it.

The two notes fail in the same place. Neither says what Erin believed at 18:40, how firmly she believed it, or what would have changed her mind. Each of those fits in a sentence.

A diagnosis with a date on it

A *working diagnosis* is your best explanation for now, held firmly enough to act on and loosely enough to revise. The *final diagnosis* is the one the facts eventually confirm, sometimes days later and from someone else. Reviewers

know the two often differ early in an illness, so a working diagnosis that later changes is not an error in itself. What they distrust is a working diagnosis written as if it were already final.

The wording carries the difference. “Most consistent with” and “favor X over Y because” state a leaning and its basis. “Not excluded” leaves a possibility honestly open, provided the next words say what is being done about it. Avoid “r/o” as a diagnostic label: some readers take “r/o appendicitis” to mean still under consideration and others to mean already excluded, and months later nobody can tell which you meant. Match the strength of each word to the evidence. If one explanation is clearly ahead, “most likely” is accurate. If you cannot yet tell two apart, say so, and say what will.

Then add one more sentence: what would change your mind. Here is Erin's assessment, rewritten:

> Assessment: abdominal pain x 6 hrs. Working diagnosis: viral gastroenteritis, favored given crampy, intermittent pain, one episode of vomiting, roommate ill with vomiting last week, tolerating fluids in clinic. Early appendicitis not excluded: periumbilical pain can precede localization, and at 6 hrs the exam may not yet show it. At 18:35: T 99.5°F, no RLQ tenderness, no guarding or rebound, hops without pain. Would reconsider appendicitis if pain localizes to the RLQ, becomes constant or steadily worse, or hurts with walking, or if fever develops or he cannot keep fluids down.

The rewrite drops the diagnoses Erin never seriously weighed and keeps the two she did. Its last sentence is the hinge of the note. It shows an uncertainty that was real, specific and watched, and it gives the plan its content.

The plan the uncertainty writes

Every condition in “would reconsider if” has to reappear in the plan in two forms. It becomes something a clinician will check at a stated time, and something the patient will watch for until then. A reviewer will set the assessment beside the plan and look for the match. A risk that the assessment names and the plan never mentions looks like the moment it was forgotten. Erin's plan does both:

> Plan: ondansetron, small sips of clear fluids. Options discussed: (1) ED now for blood work and imaging, likely several hours, may show nothing; (2) home, recheck here 09:00 tomorrow, when the exam may show what it cannot now, accepting that appendicitis, if present, would be found later. Patient prefers 2, to avoid a night in the ED; roommate home overnight, has a car. Told to go to the ED tonight, not this clinic (closes 21:00), if pain moves to his lower right belly, hurts when he walks or rides over bumps, or keeps getting worse, or if he gets a temperature of 100.4°F or higher or cannot keep fluids down. Repeated the list back. Recheck booked 09:00; front desk to call him if not here by 09:30.

Three details do the work. The recheck has a time and an owner, so the reassessment is a commitment rather than a hope. The warning signs are the conditions the assessment named, translated from "RLQ" into words Ben will use about his own belly. And the instructions name a place that will be open when he needs it. In a hospital the person watching is usually a nurse, and the same conditions become limits set in advance, as in Nadia Brooks's rewritten entry in the chapter on what a chart becomes after a bad outcome. The chapter on discharge works these instructions in full. What matters here is their source: the uncertainty you wrote down, not a generic sheet.

When the evidence leaves a real choice

Erin did not decide alone where Ben would spend the evening, and she did not have to. With a reassuring exam six hours in, going to the emergency department now and coming back for a recheck in the morning were both reasonable, and each had a cost that the plan names. When the evidence leaves a real choice like this, the patient's values belong in the decision. That is *shared decision-making*: the clinician sets out the reasonable options and what each one means, and the patient's priorities settle which one fits.

On the record, a shared decision has three parts. The options appear as they were offered. Each comes with what it trades away, in terms the patient could weigh. And the choice is recorded with the reason the patient gave and anything that makes it safe, here the roommate and the car. Priya Raman's plan, in the chapter on the provider note, offered that kind of choice in two sentences.

"Risks and benefits discussed, patient agrees with plan" does none of this: it names no option, risk or preference, and it would read the same for any patient.

There is one limit. A choice is shared only when you would stand behind every option you offer. If Ben had shown rebound tenderness and a fever, going home would not have been a reasonable option. His wish to go anyway would have been a refusal, not a preference. Writing that up as a shared decision would misstate what happened, and that note belongs to the chapter on informed refusal.

The symptom that has no name yet

Some visits end without a working diagnosis worth the name. Take a constructed one from a primary care practice. Rachel Dunmore, one of its NPs, sees Joan Petrakis, fifty-four, at 10:40 on a Monday; Mrs. Petrakis has felt tired for three months. The clinic scale recorded 162 pounds four months ago and 151 today, and Mrs. Petrakis has not been trying to lose weight. Nothing in the history or exam points anywhere in particular, so Rachel orders a first set of blood tests.

Two weak entries are tempting. "Fatigue, likely stress" gives the symptom a convenient explanation with nothing underneath it. Once such a label reaches the problem list, later notes tend to repeat it until nobody looks further. "Weight loss, r/o malignancy" has the ambiguity described earlier, and it names the most frightening possibility without saying what will settle it. The honest version makes the symptom itself the problem and says where the search stands:

> Assessment: unintentional weight loss, 11 lb over 4 months (162 to 151 lb, clinic scale), with fatigue x 3 months. Cause not yet identified. No fever, night sweats, cough, change in bowel habits or blood in stool; appetite reduced; depression screen negative today. Initial labs ordered. Plan: will call patient with results Thursday. If not reached, portal message that day and a second call Friday. Weight on the same clinic scale at every visit. Return visit in 2 weeks whatever they show, sooner for fever, night sweats, blood in stool or further weight loss. Patient told the cause may take more than one visit to find; if labs do not explain it, next steps chosen together at the 2-week visit.

A symptom is a legitimate entry in an assessment, and “cause not yet identified” is a sentence a reviewer can respect. The hedged version of Erin's note said something similar with “etiology unclear,” but Erin had a leaning and hid it behind the phrase. Rachel has no leaning yet, says so, and puts a date on what happens next. An unexplained complaint becomes dangerous on the record when it lacks a next step, not when it lacks a diagnosis. Keep the symptom open on the problem list until something explains it. Let each later note say what has been excluded and what remains, so that a reader a year from now can follow the question to its answer, or to the last day anyone was still pursuing it.

CHAPTER 6

Facts, Quotes, and What Never Goes in the Chart

A hard morning, written two ways

A new IV line went into Raymond Teague's arm at 05:15 on Sunday. Overnight he had pulled out the old one, saying it burned, and the cefazolin dose due at 02:00 was given at 05:30. The night nurse filed an incident report about the late dose. Mr. Teague, sixty-two, was admitted on Friday with cellulitis of the left lower leg, and he has type 2 diabetes. At 07:40, when Kendra Walsh begins her weekend rounds as the hospital medicine NP, he is first on her list.

Theirs is a story written to teach. When she walks in, he is sitting up and speaking loudly. He swears several times while describing the night. He says his leg pain is 8 out of 10, that "the pills don't touch it," and asks for IV hydromorphone by name. His fasting glucose is 287; asked about it, he says he stopped metformin about four months ago. Before she leaves he says he is going to get a lawyer. At 08:30, still irritated, she writes:

> Pt belligerent and uncooperative on rounds, cursing at staff. Pulled out IV overnight; IV team didn't come for 3+ hrs (unit short-staffed again), so cefazolin given 3.5 hrs late. Hx noncompliance, off metformin for months. Drug-seeking, demanding IV Dilaudid. Apparently he knows more about his treatment than we do. Incident report filed re: late dose. Pt states he will "get a lawyer." Continue current plan.

Very little of that is false. Mr. Teague was loud, the dose was late, and he did ask for hydromorphone. But almost every line gives a verdict instead of evidence, and several lines are not about the patient at all. The version below is built from observations, quotations with their speakers, and times:

> 07:40, seen at bedside. Sitting up, speaking loudly; swore several times while describing the night. States: "That IV was burning and nobody came, so I pulled it." Per RN Kwame Osei, PIV removed by patient ~01:40, new PIV R forearm 05:15; cefazolin due 02:00 given 05:30. Reports L leg pain 8/10, "the pills don't touch it"; asked for IV hydromorphone by name. Last oxycodone 5 mg at 04:10. States he stopped metformin ~4 months ago: "It tore up my stomach." Fasting glucose 287.
>
> Exam: T 99.1°F, HR 92. L lower leg erythema within the border marked on admission; tenderness limited to the red area; no crepitus, blisters or dusky skin. New IV site clean. Asked him to call his nurse if the IV burns again rather than remove it; he said, "Fine, as long as somebody comes."

The plan is left aside, because the pain request belongs to the chapter on prescribing. What changed is the language. The rewrite keeps the loud voice, the swearing, the removed line, the late dose and the request by name. It drops the verdicts on Mr. Teague's character, the account of who failed overnight, the report, the lawyer and the sarcasm. Where the weak note offered explanations, the rewrite gives times.

Observations instead of adjectives

An adjective about a patient is a conclusion. "Belligerent" asks the reader to accept your judgment of a man they have never met. "Speaking loudly; swore several times while describing the night" gives them what you saw and heard. Later readers will judge anyway, so let them judge from the facts and not from your mood at 08:30. A simple test separates the two: could someone else in the room have confirmed it? A camera would have recorded the volume and the words. It would not have recorded belligerence.

The test does not ban clinical description. “Diaphoretic,” “pale” and “alert” are findings with an accepted meaning, and another clinician could check them. The words to replace are judgments of character, motive or mood: rude, difficult, manipulative, uncooperative, hysterical. “Agitated” becomes “pacing, pulled at the IV line twice, did not sit when asked.” “Intoxicated” becomes “odor of alcohol, slurred speech, unsteady gait,” followed by a level if you drew one. “Anxious” becomes “tearful, hands shaking.”

There is a clinical reason too. Confusion from low blood sugar, low oxygen or infection can look like hostility. “Uncooperative” records your interpretation and erases the finding underneath it. If the patient turns out to have been delirious, a reader who already knows the ending will see that word as the moment the sign was missed.

Quoting the patient

The patient's own words matter most where they carry the decision. That means a symptom described in terms that change the workup, such as “the worst headache of my life.” It means a refusal, a plan to leave, a thought of self-harm or a threat to hurt someone. It means what the patient says back when you check what they understood. And it means the reason a patient gives for something you might otherwise label. “It tore up my stomach” turns Mr. Teague's stopped medication into a side effect, and a side effect has remedies.

Quotation marks make a promise: these exact words were said. Put them only around words you are sure of, and write them down soon, before memory smooths them. If you remember only the gist, write the gist without the marks: *States he stopped metformin because of stomach upset.* Do not correct the grammar or tidy the wording. Name the speaker: a daughter's account is not the patient's, and a nurse's report is not your exam. When an interpreter is used, note it, since the English inside your quotation marks is the interpreter's rendering.

The same marks let you record a complaint about the care without making it yours. “Nobody came,” in Mr. Teague's words and attributed to him, is a fact about the encounter. The same claim in your voice would be an accusation. When the words themselves are the clinical fact, as in a threat, quote them exactly, profanity included; when they are not, describe the behavior, as the

rewrite does with "swore several times." Quote a phrase or a sentence, not a transcript. A note made mostly of quotations buries your own observations and your reasoning.

Labels that read badly later

Some words are not descriptions at all but labels: short verdicts that travel. They get copied into later notes and onto the problem list, as the previous chapter warned with "likely stress," and each copy sits further from the facts behind it.

"Noncompliant" says the patient failed without saying at what, for how long or why. A reviewer reads it as the point where the clinician stopped asking. Swapping in "nonadherent" helps less than it seems, because the real problem is the missing facts, not the word. The factual version names the behavior, the patient's reason and what you did about it: "Has not taken metformin for ~4 months; states it caused stomach upset. Discussed extended-release metformin and other options; see plan."

"Drug-seeking" names a motive, and motives cannot be observed. The factual version records the exact request, the recent doses, any checks you ran and your assessment of the pain itself. The label has a clinical cost too. A patient with an infected leg whose pain keeps climbing needs another look at the leg, and a word in the last note that explains the pain away makes that look less likely. Kendra's exam line shows that she looked. "Poor historian" usually describes the conversation rather than the patient; write what you could not obtain and where the information came from instead. "Frequent flyer" belongs nowhere in a record.

Patients have a legal right to their records, and under the federal information-blocking rules in force since 2021, most US providers may not withhold signed notes without a recognized exception, so many notes reach the patient portal as soon as they are signed. If Kendra signs her 08:30 version, Mr. Teague can read "belligerent" and "noncompliance" on his phone before she finishes rounds. A patient who reads "drug-seeking" about himself may stop reporting his pain, or may answer with a complaint to your board. As the chapter on where claims begin noted, a complaint needs no injury behind it.

What never goes in the chart

The chart is the record of the patient's care. Some things that feel urgent while you write belong somewhere else, and each has a place to go.

Blame of colleagues comes first. “IV team didn't come for 3+ hrs” states a cause Kendra did not witness; the IV nurse may have been at a code. The facts that matter are in the rewrite, each with its time. A note that accuses invites the other clinician's note to answer, and a record in which clinicians argue helps no one defend the care. Writing about a colleague who never responds is worked in the chapter on the consultant who never comes.

Staffing complaints are next. “Short-staffed again” is an opinion about the cause, not a clinical fact. If staffing delayed care, the delay goes in the chart as a fact with its time and its source, the way the rewrite records the late dose. The cause goes to your manager or through your organization's reporting channel, where someone can act on it.

The incident report is an internal document, and organizations generally keep it inside their safety and quality process. How far the law protects it varies, and depends partly on how it is handled. A line in the chart that names the report points every reader of the record toward it. The chart carries the clinical event instead: what happened to the patient, what you found and what you did. The chapter on the adverse event returns to this boundary.

Litigation stays out too, whether as your guess or as the patient's threat. “Pt states he will 'get a lawyer'” changes nothing about the care Mr. Teague needs. Tell your risk manager instead, and ask how your organization wants such statements handled. “Documented for legal purposes” is worse: it tells every reader you were writing for a lawyer rather than for the next clinician.

Last comes sarcasm, along with jokes, exclamation points and your feelings about the encounter. “Apparently he knows more about his treatment than we do” may have been a private sigh at 08:30. Read aloud to a jury, it sounds like contempt, and no explanation given later will change how it sounded.

Reading your note as a stranger would

Before you sign, give the note one more pass, for tone alone. It takes less than a minute. Read it as three strangers would: the patient opening it in the portal, the

colleague named in it, and a juror who has never met any of you. For every adjective about the patient, ask whether you could point to what you saw or heard. Check that each pair of quotation marks holds words actually said, with a speaker. Look for any line that explains someone else's failure, the staffing, a report or a lawyer.

The reread matters most on the notes you write while angry. Kendra's two versions describe the same morning. Much of the distance between them can be covered in that one pass for tone, made before the patient, the colleague or the juror reads a word.

CHAPTER 7

Templates, Copy-Forward, and the Late Addendum

Yesterday's exam, signed today

At 05:30 on a Wednesday, a nurse charts new confusion in Evelyn Kraus, eighty-one, admitted on Monday with worsening heart failure and on IV furosemide since. The hospital medicine NP on her case, Mateo Ruiz, sees her at 08:45: drowsy, oriented only to herself, moving all four limbs. Her sodium, 134 on Tuesday, is 129 this morning, and at 08:55 he holds the furosemide. Mateo's morning is fiction, down to the minute.

At 09:10 he opens Tuesday's progress note, copies it forward into a new one, updates the hospital day, the weight and the sodium, and signs at 09:24. Part of it reads:

> Hospital day 3. Feels better, breathing easier, slept well. Exam: alert and oriented x3, NAD. Lungs: fine crackles at both bases. 1+ edema to mid-shin. Wt 71.2 kg. Na 129. Assessment: acute on chronic systolic heart failure, improving. Plan: continue IV furosemide 40 mg BID. Start lisinopril tomorrow if creatinine stable.

Every copied line was true on Tuesday, when Mateo wrote it. Signed on Wednesday, at least three are false: she did not say she felt better, she was not oriented, and the drug he says to continue was stopped twenty-nine minutes

before he signed. “Tomorrow” now means Thursday, because copied text keeps the tense of the day it was written. And the one new number, a falling sodium, sits beside an assessment that still says “improving.”

On the timeline, this note sits between the 05:30 nursing entry and the 08:55 order and fits neither. A reviewer can draw only two conclusions: Mateo did not examine her, or he examined her and missed her confusion. Neither is true, but his own note now contradicts the order that shows what he did.

Your signature adopts every line above it, as the chapter on the provider note explained, so yesterday's exam becomes your statement that you examined the patient today and found exactly that. *Copy-forward*, carrying an earlier note's text into a new one, saves real time, and many organizations permit it within limits set by policy. What it costs is the comparison a progress note exists to make. These are the lines that should have changed:

> Hospital day 3. Seen 08:45. RN charted new confusion at 05:30. Drowsy, rouses to voice, oriented to self only; oriented x3 on my exam yesterday. Follows commands, moves all limbs, face symmetric. Lungs: fine crackles at both bases, less than yesterday. Edema 1+ to ankles. Wt 71.2 kg (72.6 yesterday). Na 129 (134 yesterday). Assessment: new confusion, sodium falling on IV diuresis; other causes under evaluation. Plan: IV furosemide held 08:55. Lisinopril deferred until sodium and mental status improve.

Treat a copied note as a draft that is wrong until you have checked it. Stable sections, such as past history, allergies and home medications, can carry forward once you have read them against today's chart. The sections that describe a day (the patient's report, the exam, the assessment, the plan) get written fresh or checked line by line against this morning. Watch for words with a date hidden inside them: today, tomorrow, overnight, “will start.” Then write the comparison yourself. “Less than yesterday” is the kind of phrase copy-forward erases, and it shows a patient being followed.

Defaults that argue with the history

Templates cause the same problem from the other side. Instead of carrying yesterday's facts into today, they supply facts nobody observed. Many exam

templates open with *default normals*, every system already marked normal until you change it, and many review-of-systems templates end in "all other systems negative." Some fields arrive prefilled, such as smoking status or "no known drug allergies." Some phrases insert themselves at a keystroke: "risks, benefits and alternatives discussed," "return precautions given," "patient seen and examined." Each is a sentence you sign without having written it.

A templated exam can be accurate and still argue against nothing, as the chapter on the differential showed. Worse is the template that says something false, and reviewers know where to look. One place is a contradiction inside the note: three days of vomiting in the history, and "GI: denies nausea, vomiting, diarrhea" below it. Another is a contradiction with the timeline, such as "gait steady" for a patient whose nurse, that same hour, charted a two-person assist to the commode. A third is the finding nobody could have made: equal pedal pulses in a patient with a left below-knee amputation, or a full review of systems, all negative, for a patient intubated since admission. The last is repetition: the same exam, down to a misspelled word, across several days or several patients.

One such line costs more than itself. The reader cannot sort the findings you checked from the ones the template supplied, so they discount all of them, including the pertinent negatives that carried your exclusions. An exam recorded but never performed raises a different question, too: not whether your judgment was sound, but whether your record is true. That question reaches every other note you have signed.

The check takes about a minute. Before you sign, read each templated section against the history you took and the exam you did. Delete the systems you did not ask about instead of leaving them "negative," and write "not examined" for what you did not examine. Delete an inserted phrase unless the thing it names happened, and when it did, say what was actually discussed or given.

Correcting a signed note

At 10:05 that Wednesday, Mrs. Kraus's nurse asks Mateo why his note says to continue a drug he stopped. The 09:24 note cannot become the rewrite now. What he can do is leave it standing and add a correction:

> Correction, 10:15, to progress note signed 09:24 today. Subjective, exam, assessment and plan were carried forward in error from yesterday's note. Correct findings at 08:45: drowsy, oriented to self only, follows commands, no focal deficit. New confusion; sodium 129, down from 134. IV furosemide held 08:55, not continued. Lisinopril deferred. Weight and labs as signed are correct.

The correction follows the old rule of the paper chart, where an error got a single line through it, the right entry beside it, and the writer's initials and the date. The old text stays readable, and the new text says who changed it, when and why. Most record systems offer a way to correct or amend a signed note, and your organization's policy says which one to use. What you never do is type over a signed note, delete it and sign a clean copy, or remove an entry because it looks bad. A note in the wrong patient's chart goes through your organization's procedure too, usually via health information management, not the delete key.

Deleting is the worst move because it does not work. A silent edit does not remove the error; it adds a record that the entry was changed, by whom and when. A mistake corrected openly is one of the most ordinary entries in any chart. The same mistake rewritten in silence and found later turns a question about care into a question about honesty, and a reader who doubts one entry doubts the rest. Altering a record once a claim is pending or reasonably expected can also be treated as **spoliation**, the destruction, alteration or failure to preserve evidence. What follows depends on the state and the court, and the consequences can weigh more than the original question about the care.

Late entries and addenda

Some care will be written after it happened, and the record has an honest way to show that. A **late entry** documents care that was given earlier but not written at the time. An **addendum** adds information to a note already signed: a result that came back, a call returned, a detail left out. Organizations name and define the two differently, so follow your policy on the label; the requirements underneath are the same.

At 11:30 Mateo goes back to Mrs. Kraus. Two admissions arrive at noon, and he writes nothing until 15:50:

> Late entry, written 15:50, for bedside reassessment at 11:30. Delayed by two admissions. At 11:30: awake, oriented to person and place, not date. Daughter at bedside said she seemed more like herself. Repeat Na 131 (resulted 11:10). Plan unchanged.

The entry is labeled for what it is. It gives two times and keeps them apart: the system stamps the time of writing on its own, but the time of the care has to be in the text. It states the reason for the delay in a few factual words, with no defense attached. It records only what Mateo remembers with confidence or can take from another source, such as the lab result; the daughter's remark has no quotation marks, because four hours is enough to lose her exact words. A late entry that says it is late is ordinary; one disguised as contemporaneous is not.

A delay costs weight. Written four hours later, before anything has gone wrong, this entry reads as a busy afternoon. Written after a fall or a complaint, the same words read as a record being built for an argument, however accurate they are. Once a complaint, a claim or an attorney's request has arrived, add nothing to the chart about the care in question and change nothing in it, and ask risk management first; the chapter on the adverse event covers that moment.

What the audit trail sees

Behind the chart you read, the system keeps a second record you usually cannot see. The **audit trail**, or audit log, records activity in the chart: which account opened which patient's record, when, and often from which device; what was viewed, created, edited, signed, printed or deleted; and in many systems each earlier version of a changed note. It does not depend on what anyone chose to write. What your system captures, and for how long, is a question for your health information management or privacy office.

After a bad outcome, the organization's reviewers can pull the audit trail, and in many courts the other side can ask for it in a lawsuit. Whoever has it lays it beside the notes, minute by minute. For Mrs. Kraus it would show a note opened at 09:10 and signed at 09:24, a correction at 10:15 and a late entry at 15:50, each saying what it is. The two records agree, and that agreement is what a reviewer looks for. A signed note changed in silence, or an entry describing 08:45 that was created at 16:00 and never says so, shows up just as plainly. In most

systems, so does every look, including a look at the chart of a patient who is not yours.

Before you sign, a change is just an edit. After, it is an entry of its own, with your name and the minute attached. The only kind that helps you is the one that says what it is.

PART II

Orders, Waits, and Pushback

CHAPTER 8

The Test You Order and the Test You Don't

Ice in the parking lot

The coworker who drove Curtis Faber to urgent care saw him fall. At 07:50 on a Tuesday in January, Mr. Faber, thirty-eight, slipped on ice in the parking lot at work, pitched forward onto his outstretched right hand and struck his forehead on the pavement; he was "out for maybe five seconds," the coworker says. The clinic has X-ray on site but no CT scanner; the nearest one is in an emergency department twenty minutes away. His turn comes at 10:20, with Celeste Hartley, the NP on shift.

The Canadian CT Head Rule, which Celeste applies, is real; the case around it is not. He remembers slipping and everything after he came to. He has a mild headache, has not vomited, takes no medications and has had no alcohol. His vital signs and neurological exam are normal, and apart from a swelling on his forehead he shows no sign of a skull fracture. His right wrist is tender in the hollow at the base of the thumb, the anatomic snuffbox.

Celeste orders an X-ray of the wrist with scaphoid views, and it shows no fracture. She does not order a CT of his head. He has none of the rule's seven criteria, and, told that a scan means hours away from home and a small radiation dose, he chooses to go home, where his wife can watch him tonight. The first decision leaves an order, a timestamp and a result in the chart. The second leaves nothing unless she writes it down.

What the record will be asked

After a bad outcome, a reviewer checks the tests from both sides. For every test that was done, the question is why: what was it looking for, and what did the result settle? For a test that could have been done and was not, there are two questions: whether the clinician considered it, and whether the decision was reasonable with what was known at the time.

Whether the test was considered is hard to answer from the record. On the timeline described in the chapter on what a chart becomes after a bad outcome, a CT that was weighed and set aside looks exactly like a CT nobody thought of: in both cases, there is simply no order. Only a sentence in the note tells them apart. Without it, a reader who already knows that the patient came back two days later with a bleed will tend to read the empty space as an omission.

Whether the decision was reasonable is the standard-of-care question, and it is answered with findings that have times attached. The reviewer will look for the facts that made a scan unnecessary at 10:20 and check each against the triage entry. The setting belongs in the answer too. In a clinic without a scanner, a CT means sending the patient to an emergency department, and a note that says so shows the decision where it was actually made.

The note as written

The note Celeste signs at 11:15 reads, in full:

> 38M slipped on ice 07:50, struck forehead, brief LOC. GCS 15, neuro intact. No indication for CT head. R wrist pain, XR neg. Thumb spica splint. Dx: concussion, R wrist injury. Head injury instructions given. F/u 10 days.

"No indication for CT head" is a verdict with nothing underneath it, the same problem the chapter on the differential found in "PE unlikely." A reviewer cannot tell whether Celeste applied a rule, which one, or whether Mr. Faber fell inside it. "Brief LOC" is the very finding that made the scan worth considering, and the note gives it no witness and no duration. Nothing says whether he takes a blood thinner, whether he vomited or whether he remembers the fall.

The wrist line fails differently. "XR neg" seems to close a question the X-ray could not close, because a scaphoid fracture can be invisible on the first film. Celeste knew that, which is why she splinted the thumb and booked the recheck. But "F/u 10 days" does not say what the follow-up is for, who will order the second X-ray or what Mr. Faber was told. Read six weeks later, after a fracture that failed to heal, the note shows a normal X-ray and a plan that could belong to a sprain.

The rewrite

With both decisions on the page, the note reads:

> 38M, slipped on ice in work parking lot 07:50, fell forward onto outstretched R hand, struck forehead on pavement. Coworker who drove him here saw the fall: "out for maybe five seconds." Recalls the slip and everything after coming to. Headache 3/10. No vomiting, no seizure. No anticoagulant or antiplatelet, no bleeding disorder, no alcohol today. Seen 10:20: BP 128/78, HR 74, GCS 15. 4 cm frontal hematoma, no step-off. No hemotympanum, periorbital or mastoid bruising, or fluid from nose or ears. Pupils equal and reactive, no focal deficit, gait normal. No midline neck tenderness, full painless rotation. R wrist tender in anatomic snuffbox; XR with scaphoid views: no fracture seen.
>
> Assessment: concussion with brief witnessed LOC; frontal scalp hematoma. Intracranial injury considered. Canadian CT Head Rule applies (minor head injury with witnessed LOC, age 16 or over, no anticoagulant); none of its 7 criteria present: GCS 15 at 10:20, 2.5 h after injury; no sign of open, depressed or basal skull fracture; no vomiting; age 38; no amnesia before the fall; mechanism a ground-level slip. CT not required by the rule. R wrist: possible scaphoid fracture, which can be occult on the first X-ray.
>
> Plan: rule explained to patient. Options: CT at nearest ED (none on site; several hours, small radiation dose, not needed per rule) or home with someone watching him tonight. He said: "If the rule says I don't need it, I'd rather go home and have my wife keep an eye on me." Wife home tonight. Told to go to the ED for worsening headache, vomiting more than once, confusion, trouble waking, weakness or seizure; repeated these back. Thumb spica splint, worn at

all times. Recheck here booked Fri 09:00 (day 10); repeat scaphoid views ordered today for that visit. Told the first X-ray can miss this fracture, so the splint stays on until then; will call to rebook if he cannot come.

The rule and the patient's view

For the CT, the rewrite supplies what the chapter on the differential asks of any decision tool: its name, its inputs and whether the patient fell within its reach. Three details matter most for a test not ordered.

The population comes first. The Canadian CT Head Rule applies to patients aged sixteen or older with a minor head injury, meaning a witnessed loss of consciousness, definite amnesia or witnessed disorientation, with a Glasgow Coma Scale score of 13 to 15. The coworker's account placed Mr. Faber inside it, so the note gives that account a speaker and quotation marks. The rule was not built for patients taking an oral anticoagulant or with a bleeding disorder; Dolores Fitch, on apixaban in the chapter on the provider note, fell outside it. When a rule does not apply, say so, and give the clinical reasons for your decision instead.

The times come next. One criterion is the Glasgow Coma Scale score two hours after the injury, so the note needs both the time of the fall and the time of the exam. “GCS 15” with no time beside it does not let a reader apply the rule.

Then the output, stated no larger than it is. “CT not required by the rule” is not “no brain injury,” and the working diagnosis stays concussion. A rule that says a scan is unnecessary has not said that nothing is wrong. That is the honest uncertainty of the chapter on the working diagnosis, applied to a decision tool, and it is why the plan still tells Mr. Faber what would bring him back.

The plan records a choice that was shared. Mr. Faber was neither asking for a scan nor refusing one; he was weighing hours in an emergency department against an evening at home with someone watching, and his own words show which mattered to him. The three parts of a shared decision from that same chapter are all there, his choice recorded with his reason and the person who makes it safe. If Celeste had not offered the scan and he had asked for one, the encounter would belong to the chapter on the antibiotic or scan you decline.

The test that waits

The wrist shows a third kind of decision: a test that belongs to a later visit. A deferred test is a plan, and the record will ask of it what it asks of any plan: who will order it, when, and how the patient knows. Celeste's rewrite answers each in a line. She placed the order herself before he left, so the second X-ray does not depend on whoever sees him in ten days working out why he came. The recheck has a day and an hour. And Mr. Faber was told why, in words that also explain the splint.

When the deferred test belongs to someone else, the note has to carry the handoff. "Repeat imaging per PCP" gives the primary care clinician a task they may never hear about. Write who was told, by what means, and what the patient was asked to do. What to record when the follow-up never happens is worked in the chapter on missed visits and referrals that go nowhere.

A test you recommend and the patient declines is neither a deferral nor a shared choice. If Mr. Faber had refused the wrist X-ray, or a CT the rule called for, the note would need everything the chapter on informed refusal sets out. Writing "patient deferred imaging" would make a refusal look like a plan.

What the note must contain

- **The indication, or the reason not to test.** For a test ordered, the question it was meant to answer and the finding that raised it. For a test weighed and not ordered, the test by name and why it was not needed.
- **The decision rule and its inputs.** The rule by name, the findings that place the patient inside it, each criterion with its value, the times the rule depends on, and the output stated no larger than it is.
- **The patient discussion.** When the evidence left a real choice, the options, what each would cost, and the patient's preference in their own words.
- **The deferred plan.** For a test left for later, who orders it, when it happens, what the patient was told and what they will do if the date slips.

CHAPTER 9

Closing the Loop on the Abnormal Result

A potassium at five past five

At 17:05, in a primary care practice that closes at 17:30, the laboratory phones with a *critical value*, a result on its list of values too dangerous to wait in an inbox: potassium 6.6. Gina Calloway, the medical assistant at the front desk, reads it back and brings a slip of paper to Hannah Albright, the NP who ordered the test. No real practice, assistant or patient sits behind this call.

The result belongs to Walter Delgado, seventy-one, who has heart failure and takes lisinopril. A week ago Hannah added spironolactone, and at 09:40 on Tuesday the clinic drew a basic metabolic panel to check his potassium and kidney function. The same panel shows creatinine at 1.9, up from 1.3 in March, and the lab notes no hemolysis that would make the number doubtful.

Hannah calls his cell phone at 17:14 and gets voicemail. At 17:20 she reaches him at home. He feels "a little tired, otherwise fine," and asks whether he can come in tomorrow. She tells him to go to the emergency department now and to take neither spironolactone nor lisinopril until he hears from the clinic. Then she phones the department, and at 19:00 she calls back to check that he has arrived.

What the record will be asked

A reviewer follows a result along the timeline. It posts at a time the lab system stamps. Someone opens it, and many record systems log who did and when, in

the audit trail described in the chapter on templates and addenda. Then something happens, or nothing does. The questions follow the same path: who saw the result and when, what was done about it, and who was told, when and by what means.

The systems often answer the first question, and for a phoned critical value the lab usually logs the call. Only your note answers the other two. Opening a result, or clicking the button that marks it reviewed, records a look, not a decision, and a result opened and filed without a word shows that someone knew. Your organization's policy decides who owns a result; in many, it is the clinician whose name is on the order. The owner writes the step after the look.

One more question matters most when the chain breaks: what was the plan if the patient could not be reached? A voicemail is an attempt, not a notification, because nothing in it shows that the message was heard.

The note as written

At 17:45 Hannah opens a telephone encounter and writes:

> Lab called critical K 6.6. Pt notified, advised ED. Hold spironolactone.

The note is accurate and leaves out most of the chain. Nothing says who took the lab's call, when it reached Hannah or whether it was read back, so the minutes after 17:05 are blank on her side while the lab's log is not. “Pt notified” covers a voicemail and a second call without saying which one reached him. “Advised ED” could mean go now or go if you feel worse, and it hides both his wish to wait and his agreement not to. The department that was expecting him does not appear.

Suppose Mr. Delgado never reaches the emergency department. His wife may say he was told to go only if he felt worse, and the note cannot answer her. It never says why the result could not wait until morning, and it holds one drug where Hannah held two, so a reader will assume lisinopril continued.

The rewrite

The same call, link by link, from the lab to the hospital:

Telephone encounter. Lab called critical K 6.6 at 17:05 to Gina Calloway, MA, who read it back; given to me 17:10. Drawn 09:40 today. Same panel: creatinine 1.9 (1.3 in March). Lab reports no hemolysis. On lisinopril 20 mg; spironolactone 25 mg added 7 days ago.

17:14 called cell: no answer; voicemail asked him to call clinic, result not stated. 17:20 called home, reached patient, identity confirmed with date of birth. Feels "a little tired, otherwise fine." No palpitations, chest pain, shortness of breath or new weakness.

Assessment: hyperkalemia with rising creatinine, likely from spironolactone added to lisinopril. Needs ECG, repeat level and possible treatment tonight, beyond what this clinic can provide.

Plan: told him his potassium is high enough to disturb his heart rhythm and he needs the ED now. He asked, "Can't I come in tomorrow?" After the explanation: "All right. My wife will drive me now." Told to take no spironolactone or lisinopril until he hears from us. 17:35 called County Hospital ED, gave result and history to Dr. Imran Haddad, ED attending. Will call ED at 19:00 to confirm arrival; if he is not there, will call home and cell again, then his wife's cell listed in chart.

Addendum, 19:05: ED reports arrival 18:02, under evaluation.

Each link now has a name and a minute. The failed call belongs in the note: it shows the attempt and accounts for the minutes before Hannah reached him. The voicemail leaves out the result, because someone else may hear it; what a message may contain is set by your organization's policy. Mr. Delgado's first answer sits beside his second, so the record shows an instruction understood and accepted. And the call to Dr. Haddad hands the result to the clinician who acts on it next, before the patient walks in.

The plan's last line does the most work. Telling a patient where to go does not close the loop; it closes when the patient is in front of someone who can act. Hannah's plan says how she would check, and what she would do if he had not arrived, and the 19:05 addendum records the answer.

The finding nobody asked about

Simone Bergstrom is an NP on the hospital medicine service, and a fictional one. On a Monday she admits Frank Iverson, fifty-two, whose CT of the abdomen and pelvis shows sigmoid diverticulitis without abscess. The report's last line answers a question nobody asked: a 7 mm solid nodule in the right lower lobe, for which the radiologist recommends a chest CT in 6 to 12 months.

By Wednesday he is ready for home. Simone has told him about the nodule and messaged his primary care physician, but her draft discharge summary says only "Incidental RLL nodule, f/u PCP," a line that would read the same if neither had happened. As the chapter on the test you order and the test you don't showed, "per PCP" hands a task to someone who may never hear of it, and a finding nobody was waiting for is easier still to lose.

Whoever owns a scan owns its whole report, not only the answer to its question; here, that is Simone's team. Ownership passes to another clinician only when that clinician knows about the finding and accepts it, and the summary has to show that moment. Her rewrite moves the finding to the top:

> Incidental finding requiring follow-up: 7 mm solid nodule, right lower lobe, on CT abdomen/pelvis 6/9. Radiology recommends chest CT in 6–12 months. 6/11 09:30, EHR message to Dr. Ruth Lindqvist, PCP, naming the nodule and the recommended scan; her reply 11:05: "Got it, will order at his follow-up visit." Discussed with patient at bedside 11:40: told it is probably harmless but needs a repeat scan to be sure, and that Dr. Lindqvist will order it. He said: "So another scan in six months to a year, and she sets it up." Report copy given; finding listed in his discharge instructions.

The rewrite shows Mr. Iverson hearing about the nodule in plain words, with a reason not to panic and a reason not to forget, and his instructions carry it home. The bedside talk at 11:40 comes after the reply at 11:05, so the plan Simone describes to him is already agreed. Dr. Lindqvist's reply is the moment ownership changes hands. Without one, the note would say whom Simone called next. For a patient with no primary care clinician, it would name who will order the scan, and that may be the team that found it.

When the patient has already left

The third case brings its own cast of characters. At 11:00 on a Saturday, Joy Kessler, an urgent care NP, sees Ana Gutierrez, thirty-four, with burning on urination. She sends a urine culture, prescribes trimethoprim-sulfamethoxazole and writes the pending test into the plan: "Urine culture pending; clinic will call if treatment needs to change. Cell number confirmed." That is the deferred-test plan from the previous chapter, applied to a result.

On Monday at 14:20 the culture posts: *E. coli*, resistant to the drug she went home with. Joy is on shift, and she records each step as an addendum to the Saturday visit when she takes it:

> Addendum, Mon 14:30. Urine culture from Saturday: E. coli, resistant to trimethoprim-sulfamethoxazole (prescribed), sensitive to nitrofurantoin. Antibiotic needs changing. 14:35 called cell: no answer; voicemail asked her to call clinic, result not stated.
>
> Addendum, Mon 16:50. Second call, no answer. Portal message asking her to call. If not reached by Tue 12:00: call emergency contact on file, then letter per clinic policy.
>
> Addendum, Tue 09:10. Reached patient by phone, identity confirmed. Still burning; no fever, back pain or vomiting. Told culture shows the first antibiotic will not treat this infection. Nitrofurantoin sent to her pharmacy; told to stop trimethoprim-sulfamethoxazole. ED for fever, back or side pain, or vomiting. She said: "I'll pick it up at lunch."

The entries were written one at a time, and the system stamped each at the moment it describes. Together they show a result seen within ten minutes and pursued until it reached the patient the next morning. The middle entry holds the plan for a patient who cannot be reached, written before anyone knew it would be needed. How many attempts, by what means, and whether a letter goes by certified mail are set by your organization's policy; ask your risk manager. Had the culture posted on Joy's day off, the note would have to show who reviewed it instead, which belongs to the chapter on handoffs and cross-cover.

What the note must contain

- **The result and its reviewer.** The value, when it posted or was phoned in and to whom, and when you saw it.
- **The action.** What you decided about the result, including a decision to change nothing, with its reason.
- **Who was notified.** The person you reached, how you confirmed who they were, what they were told and what they said back; when another clinician takes over, that clinician by name and their reply.
- **The method and the time.** Phone, portal, bedside or letter, each attempt with its time, failed ones included.
- **The plan if the patient cannot be reached.** The next attempt, its method and its deadline, written before you know you will need it.

CHAPTER 10

The Delayed Scan and the Bed That Never Opens

A scanner out of service

One CT scanner serves the whole community hospital. At 09:25 on a Friday Bianca Ellison, an NP in its emergency department, sees Victor Langley, sixty-seven, who has had cramping abdominal pain since 06:00. He has vomited four times and has passed no stool or gas since the night before. In his thirties he had open surgery for a ruptured appendix. His abdomen is distended and tender all over, without guarding or rebound.

Bianca, drawn for this book along with her patient, suspects a small bowel obstruction. At 09:40 she orders blood work and a CT of the abdomen and pelvis with IV contrast, and starts IV fluids, an antiemetic and morphine. At 09:55 the CT technologist calls: the scanner is out of service, and the repair should be done by noon. It is back at 12:40.

The scan, read at 14:05, shows an obstruction without signs that the bowel is losing its blood supply, and the general surgeon recommends managing it without an operation. The hospitalist accepts Mr. Langley at 15:20, but there is no medical bed. In this hospital, by policy, the emergency clinician keeps writing orders for an admitted patient until a bed is assigned. His is assigned at 20:35. Until then he waits in her department with a tube draining his stomach, and Bianca returns to examine him through the evening.

What the record will be asked

The order and the scan each carry a timestamp, so the gap between them is on the timeline before anyone writes a word. The questions start there. What did you know about the delay, from whom and when, and what estimate were you given? What did you do in the meantime, for the condition you suspected rather than the one the scan later confirmed? And how often did you look again, and what did you find each time?

Behind them sits the standard-of-care question from the chapter on what a chart becomes after a bad outcome, asked now of the wait itself. Waiting is a decision whenever another path exists, such as a scanner elsewhere or a surgeon at the bedside before the images. A reviewer will ask whether a reasonable NP, knowing what you knew at each hour, would have kept waiting. *Boarding*, holding an admitted patient in the emergency department because no inpatient bed is free, adds one more question: who was responsible for him while he waited? Your organization's policy decides; the note says what it decided.

The note as written

At 20:40, with the bed assigned, Bianca writes:

> 67M abd pain, vomiting, prior abd surgery. CT delayed, scanner down. CT: SBO, no ischemia. Surgery saw, recs nonop mgmt, NGT. Admitted to hospitalist, boarding in ED awaiting bed. Pt resting comfortably, VSS. Continued to monitor.

More than eleven hours fit into one clause and two reassurances. “CT delayed, scanner down” does not say when Bianca learned it, what she was told or whether she looked for another way to get the scan. “Resting comfortably” and “VSS,” for vital signs stable, carry no time, and the nursing flowsheet shows a heart rate of 112 at 12:20; a reviewer will look for her response to that number. “Continued to monitor” gives no interval, no findings and no limit that would have ended the wait. Had his bowel perforated at 18:00, nothing here would show that anyone examined him between the scan and the bed.

The rewrite

In the rewrite, every step of the wait has its time, from the first exam to the bed:

67M, cramping abdominal pain since 06:00, vomiting x4, no stool or flatus since last night. Open surgery for ruptured appendix, age 34. Seen 09:25: T 98.9°F, HR 98, BP 138/84. Abdomen distended, diffusely tender, no guarding or rebound. Assessment: suspected small bowel obstruction, likely adhesive; compromised bowel not excluded. Plan: NPO, IV fluids, ondansetron, morphine; labs and CT abdomen/pelvis with IV contrast ordered 09:40.

09:55. CT technologist reports scanner out of service, repair expected by 12:00. Nearest other CT 40 min by ambulance; transfer for imaging would not be faster than the repair estimate. 10:10 Dr. Marisol Gaines, general surgery, told by phone of suspected SBO and CT delay; will see after CT, sooner if called. RN Theo Chandler to call me for HR above 110, systolic BP below 100, T 100.4°F or higher, worsening pain or more vomiting. Will call Dr. Gaines for guarding or rebound, constant pain or fever.

11:00. Pain 4/10 after morphine 4 mg IV at 10:05; no further vomiting. HR 92, BP 132/80. Abdomen unchanged. WBC 11.8, lactate 1.4. Continue to wait for CT.

12:25. Called by RN for HR 112, pain 8/10. Pain still cramping and intermittent. Abdomen unchanged, no guarding or rebound; T 99.1°F. Morphine 4 mg IV 12:30; at 12:50 HR 101, pain 4/10. Scanner back in service 12:40, one trauma scan ahead of him. Continue to wait.

14:05. CT: small bowel obstruction, transition point in right lower quadrant; no sign of ischemia, no free air. 14:40 seen by Dr. Gaines: nonoperative management, NG tube, surgery to follow. NG placed 14:55, 400 mL bilious output.

15:20. Accepted by Dr. Naomi Harrow, hospitalist. No medical bed; house supervisor: third in line, no estimate. Per hospital policy, ED orders and reassessment continue until a bed is assigned; RN call limits as at 09:55. Patient and wife told: admitted, no bed or estimate yet.

17:15. HR 88, pain 2/10. Abdomen softer, less distended. NG 300 mL since placement. Plan unchanged.

19:10. HR 86, pain 2/10. Abdomen soft, mildly tender. NG 150 mL since 17:15. Plan unchanged.

20:35. Bed assigned. 20:40 report by phone to Dr. Harrow: course, NG output, exam at 19:10.

Times, findings and decisions

Each reassessment has the same three parts: a time, the findings that bear on the danger named at 09:25, and a decision. The findings are chosen, not complete: at 17:15 Bianca records the pulse, the pain, the abdomen and the tube output, which are what would show the bowel getting worse. The decision is written even when it is to keep waiting, because "continue to wait" can be judged against the findings beside it, and "continued to monitor" cannot.

The 09:55 entry does the most work. It records the delay as a fact, with its source, its time and its estimate. It shows the alternative Bianca weighed and the surgeon who now knew. And it turns the sentence about what would change your mind, from the chapter on the working diagnosis, into limits set in advance, with a nurse to watch for them. At 12:25 the limits work as intended: the nurse calls when the pulse crosses one, Bianca comes, and the entry shows why she does not yet call the surgeon.

How often to look again is your clinical judgment, set by how fast the danger you named could move; a reviewer will set your entries beside the flowsheet and look at what happened between them. An entry written after the fact is labeled as the chapter on templates and the late addendum describes. "Scanner out of service, repair expected by 12:00" belongs to the timeline; a remark about radiology is commentary, which the chapter on facts and quotes keeps out of the chart.

The transfer that stalls

Another constructed case moves the wait to a forty-bed rural hospital, where Corinne Abbott, a former flight nurse, works as a hospital medicine NP. Elaine Burke, fifty-eight, was admitted on Sunday with right upper abdominal pain and jaundice. Her ultrasound shows a dilated bile duct, her bilirubin is rising, and the plan is an ERCP, an endoscopic procedure to clear the duct, which this hospital does not offer. By Monday evening the chart could say "Accepted at Regional, awaiting bed. Pt and family aware." Some of Corinne's entries show what that line would hide:

> 10:15. Transfer requested for ERCP, not available here. 10:50 accepted by Dr. Isaac Farrow, gastroenterology, Regional Medical Center, pending bed; no estimate. 13:40 Regional: no bed yet. University Hospital called as an alternative: no bed.
>
> 14:00. T 98.8°F, HR 84, BP 126/78, alert. RUQ tenderness unchanged. Bilirubin 4.1 (3.6 at 05:00). Waiting remains reasonable; reassess every 2 h. For fever, rigors, systolic BP below 100 or confusion: blood cultures, antibiotics, call Dr. Farrow about priority and air transport. RN to call me for any of these. If no bed by 20:00, will call both centers again.
>
> 16:30. Patient and daughter Megan Burke told: accepted, no bed yet, checks every 2 h; tell her nurse at once for fever, shaking chills, feeling faint or confusion. Megan asked, "What if she gets worse while she waits?" Told: antibiotics here and a call to Regional about moving her sooner.
>
> 18:20. Bed assigned. Ground ambulance earliest 22:00 per dispatch, crew on another transfer. 18:35 Dr. Farrow by phone: air transport not needed while stable. Patient told 18:45; she said, "Fine, as long as I get there tonight."

Acceptance is not a bed. Who accepted, when, and what the acceptance is pending are separate facts, and the bed gets its own time when it appears. Transport is a second delay, with its own source and estimate, and how she travels is a decision; Dr. Farrow's view is recorded as his, with the time he gave it.

The alternatives, a second center and air transport, each have an answer, so a reviewer asking whether there was a faster route finds that Corinne looked for one. If Mrs. Burke worsens on the road, Megan will be asked what the hospital told her, and the 16:30 entry is the record's side of that answer. The 14:00 entry also holds the plan for the bed that never opens: an hour at which Corinne stops waiting and calls again, written before anyone knows it will be needed, like the plan for the unreachable patient in the chapter on closing the loop on the abnormal result.

Sometimes the bed does not open before your shift ends. The last entry is then the handoff: its time, the clinician taking over, by name, and what you told them about the danger you named, the limits, the latest findings and when to look again. If Regional still had no bed at 20:00, Corinne's note would end with both calls and their answers, then her report to the night clinician, whom the nurse would now call. From that minute the record would show who was responsible for Mrs. Burke.

What the note must contain

- **The delay as a fact.** What is delayed, who told you and when, the estimate and each change to it, with no comment on the department or facility.
- **The interim plan and its owner.** What you did for the condition you suspected, and who was responsible for the patient while they waited.
- **The limits that end the wait.** The findings that would make you stop waiting, who watches for them and what you will do.
- **Each reassessment.** Its time, the findings that bear on the danger and the decision, including the decision to keep waiting.
- **For a transfer.** The accepting facility and clinician, when they accepted and what it is pending, transport and its estimate, the alternatives considered, what the patient and family were told, and when you will act if the bed does not open; and, if your shift ends first, the handoff: time, the clinician by name, what you told them.

CHAPTER 11

The Consultant Who Never Comes

Two pages and a cold foot

Gloria Ainsworth, seventy-three, was admitted on Tuesday with a urinary tract infection. She has peripheral artery disease and a stent in the main artery of her left thigh. At 13:50 on Thursday her nurse, Diego Fuentes, calls Jasmine Holloway, a former cardiac catheterization lab nurse now in her third month as a hospital medicine NP. The left foot is cold, and Mrs. Ainsworth says it has hurt since lunch. Everyone in this case is constructed, the surgeons included.

At 14:00 the foot is pale and cool to the ankle, with no pulse Jasmine can feel and only a faint signal on Doppler; sensation and movement are intact. She suspects the stent has closed, orders heparin and at 14:05 pages the vascular surgeon on call, Dr. Michael Kirby. No call comes back. At 14:20 she pages him again, and at 14:25 she calls Dr. Omar Bashir, the attending hospitalist on her service.

By 14:40 the toes are numb. Dr. Bashir learns from the operating room desk that Dr. Kirby is scrubbed in a case expected to finish around 16:30, and through the house supervisor he reaches the chief of surgery, Dr. Helen Eastman, who is at the bedside at 15:25. Dr. Kirby never comes; his only answer is a call from the operating room at 16:35, after the decision has been made without him, and he is told of Dr. Eastman's plan. Nobody need have failed for that to happen, and that is what makes the note hard to write.

What the record will be asked

If Mrs. Ainsworth loses her leg, a reviewer will spend longest on the eighty minutes between the first page and a surgeon at the bedside. Who was called, and when? What did each call say, and what did it ask? What came back, and what was done while nothing did? And when the silence went on, who decided it had lasted long enough, and what did they do?

These questions turn the treatment allegation from the chapter on where claims begin, "a consultant not called, a worsening patient not escalated," into a timeline. Asking for a consult does not hand the patient over: until the specialist arrives, her care rests with her own team, and the reviewer will look to their notes for the time in between. Paging systems often log each page with its time and number; only your note shows what you did when nobody answered.

The note as written

At 16:40, with Mrs. Ainsworth in the angiography suite, Jasmine writes:

> 73F PAD s/p L SFA stent, L foot cold and painful this afternoon, concern for acute limb ischemia. Heparin started. Vascular paged multiple times without response; attending aware. Escalated per policy. Eventually seen by surgery, to IR for angiogram.

Almost none of it can be checked. "Paged multiple times" gives no times and no content: a reader cannot tell how far apart the pages came or what they said. "Attending aware" hides that Dr. Bashir came to the bedside, examined the patient and made the next calls himself. "Escalated per policy" names no person and no time. And the numb toes at 14:40, the finding that made waiting unsafe, are missing, so the escalation has no reason on the page.

"Without response" and "eventually" are not findings but complaints. They set Jasmine's note against Dr. Kirby, who may answer that he was in an emergency operation and returned the page as soon as he could: the argument between clinicians that the chapter on facts and quotes warned against. "Eventually" also stands where 15:25 belongs, and the wait it hides could have lasted eighty minutes or five hours.

The rewrite

The same afternoon, written as it happened:

14:00. Called by RN Diego Fuentes 13:50 for cold, painful L foot. 73F admitted Tues with UTI; PAD, L superficial femoral artery stent 2 yrs ago. L foot pain since ~13:15, 7/10. L foot pale and cool to ankle, capillary refill 5 s; no palpable DP or PT pulse; Doppler: faint PT signal, no DP signal. R foot warm, pulses palpable. Light touch and toe movement intact L. Assessment: suspected acute L limb ischemia, possible stent occlusion. Plan: heparin per protocol, bolus 14:15; NPO; urgent vascular surgery consult. RN to check foot q15 min, call me at once for numbness, weakness or color change.

14:05. Paged Dr. Michael Kirby, vascular surgery on call per operator: "Urgent. 73F, 4 East bed 12, cold pulseless L foot since 13:15, prior SFA stent, heparin ordered. Need evaluation for revascularization. Call NP Holloway x4417."

14:20. No callback. Operator confirmed Dr. Kirby on call, same pager; second page, same text. 14:25 called Dr. Omar Bashir, attending hospitalist: gave exam, onset, heparin and both pages. He is coming to bedside.

14:40. Reassessed with Dr. Bashir, who examined her. New numbness L toes; light touch reduced over toes, movement intact. PT Doppler signal now absent. Worse than at 14:00; heparin running since 14:15. Decided with Dr. Bashir not to wait for a callback.

14:45. Dr. Bashir called OR front desk: Dr. Kirby scrubbed in a case, expected to finish ~16:30; message left with circulating RN. 14:50 Dr. Bashir called house supervisor Lydia Coates to reach a second vascular surgeon.

15:05. Dr. Helen Eastman, chief of surgery, by phone with Dr. Bashir and me; told of 14:00 exam and 14:40 change. Has not seen patient. Her recommendations, read back: continue heparin, keep NPO. Will be at bedside by 15:30.

15:25. Dr. Eastman at bedside, examined patient. Her plan: angiography with possible intervention now; consent obtained by her. To angiography 16:10.

16:35. Dr. Kirby returned page from OR; told of Dr. Eastman's plan. Plan unchanged.

The call log inside the note

Each call in the rewrite has four parts: the time, who was called and by what route, what was said, and what came back. A page can go astray, and one sent to the wrong number looks, from your end, exactly like a page ignored. “Operator confirmed Dr. Kirby on call, same pager” shows that Jasmine checked before she paged again.

Most notes leave out the text of the page, which does the most work. “Please call 4 East” gives whoever reads it no way to judge urgency. Jasmine's message gives the finding, its onset and the question, so it shows that the urgency traveled with the request. Not every paging system keeps the text, so put the words in your note.

Answers get the same care. “No callback” is an answer, written with the time you noted it. A phone recommendation is recorded as the consultant's, with name and time, read back and matched by your orders. The note also says whether the consultant has seen the patient, as the chapter on the provider note asked of an attending's role: at 15:05 Dr. Eastman's advice rests on Jasmine's report, and at 15:25 the plan is hers, made at the bedside.

Between the calls, the entries about Mrs. Ainsworth herself follow the method of the chapter on the delayed scan: a time, the findings that bear on the danger, a decision. Here each reassessment carries one more decision, whether to escalate. The 14:40 entry is that decision on the record, and it ties every call after it to a limb that was getting worse.

Escalation, step by step

The first step is the repeat page. How long to wait before it is a clinical judgment set by how fast the danger can move; some hospitals also set an expected response time for on-call consultants, so find out whether yours does. With the

repeat, try any other route the schedule lists, such as a backup number or the operating room desk.

The second step is the physician you work with. Whether Dr. Bashir is Jasmine's collaborating or supervising physician, or simply the attending on her service, depends on her state and her employer, as the chapter on the provider note explained. Many organizations expect you to call that physician when the finding worsens, when waiting could cost the patient something, or when the next step is one your policy reserves for a physician. The trigger is the patient, not a number of unanswered pages. On the record the call is a name, a time, what you told them and what they did.

The third step is the **chain of command**: the route your organization sets for taking a clinical concern past the people who have not resolved it. Who comes next differs between organizations; a common path runs through the house supervisor or the administrator on call to the chief of the consultant's department, then to the chief medical officer. Read your policy before you need it. Use it when the consultant cannot be reached in a time the patient can afford, or when a consultant's answer leaves the patient at risk and a direct conversation has not changed it. In the note each step is a role, a name, a time, what was asked and the answer.

Had Dr. Kirby called back at 14:30 and said he would see Mrs. Ainsworth after his last case, the note would carry that plan as his, with its time, then the numb toes at 14:40, the call telling him of them, his answer and what Jasmine and Dr. Bashir did next. If no one with the needed skill can come, the escalation becomes a transfer, worked in the chapter on the delayed scan and the bed that never opens.

A time and an action

The chapter on facts and quotes already settles the hardest line in this note: facts with times, and no account of anyone else's failure. An unanswered page is a time, a fact and your next step, as at 14:20. “Despite multiple pages,” “unresponsive,” “failed to call back” and “finally” turn the same facts into a complaint no reader can check against the log. The callback, when it comes, is written as plainly: the 16:35 entry has no “finally.” Leave out reasons you were not given, such as “probably in surgery”; one you were given goes in with its

source, as at 14:45. If the delay needs review, raise it with your manager or your organization's reporting channel, not in the chart.

What the note must contain

- **The trigger.** The finding that made the consult urgent, with its time and onset.
- **Each call.** The time, the person or role, the route, and the message or question as sent.
- **Each answer.** The reply or its absence, with the time and with no adjective or guessed reason; a phone recommendation as the consultant's, read back, with whether they had seen the patient.
- **The interim care.** What you did while waiting, and each reassessment with its findings and its decision, including the decision to escalate.
- **Each escalation step.** The physician you work with and each chain-of-command contact: name, role, time, what you told or asked them, and what they did.
- **The handover.** Who took over, when, and with what plan.

CHAPTER 12

Why This Drug, This Dose, This Patient

A fibula on a Saturday

Russell Eckert's X-ray shows a nondisplaced fracture of the distal fibula at the level of the joint, with the joint still aligned. He is fifty-three, and at 14:30 on a Saturday he missed the bottom step of a friend's deck and rolled his left ankle. Audrey Kincaid, who saw him at 16:20 in an urgent care clinic, spent nine years as a hospice nurse giving opioids that others had ordered; she has been an NP for a year. The drugs, doses and quantities below belong to a constructed patient. They show how to record a decision, not what to prescribe; check current labeling, your formulary and your state's rules.

Mr. Eckert takes sertraline for depression and sleeps with CPAP for obstructive sleep apnea. He drinks two or three beers on weekend evenings and lives with his wife, who drove him in, and their sixteen-year-old son. Ibuprofen at 16:30 brings his pain from 8 out of 10 to 6, and at 17:00 he asks: “Ibuprofen won't get me through the night. Can I get a Percocet?” At 17:05 Audrey searches the **prescription drug monitoring program**, or PDMP, the state's database of dispensed prescriptions for controlled drugs such as oxycodone. The search shows no controlled drug filled in the past year. Then she prescribes ten tablets of oxycodone 5 mg beside scheduled ibuprofen and acetaminophen, not the Percocet he named. She adds a naloxone nasal spray because of his sleep apnea, and books him with an orthopedist for Tuesday morning.

What the record will be asked

A prescription leaves a longer trail than most decisions: the order, the pharmacy's fill and, for a drug like oxycodone, a PDMP record, each with a time. None of them says why. Suppose that on the second night his wife cannot wake him, or his son finds the leftover tablets. A reviewer will ask the questions in this chapter's title: why a drug, and why this one over the alternatives; why this dose and quantity, for this many days; and why for this patient, given what you knew of his medicines, allergies, conditions and history.

A fourth question concerns the conversation: which risks did he hear, in what words, and what did he say back? Oxycodone adds a fifth, because it is a **controlled substance**, a drug that federal and state law regulate more tightly for its potential for dependence or misuse: which checks did the law or your organization's policy require, and did you run them before you prescribed? Like the test not ordered in the chapter on the test you order and the test you don't, a drug weighed and set aside leaves no trace unless the note gives it one.

The note as written

At 17:30 Audrey writes:

> 53M rolled L ankle, nondisplaced distal fibula fx on XR. Posterior splint, crutches, NWB. Pain 8/10. Rx oxycodone 5 mg q6h prn pain #10, ibuprofen, APAP. PDMP reviewed. NKDA. Risks/benefits discussed. F/u ortho.

No line shows a decision, and one is wrong: "NKDA" came from a prefilled field, the trap described in the chapter on templates, while the allergy list shows penicillin. "Rx oxycodone" gives no reason for an opioid at all, or for this one over the combination he asked for. "Prn pain" sets no threshold: any ache would qualify. "PDMP reviewed" has no time and no result.

"Risks/benefits discussed" is the inserted phrase from the same chapter. It names no risk, and none of the ones that mattered for this man appears: not his sleep apnea, his beer, his son or his sertraline. "F/u ortho" names no one and no day, so ten tablets have no end but the bottle. Nor does anything tie the drug to the danger it could hide.

The rewrite

The rewrite puts each decision beside its reason:

53M, rolled L ankle 14:30 missing bottom step of a deck. Seen 16:20. Lateral malleolus swollen, tender; foot warm, cap refill under 2 s, sensation and toe movement intact. XR: nondisplaced distal fibula fracture at level of joint, mortise intact. Posterior splint, crutches, non-weight-bearing. Pain 8/10; ibuprofen 600 mg 16:30, pain 6/10 at 17:00. Asked: "Ibuprofen won't get me through the night. Can I get a Percocet?"

Sertraline 100 mg daily, depression stable. OSA, CPAP nightly. 2–3 beers on weekend evenings, none today. No prior opioid prescriptions, substance use disorder or overdose; no benzodiazepines, sleep aids or other sedatives. Lives with wife and son, 16. Allergies: penicillin (hives); none to analgesics. PDMP 17:05: no controlled-substance prescriptions filled in past 12 months.

Assessment: acute pain from nondisplaced distal fibula fracture, likely severe for 2–3 days.

Plan: ibuprofen 600 mg every 6 h with food, up to 5 days, and acetaminophen 1,000 mg every 8 h, max 3,000 mg/day given alcohol use, both scheduled. Oxycodone 5 mg, 1 tab up to every 6 h, only for severe pain these do not relieve; #10, no refills, enough until ortho visit. Oxycodone alone, not the combination he asked for, so acetaminophen is counted in one place. Tramadol not used: serotonin syndrome and seizure risk with sertraline. Ibuprofen kept to 5 days: SSRI plus NSAID raises GI bleeding risk; no ulcer or GI bleed history.

Discussed with patient and wife, present at his request: oxycodone can cause drowsiness and slowed breathing, more so with sleep apnea or alcohol. No alcohol or driving while taking it; CPAP for any sleep, naps included; may constipate. Store tablets out of son's reach; leftovers to a drug take-back site. Naloxone nasal spray prescribed given OSA; use explained to both. He said: "Ibuprofen and Tylenol on the clock, the oxy only if the pain won't let me sleep, and no beer." To ED for pain the medicines no longer control, numbness, or pale, blue or cold toes: the splint can tighten as swelling rises. Ortho: Dr. Lena Jaffe, Tue 09:40, booked; she will decide on further pain treatment.

Dose, days and the next look

In the rewrite, a diagnosis with an expected course justifies a drug for severe pain; the scheduled drugs carry the base, and the as-needed dose has a threshold Mr. Eckert can recognize at two in the morning. The quantity is arithmetic anyone can check: ten tablets at up to four a day reach Tuesday's visit, where Dr. Jaffe makes the next decision. His request is quoted with no label, as the chapter on facts and quotes asks, and the change to it has its reason beside it. The last instruction ties the drug to what it could hide: in this scenario, pain the medicine no longer controls could mean a splint too tight or pressure building in the leg, a reason for Mr. Eckert to come back rather than take another tablet.

The checks a controlled substance adds

Federal law sorts controlled substances into schedules, and oxycodone is in Schedule II, the most restricted group that can be prescribed at all. Each check such a drug brings goes into the note as what you did, when, and what it showed. A timed PDMP result is a finding even when it is empty. When the report shows something, such as an opioid from another prescriber, record it as data, with what the patient said and what it changed, in the neutral words of the chapter on facts and quotes. If the database could not be reached, write that and what you did instead.

The history that matters is the history that could change the prescription: earlier opioids and how the patient did on them, substance use disorder or overdose, alcohol, other sedating drugs, breathing during sleep and who else lives in the home. Each line either changed the plan or cleared the way for it. Mr. Eckert's beer capped the acetaminophen and added a warning, his son put storage into the conversation, and the absence of sedatives and earlier opioids let a small dose stand.

A **controlled-substance agreement** sets out in writing the terms of ongoing prescribing, such as a single prescriber and pharmacy, how refills are requested and what monitoring to expect. Whether one is required, and for which prescriptions, depends on your state and your organization; Audrey's clinic requires none for a few days' supply after an injury. Where one applies, the note says it was discussed and signed. It records terms, not the conversation.

The rules behind these checks vary by state, so ask before your first controlled-substance prescription in a new job or a new state. Your state board can tell you which schedules your license lets you prescribe and on what terms, when a PDMP search is required, whether a first opioid prescription for acute pain is limited in days or quantity, and whether an agreement, a signed consent or an offer of naloxone is ever required. Your organization's policy may ask for more than the law, and your risk manager can tell you what it adds.

Allergies, interactions and the patient at higher risk

An allergy line shows it was considered when it names the reaction. “Penicillin (hives)” tells a reader what happened, and “none to analgesics” shows that the question was asked about the drugs actually prescribed. When you prescribe a drug related to a listed allergy, write the reaction, how long ago it happened and why you judged the risk acceptable. A side effect filed as an allergy, such as nausea from codeine, deserves the same care, because the reaction decides what is safe, not the label.

Clicking past an interaction alert records only that the alert appeared. The line a reviewer looks for names both drugs and what the pairing changed, as when Mr. Eckert's sertraline took tramadol off the list. An interaction you checked and found irrelevant needs no line. One you accepted needs its reason and its limit, like five days of ibuprofen in a man with no ulcer history.

A patient at higher risk is written the same way: the condition, then what it changed in the drug, dose, quantity, instructions or monitoring. Sleep apnea changed Mr. Eckert's instructions and added naloxone. For Dolores Fitch, on apixaban in the chapter on the provider note, the same fracture would have taken ibuprofen off the list, and her note would say why. A risk factor written in the history and never seen again in the plan reads as one nobody weighed.

What the note must contain

- **The indication.** The diagnosis and expected course that call for the drug, why this drug over the alternatives, and the dose, frequency and threshold for any as-needed dose.

- **The risks discussed.** The risks for this patient in the words used, the patient's reply, and instructions on alcohol, driving, storage and disposal where they apply.
- **The checks performed.** Allergies with their reactions, interactions with what they changed, the risk factors behind the plan, and for a controlled substance the PDMP result with its time, the relevant history and any agreement your state or policy requires.
- **The duration.** The quantity, the refills and the days they cover.
- **Who reassesses and when.** The clinician and visit that decide what comes next, and the return instructions for what the drug could hide.

CHAPTER 13

When You Say No to an Antibiotic or a Scan

An antibiotic request before a long drive

Last winter Scott Garrity had two colds like the one he has now, and another clinic gave him azithromycin for each. At 15:10 on a Thursday in February, with his nose blocked since Saturday, the forty-two-year-old sees Holly Bianchi, NP, at a family practice; both of them are fictional. His sore throat and cough have faded; the yellow discharge and pressure over both cheeks have not changed since Tuesday. He has had no fever.

His temperature is 99.0°F. His cheeks are mildly tender and his nasal lining swollen, with purulent discharge; his eyelids look normal, his eyes move without pain, and his neck is supple. At 15:25 he asks: “Every time I get this, it turns into a sinus infection. Can you just call in a Z-Pak? I'm driving to my mother's in Ohio on Saturday, and I'll be there a week.”

Holly does not call in the antibiotic. He leaves at 15:45 with a different plan, not entirely happy with it.

What the record will be asked

A declined request is a test or drug left out with a witness: the patient knows exactly what he asked for, and the chart shows it only if you write it down. Suppose that next Wednesday Mr. Garrity is in an Ohio emergency department with a swollen, red eyelid. A reviewer will ask three things of Thursday's note. Was the request heard: what did he ask for, and why? Was the reason explained:

what made an antibiotic unnecessary that afternoon, and what was he told? And what was offered instead: what did he leave with, and what would change the answer?

The same questions can come with no complication at all. A patient who leaves feeling dismissed can write to your board, and the complaint, as the chapter on where claims begin explained, needs no injury behind it.

The note as written

At 15:50 Holly writes:

> 42M sinus congestion x 6 days. Requesting azithromycin. Viral URI, abx not indicated. Educated on antibiotic stewardship. Supportive care. RTC PRN.

The request survives, but not the two things inside it that the rest of the visit answered: a belief that his colds always turn bacterial, and a week away from the clinic. “Abx not indicated” is a verdict with nothing underneath, like “No indication for CT head” in the chapter on the test you order and the test you don't. “Educated on antibiotic stewardship” says the conversation was about resistance in the community, not about him; read beside an Ohio emergency record, it says nothing about the risks of his own illness. “Supportive care” names no treatment, and “RTC PRN” gives no threshold. Nothing records his answer, so a reader cannot tell a patient who agreed from one who left unconvinced.

The rewrite

Holly could have written this instead:

> 42M, nasal congestion since Sat (day 6). Sore throat, cough days 1–2, resolved. Since Tue, thick yellow discharge and pressure over both cheeks, unchanged, not worse. No fever at home. Two similar illnesses last winter, azithromycin elsewhere both times. No drug allergies. Seen 15:10: T 99.0°F. Mild bilateral maxillary tenderness; nasal mucosa swollen, purulent discharge. No eyelid swelling or redness; eye movements full and painless. No severe headache; neck supple.

15:25, asked: “Every time I get this, it turns into a sinus infection. Can you just call in a Z-Pak? I'm driving to my mother's in Ohio on Saturday, and I'll be there a week.”

Assessment: acute rhinosinusitis, day 6, most likely viral. Bacterial sinusitis considered, IDSA criteria absent: under 10 days, no worsening after initial improvement, no high fever with severe symptoms at onset. Discolored discharge does not distinguish viral from bacterial. Spread to orbit or brain considered: no eye findings, no severe headache, neck supple.

Plan: no antibiotic started today. Explained: most likely a virus; an antibiotic would not shorten it and can cause diarrhea or a rash; usually improves within 10 days. Saline rinse twice daily, fluticasone nasal spray daily, ibuprofen or acetaminophen per label. Away 8 days from Sat, so delayed prescription: amoxicillin-clavulanate 875/125 mg twice daily for 5 days, #10, no refills, sent to his chosen pharmacy near his mother's home. Not azithromycin: not recommended for sinusitis because of resistance. Pick up and start only if no better at all by Mon (day 10), or worse after starting to improve: new fever, more face pain or discharge. If started, portal message to me. Same day to an ED for eyelid swelling or redness, pain moving an eye, double or blurred vision, severe headache, stiff neck, confusion, or T 102°F or higher. He said: “I'd still rather have it now. But if I'm not better by Monday, or it gets worse again, I pick up the pills and message you. Anything with my eye or a bad headache, I go in.”

The request, the reason and the answer

His words come first, quoted by the rules in the chapter on facts and quotes, because they hold what the plan answers. The line on discolored discharge answers his belief, and the delayed prescription answers his trip. Shrink the quotation to “requesting azithromycin” and both lines lose their reason.

The decision is written as Holly's, with a date on it: “no antibiotic started today.” “Antibiotics deferred” would hide that anything was asked for, and

“patient declined antibiotics” would put the choice in the wrong hands. “Today” keeps it a judgment about day 6 that a later day may revise.

The reason sits in the assessment, built as the chapter on the differential describes. It names the diagnosis the request aimed at and gives a finding for each criterion Holly applied, from the Infectious Diseases Society of America's guideline on acute bacterial rhinosinusitis; the dangerous diagnosis gets reasons of its own. The explanation is recorded as what Mr. Garrity heard about his own illness: what the drug would not do for him and what it could do to him. Resistance can be part of that conversation, but a reviewer who knows about the eyelid looks for this patient's risks.

What he leaves with instead

A no that ends the visit reads as a dismissal; a no followed by a plan reads as care. Symptomatic treatment, named and with its frequency, shows a man who left with something for what bothered him.

A *delayed prescription*, written today to be used only if something changes, is still a prescription, and the chapter on prescribing applies to it in full, down to why his drug differs from the one he named. It adds one element: what that something is. Holly's conditions are the bacterial criteria from her assessment, turned into a date and symptoms he can recognize: the sentence about what would change your mind, from the chapter on the working diagnosis, handed to the patient. A delayed prescription suits a patient who cannot easily come back; for one who can, a recheck on a set day may serve better. The portal message lets the record show whether and when he started.

The second list means be seen today, not start the pills: in this scenario, signs of an infection spreading toward the eye or brain call for an exam that day, not tablets at home. The note keeps the two lists apart, as the conversation did.

“I'd still rather have it now” stays in the note. A note showing a patient who disagreed, understood and left with a plan holds up better than one saying he agreed, because a complaint letter would confirm the first and contradict the second. If after the discussion you do prescribe, write the clinical reason: “per patient request” is a drug without an indication.

The scan he wanted now

Wendy Cho's visit is a hypothetical about a scan. She is a primary care NP, and at 09:30 on a Monday she sees Dale Henning, thirty-nine, whose lower back has hurt since he lifted a box at work on Thursday; the pain reaches his right buttock, not his leg. He wants an MRI and, if it shows a disc, a spine surgeon. Part of her note reads:

> 09:40, asked: "My brother waited on his back and ended up needing surgery. I want to know now." No fever, weight loss, cancer history, injection drug or steroid use; no numbness between the legs, bladder or bowel change, or leg weakness. Strength, reflexes and sensation normal both legs; straight-leg raise negative. Assessment: acute low back strain; nerve root compression, cauda equina syndrome, infection and cancer considered, not supported by findings above. MRI and surgical referral not ordered today: neither would change the plan; disc changes are common on MRI in people without back pain. Accepted: physical therapy referral, light-duty note for 2 weeks, heat, keep walking. Recheck booked in 4 weeks; MRI reconsidered then if not improving, sooner for pain or numbness spreading down a leg or leg weakness. ED at once for numbness between the legs, trouble passing urine, or loss of bladder or bowel control. He said: "If my leg goes weak or I can't pee right, I go in. Otherwise we talk MRI in a month."

A request for a scan often carries a fear, and Mr. Henning's has a name: his brother's surgery. The reason not to scan is what it could add for this patient now: nothing the plan would act on. The fear gets its own answer: the signs that would bring the scan forward, which he can watch for himself, and a date when it comes up again. That date makes the no a deferral with an owner, as the chapter on the test you order and the test you don't asks of any test that waits. The referral is declined on the same grounds, and what he accepted is written as accepted: a patient kept in care, not sent away.

What the note must contain

- **The request.** The patient's words and the concern behind them.

- **The decision.** What you did not prescribe or order, written as your clinical choice, with its date.
- **The clinical reason.** The diagnosis the request aimed at, with the findings that set it aside, and the dangerous alternative with its own.
- **The explanation and the reply.** What the patient heard about their own illness, and their answer, disagreement included.
- **What was offered instead.** Symptom relief, a delayed prescription, a referral or recheck, and what was accepted.
- **The signs that change the answer.** Those that mean start the delayed plan, and those that mean be seen today.
- **The way back.** Who reassesses and when, and how the record will learn the delayed plan was used.

CHAPTER 14

Hostile Encounters in Neutral Words

A son who wants a hundred percent

"He can't breathe. I want him on a hundred percent." Craig Lowell flew in on Saturday afternoon to see his father, Eugene, seventy-four, admitted on Friday night with an exacerbation of COPD, and he wants the oxygen turned up. His father's blood gas showed carbon dioxide building up, so the orders set an oxygen saturation target of 88 to 92 percent. At 18:30 Rebecca Dorsey, the hospital medicine NP, is called to the room by the nurse, Hector Avila.

The scene, like everyone in it, is imagined. Rebecca examines Mr. Lowell, who says he feels about the same as that morning, and explains the target. Craig swears, asks for "a real doctor, not a nurse," and at 18:50 slaps the overbed table and says he will be waiting for her in the parking lot. Hector calls security, and Craig leaves the unit with an officer at 19:00.

What the record will be asked

If Mr. Lowell is moved to intensive care at 03:00, and his son says he warned the staff and nobody listened, a reviewer will ask three things of the evening. What happened: what did the son report, and what did the clinician find when she looked? What was said, and by whom? And were the decisions driven by clinical facts? Pressure can bend care either way, toward oxygen turned up to end an argument or toward a real change dismissed because of who reported it. The note has to show that neither happened.

The note as written

At 19:20 Rebecca writes:

> Called to bedside re: son demanding O2 be increased. Son extremely hostile and aggressive, cursing at staff, refusing to listen to medical explanation. Explained multiple times that pt is a CO2 retainer. Son demanded "a real doctor." Became threatening; security called, son escorted off unit. Pt stable on 2 L. Incident report filed. Continue current O2.

The note is about the son. The patient appears twice, as "a CO2 retainer" and as "stable on 2 L." The second is the only clinical answer to the complaint, and it has no saturation, no breathing rate and no exam, nothing to show whether Rebecca looked at Mr. Lowell or at the monitor. "Hostile," "aggressive" and "refusing to listen" are verdicts that the chapter on facts and quotes replaces with what was seen and heard. "Became threatening" leaves out the words, so nobody can judge whether calling security was a measured response. The request for a physician is quoted, and nothing shows it was answered. And the incident report is named in the chart, which that chapter ruled out.

The rewrite

Mr. Lowell belongs at the center of the note:

> 18:35. Seen at request of RN Hector Avila. Sitting up, full sentences. SpO2 90% on 2 L NC, RR 22, HR 94. Scattered expiratory wheeze, no accessory muscle use; unchanged from my 08:15 exam. Alert, oriented, answers promptly. Patient: "I'm breathing about like this morning." Admission ABG Fri 21:40: pH 7.33, PaCO2 58; SpO2 target 88–92% ordered.
>
> Son, Craig Lowell, at bedside, speaking loudly; swore several times. Asked: "He can't breathe. I want him on a hundred percent." With patient's permission, explained to both: oxygen kept in this range because more can raise his carbon dioxide and make him drowsy or confused; level in range, breathing unchanged since morning. Son asked for "a real doctor, not a nurse." 18:42 called Dr. Philip Greer, attending: gave exam and request. Agrees with current target; will see

patient and speak with son at 19:30.

18:50. Son struck overbed table with his palm and said to me: “If he dies tonight, I'll be waiting for you in the parking lot.” Security called by RN; son left unit with officer 19:00. Patient: “He's scared. But I don't want yelling in here.” Asks that Dr. Greer update son by phone tonight.

19:15. SpO2 89%, RR 20, HR 90; resting, alert. Assessment: COPD exacerbation, day 2, unchanged; oxygen within target. Plan unchanged: 2 L NC, target 88–92%. RN to call me for SpO2 below 88%, RR above 28, drowsiness or new confusion.

The rewrite treats the son's alarm as a report about a patient and answers it with an exam. That is the line a reviewer will look for first, and the confrontation made it the easiest one to skip. The comparison with the morning exam says what “stable” only claimed. The reason for the target sits beside the blood gas that set it, so the unchanged oxygen reads as a clinical decision, built as the chapter on the antibiotic or scan you decline describes.

The request for a physician is answered like any request: who was called, when, and what came of it. The note quotes the remark and says nothing about it. And Mr. Lowell, alert and answering, speaks for himself.

Threats, security and the behavior agreement

The 18:50 entry is the easiest to get wrong. Without it, the chart shows a worried son removed from his sick father's room for no stated reason. Inflated, it fails as the weak note did. The threat goes in once, in its exact words, because here the words are the fact. Beside it goes what it changed for the patient: who left, when, and what Mr. Lowell asked for afterward.

Everything else goes through your organization's channels. The security report, the incident report and a workplace violence report, where one exists, hold the full account, from the officers' names to any injury. An escort to your car, and how shaken you were, are for your manager and security. A complaint from the son, if one comes, is answered through patient relations or risk management, never in the chart.

On Sunday the nurse manager and security meet with Craig and set up a **behavior agreement**. This is a written statement, made with a patient or visitor whose conduct has disrupted care, of what is expected from now on and what follows if it is not met. Who writes it and where it is filed are set by your organization's policy. Your note records what it changes about care, such as who visits and who receives updates, and that Mr. Lowell agreed: "Per patient's wishes, Dr. Greer updates son by phone daily at 11:00."

Then let the episode end. If "son hostile, security involved" is copied into each progress note, it becomes a label that travels, of the kind the chapter on facts and quotes described, ready to explain whatever happens next. If your organization uses a safety flag in the chart, the warning lives there. And the care does not change: the reassessments after 18:50 should come as often as those before.

The request that keeps coming back

Neil Carver, fifty-one, has type 2 diabetes, a cold now in its third day and one request: a steroid shot. Gemma Lockhart, an NP at a made-up urgent care clinic, declines, with reasons written as the chapter on the antibiotic or scan you decline describes. He asks again ten minutes later, then a third time, louder, standing by the door. Her first draft argues back:

> Pt repeatedly demanding steroid injection despite multiple explanations. Became argumentative. I spent 20 minutes patiently explaining why it is not indicated. Pt would not accept plan.

"Repeatedly" and "despite" turn a conversation into a contest. "Patiently" grades Gemma's own manner, and a reader told that a clinician was patient wonders why it needed saying. "Would not accept" is a verdict, and the answer she gave, the part a reviewer needs, is missing. Her rewrite keeps the sequence and drops the commentary:

> 10:20 asked for steroid injection: "I always get one for this." Not given today: viral URI, day 3. Explained a steroid would not shorten a cold and would raise his blood sugar for several days. Offered saline rinse, acetaminophen; recheck if

> not improving by day 10. 10:30 asked again; same answer. 10:40, standing, voice raised: "I'm not leaving without that shot." Answer unchanged; offered review by the physician on site, declined. Left 10:45 with written instructions.

The timestamps count the requests, and one observed behavior and one quotation carry the escalation. The clinical reason appears once, where it was first given. Repeating it at each request would make the note longer without making it stronger. The offered second opinion, with his answer, shows a decision open to review rather than a personal refusal.

When the disagreement is with a colleague

The last case, pure fiction, puts the disagreement inside the team. In an emergency department, NP Natalie Corbin sees Lorraine Abernathy, sixty-three, who fainted at her kitchen sink. Natalie would admit her for cardiac monitoring; Dr. Gordon Hale, the attending supervising her shift, thinks the faint was orthostatic. The tempting line, "Dr. Hale overruled my recommendation to admit," charts the conflict and leaves out every clinical reason on both sides. Part of her note, written as clinical content, reads:

> 15:40. Syncope, discussed with Dr. Gordon Hale, ED attending, 15:20. Admission for telemetry considered: no warning before the faint; ECG first-degree AV block, not present on 2022 ECG. Dr. Hale examined patient 15:30. His assessment: orthostatic syncope; BP 134/80 lying, 108/70 standing, with lightheadedness; hydrochlorothiazide started 12 days ago. His plan: discharge, stop hydrochlorothiazide, ambulatory heart monitor, cardiology clinic within 1 week.

The option Natalie favored stays in the note as an option considered, with her findings behind it. The decision appears with its owner and his reasons, as the chapter on the provider note asks of any physician's role. Nothing records tone or "against my advice." A disagreement with a nurse or a consultant is written the same way: each of you records what you saw and did, and neither note answers the other.

If, having heard the reasons, you still believe a plan leaves the patient at risk, the route is the chain of command, with its steps recorded as in the chapter on the consultant who never comes. A concern about a colleague's conduct, rather than about this patient's care, goes to your manager or to peer review. And an addendum written days later to put your objection on the record reads the way the chapter on templates and the late addendum warned: like a record built for an argument.

What the note must contain

- **The report and your look.** What was raised, by whom, and your exam or review in response, with times.
- **Each request and your answer.** The request in the person's words, and the clinical reason, given once.
- **The behavior, observed.** What was said and done, without adjectives, and a threat in its exact words; the rest of any incident goes in your organization's reports.
- **What changed for the patient.** Security, visiting and updates, and the patient's own wishes.
- **The physician asked for.** Who was called, when, and what they did.
- **For a disagreement.** The option considered, the findings behind it, the decision and its owner, and any escalation; never the conflict.

PART III

Choices, Continuity, and Aftermath

CHAPTER 15

The Capacity Question, Decision by Decision

A no at 16:40

Albert Yates, eighty-four, was admitted on Tuesday at 10:30 after three days of black stools. On admission he was oriented and conversant, and gastroenterology recommended an upper endoscopy, or EGD, for Wednesday at 09:00. His NP is Tessa Novak, a former geriatric nurse. He, his family and everyone caring for him exist only on these pages.

At 16:40 a GI fellow brings the consent form. Mr. Yates pushes it away: "Nobody's doing anything to me." Since 15:30 his nurse, Imani Shaw, has charted that he is pulling at his IV line and calling her by his late wife's name. The fellow pages Tessa with one question: can he refuse?

At 17:05 Tessa finds him drowsy and certain he is waiting at a bus station. She explains the procedure in plain words; two minutes later he asks, "What test?" His vital signs are steady, and nothing has to be decided tonight. At 07:35 Wednesday she goes back. He is clear-headed, and he still says no.

What the record will be asked

If Mr. Yates bleeds again on Thursday, the chart will be asked four things. Capacity for which decision: this endoscopy, not his ability to run his life. Assessed how: what he was told, what he was asked and what he answered.

Assessed by whom: which clinician, and who else confirmed or disputed the finding. And when: the time of each assessment, set against a mental state that changed by the hour. The last is the sharpest: readers who already know the ending, as the chapter on what a chart becomes after a bad outcome explained, will lay a confused evening beside a refusal accepted the next morning and ask what changed overnight.

Capacity, not competence

Decision-making capacity is a clinical judgment that a patient can make one particular decision at one particular time. The treating clinician makes it at the bedside, and it can change by the afternoon. **Competence** is a legal status decided by a court, which may appoint a **guardian** (called a conservator in some states) to make some or all decisions for a person it finds incompetent. The words are often used loosely, but on the record the difference matters: "lacks capacity" with no decision attached reads as a verdict on the whole person.

Adults are presumed to have capacity, so the finding of incapacity is the one that needs support. Neither a diagnosis nor an orientation check supplies it: a man with delirium can know the date and still not grasp that the bleeding is his. A choice you think unwise is not evidence either, though a serious refusal is a reason to look closely.

The assessment rests on four abilities, each judged against the decision at hand. *Understanding* is taking in what is proposed, why, its main risks and the alternatives, and giving it back in one's own words. *Appreciation* is accepting that this applies to oneself: this illness, these risks. *Reasoning* is weighing the options against each other and against what matters to the patient, and reaching a choice that follows. *Expressing a choice* is stating one that holds steady long enough to act on.

The note as written

Tessa's two entries, written between pages, read like this:

> 17:20. Pt refusing EGD. Confused, A&O x1. Lacks capacity. Son notified. Will readdress in AM.

08:05. Pt A&O x3 today, has capacity. Continues to refuse EGD. Risks explained. GI aware.

Orientation carries both findings, and orientation is not one of the four abilities. Neither entry says what Mr. Yates was told or records a word he said. Read together, they show a clinician who found him incapable on Tuesday and capable on Wednesday, with nothing but the clock to explain the change. If he bleeds on Thursday, a plaintiff's attorney will supply the explanation: a confused man said no twice, and the second time nobody looked closely.

The rewrite

Rewritten, the two entries hold nothing beyond what Tessa asked, heard or did:

17:05. Capacity assessment for one decision: EGD recommended by GI for 09:00 tomorrow, declined when consent sought at 16:40. Oriented and conversant on admission at 10:30; per RN I. Shaw, since 15:30 pulling at IV and calling her by his late wife's name. Now drowsy, says he is "at the bus station." Explained purpose and main risks of EGD in plain words. Understanding: could not say why he is in the hospital; 2 min later asked, "What test?" Appreciation: "I'm not sick. I need to get home." Reasoning: gave no reason related to the procedure. Choice: says no each time asked. Assessment: lacks capacity at this time for this decision, due to fluctuating delirium. Decision can wait: HR 84, BP 128/72, no melena since 12:00, Hb 8.4 at 16:00. Reassess 07:30, before procedure. Son Paul Yates, his health care agent per document in chart, updated by phone 17:30; agrees to wait for morning reassessment.

07:35. Capacity reassessment, same decision. Oriented to place, date and reason for admission; recalls "the young doctor with the papers." Explained again. Understanding: "You put a camera down my throat to find the bleeding and maybe stop it. They put me under, and it can tear something." Appreciation: "I've had black stools since Sunday and my blood's low. It could stop by itself, or it could get worse." Reasoning: "My wife choked on one of those tubes and was never right after. If I bleed more, you can give me blood, and I can always change

> my mind." Choice: declines EGD; unchanged at end of discussion. Assessment: has capacity at this time to decline EGD; deficits found at 17:05 not present. Son, by phone 07:50 with patient's permission: father has refused "any tube down his throat" since his wife's death. Will reassess if mental status changes.

Each entry opens with the decision and gives each ability its own line, in his words wherever he had any, quoted by the rules in the chapter on facts and quotes. The evening deficits are specific enough that their absence in the morning means something: the reader sees a patient who changed, not a clinician who changed her mind. Tessa may not share the morning reasoning, and she does not need to: the choice follows from facts he understood and a value he has long held. The son's call supports that history without standing in for his answers.

Asking, and asking again

Those answers exist because of how the questions were put. "Do you understand?" invites a yes that proves nothing. Open questions get answers you can quote: *Tell me in your own words what we are suggesting. What do you think is happening to you? What could happen if you don't have it? How did you decide?* Use hearing aids, glasses and a professional interpreter as needed, so the deficit you record is the patient's, not the room's. When an answer is wrong, write the wrong answer: "What test?" tells a later reader more than "poor understanding."

Fluctuation changes the timing, not the method. When a decision can wait, assess at the patient's best hour and write why waiting was safe. Each reassessment is a new entry with its own time, naming the decision again and saying what changed. A finding of capacity at 07:35 does not stretch to a different answer given at 21:00, after the confusion returns; a finding of incapacity at 17:05 does not bind the morning. If you write one up hours later, label it as the chapter on templates and the late addendum describes.

The surrogate and their choice

Suppose that at 07:35 he had still asked "What test?" while his hemoglobin kept falling. The decision could not wait for a better hour, and it would pass to a

surrogate decision-maker: the person who decides for a patient who lacks capacity. The note has to show three things about that person.

The first is who they are and where their authority comes from. A **health care power of attorney**, called a health care proxy in some states, is a document in which a patient names a **health care agent** to decide if they cannot; Mr. Yates named his son in one. A court-appointed guardian, or conservator in some states, is another source. If no one was named, state law, where it addresses the question, and your organization's policy set who may decide. Some documents and some laws also say who must certify incapacity before an agent's authority begins, so ask your risk manager whether your finding is enough or a physician must make it.

The second is what they were told: the same information the patient would have received. The third is what they chose and on what basis. A surrogate is generally expected to decide as the patient would have, from the patient's known wishes and values, a standard called **substituted judgment**; when those wishes cannot be known, the surrogate decides by the patient's **best interests**. The exact rule depends on state law. Either way, record the basis in the surrogate's words, including how they know what the patient wanted: "He told me after Mom died he'd never let them put a tube down his throat."

Psychiatry and the ethics committee

Most capacity assessments are made by the treating clinician. Many clinicians ask psychiatry when the question is hard: when a psychiatric illness such as severe depression or a psychotic disorder may be distorting appreciation, when your findings are borderline and the stakes high, or when someone disputes your conclusion. Put the decision in the consult request, "Does Mr. Yates have capacity to refuse EGD?" rather than "capacity eval." Record the question, the consultant's findings and what you did with them; the consult informs your decision without taking it off your hands.

The ethics committee answers a different question: not whether the patient can decide, but what to do when the people deciding disagree, or when nobody can. Common reasons are a surrogate whose choice seems to contradict the patient's known wishes, relatives who disagree, and a patient without capacity and without a surrogate. In most hospitals the committee advises rather than

decides. Record when you asked, who took part, what was recommended and what followed.

What the note must contain

- **The question.** The decision assessed, named specifically, and the time of the assessment.
- **The information.** What the patient was told, and any interpreter or sensory aid used.
- **The assessment.** Each of the four abilities, with the patient's own answers, right or wrong.
- **The conclusion.** A finding limited to this decision at this time, with its likely cause.
- **The timing.** Whether the decision could wait; each reassessment, timed, with what changed.
- **The clinicians.** Who assessed, and any psychiatry or ethics consultation: question, answer, action.
- **The surrogate.** For a surrogate: name, relationship, source of authority, what they were told, what they chose and why.

CHAPTER 16

Informed Refusal and the Partial Yes

Antibiotics yes, needle no

Light hurts Derek Sutton's eyes, and his neck is stiff. He is thirty-eight and has had fever and a headache since the previous evening. Carmen Vasquez, one of the NPs in the emergency department of a county hospital, sees him at 13:25 on a Thursday. His temperature is 102.6°F and his heart rate 112. He is alert and oriented, with no rash and no focal deficit.

Mr. Sutton is an illustration, not a case report. Carmen thinks first of bacterial meningitis and goes straight to Dr. Joanne Whitaker, the ED attending. Blood cultures are drawn at 13:40, and by 13:50 ceftriaxone and vancomycin are running, dexamethasone given with the first dose. Dr. Whitaker will do the lumbar puncture, or LP, and at 14:05 Carmen goes back to explain it. Mr. Sutton hears her out, then shakes his head. “I'll take the antibiotics, and I'll stay as long as you want. But nobody's putting a needle in my back.”

She talks it through with him and his wife for fifteen minutes, and Dr. Whitaker joins them at 14:10. He does not change his mind. At 14:30 he signs the hospital's refusal form, and at 14:40 Carmen calls the admitting hospitalist.

What the record will be asked

Informed consent is the process in which a clinician explains a proposed test or treatment, its purpose, its main risks and benefits and the reasonable alternatives, and a patient with capacity agrees. **Informed refusal** is the same process ending

in no. It puts at the center an element that consent often treats in passing: what could happen if the patient says no. What must be disclosed, and by what legal standard, varies by state; ask your risk manager what applies where you work.

Suppose that on Saturday Mr. Sutton is worse, and the cause turns out to be an infection the antibiotics do not treat. A reviewer will bring one question to the 14:05 conversation, in two parts: did he understand what he was refusing, and did he understand what could happen as a result? Three more questions make the answer believable. Could he make this decision? What alternatives did he hear? And once he said no, what did his clinicians do with the plan that was left?

A signature is not a conversation

Many organizations have a refusal form, and some policies require it; use it when yours does. It holds a name, a date, a signature and a printed paragraph of risks that fits every patient and every test. It cannot show what Mr. Sutton was told about this test, in what words, or what he said back. It cannot show whether he could make the decision, which alternatives he was offered or what happened next. A signature proves that a pen touched paper, much as an order, in the chapter on what a chart becomes after a bad outcome, proved only that someone touched a keyboard.

The form records a signature; the note records the conversation. A form signed in thirty seconds with nothing explained looks exactly like one signed after fifteen minutes of discussion, and only the note tells them apart. If a patient declines to sign, write that, and who was present. The refusal is no less real, because the record of it was never the form.

The note as written

Carmen's note, signed at 14:50, runs to five short sentences:

> 14:50. Pt refuses LP. Risks explained, pt verbalized understanding. Refusal form signed. Abx given. Admit to medicine.

It answers neither half of the question. "Risks explained" does not say which risks: those of the needle, or those of going without it. "Pt verbalized understanding" is a conclusion with nothing inside it, the written version of the "Do you understand?" that the chapter on the capacity question warned against. There is no capacity finding and no alternative. "Refuses," standing alone, reads as obstinacy, and the reason he gave is gone. Nor does the plan say how the refusal changed it, or whether anyone means to ask him again.

The rewrite

Signed at 14:50, the note can carry the whole conversation and the next offer:

> 14:50. Lumbar puncture declined; rest of plan accepted. Suspected bacterial meningitis (history and exam above). Blood cultures 13:40; ceftriaxone and vancomycin started 13:50, dexamethasone with the first dose. LP recommended to confirm the diagnosis, identify the organism and guide the choice and length of treatment; Dr. J. Whitaker, ED attending, to perform.
>
> Discussed 14:05–14:20 with patient and his wife, present at his request; Dr. Whitaker from 14:10. Explained in plain words: without the LP we may not learn whether this is bacterial, viral or something else, and could miss an infection that needs a different drug; he may stay on IV antibiotics longer than he needs; the longer antibiotics run, the less likely a later LP is to identify the germ. Risks of LP: headache afterward, sore back, rarely bleeding or infection; the needle goes in below where the spinal cord ends. Options offered: LP now, with medication for anxiety and extra numbing; LP later, knowing it will show less; no LP, with admission and a full course of IV antibiotics as if bacterial.
>
> Patient, 14:05: "I'll take the antibiotics, and I'll stay as long as you want. But nobody's putting a needle in my back." His reason: "My brother-in-law had one, and his back's never been right." Asked what could happen without it: "You can't be sure what bug it is, so I'm on the strong stuff longer, and you might miss something else." Capacity for this decision: alert, oriented; restated purpose and consequences in his own words, applies them to himself, reason consistent, choice unchanged throughout. Has capacity at this time to decline LP.

Plan: admit to medicine; IV antibiotics as above, now a full course. LP order cancelled, reason "declined by patient." Told he may change his mind at any time; RN Luis Navarro to page if he does. Capacity reassessed if confused. Refusal form signed 14:30. 14:40, Dr. Anita Rhodes, admitting hospitalist, told of the refusal and his reason; she will offer the LP again on admission.

What the conversation has to show

Four elements carry the rewrite. First come the risks, in the words Carmen used. "Risks explained" could cover anything; "could miss an infection that needs a different drug" is the risk a reviewer will test if Saturday goes badly, and the note shows that Mr. Sutton heard it before anyone knew it mattered. Write the risks of refusing for this patient and this test, the most serious first, and the risks of the procedure itself, since a patient deciding about a needle is weighing both.

The alternatives show that the no answered a real offer. Medication for anxiety, extra numbing, an LP later: each was a way the test could still have happened, and each one he declined makes the refusal more clearly his. Going without belongs on the list too, with what replaces the test, because that is the plan he chose.

The patient's own words carry two things: his reason and his account of the consequences. The reason explains a decision that would otherwise look arbitrary. The account is what "verbalized understanding" leaves out, and it answers the second half of the reviewer's question in his voice rather than yours. Quote both by the rules in the chapter on facts and quotes.

The capacity line is short because the method lives elsewhere. The chapter on the capacity question shows how to assess the four abilities for one decision; the refusal note names the decision, states the finding and points to the patient's answers that support it. In Carmen's note they are his account of what could happen without the LP and a choice that held for the whole fifteen minutes. Her "at this time" is literal: if he becomes confused, capacity is reassessed. A bare "has capacity" would not carry a refusal this consequential. When capacity is in doubt, settle it first: a refusal is the patient's only if they could make it.

The partial yes

Mr. Sutton's no was partial, and many refusals are. A patient accepts the admission and declines the procedure, or takes the pills and declines the injection. The note keeps three things apart. The part accepted comes first, so that the no is not read as a refusal of care. The part refused is named exactly and as a refusal: "LP declined," not "LP deferred," which makes a refusal look like a plan. And the plan that remains says what the refusal changed. Without the LP, the antibiotics are no longer a bridge to a result; they are the treatment for a diagnosis that may never be confirmed, and the admitting team needs to know it.

The orders have to agree with the note. An LP order left open after the refusal sits on the timeline as a procedure nobody performed, like the result nobody acted on in the chapter on closing the loop. Carmen cancels it with a reason and hands the refusal, with the plan to ask again, to the next clinician by name.

A refusal also ages. The chapter's other illustration shows how: Isabel Torres, a hospital medicine NP, cares for Martin Zielinski, seventy-six, admitted with pneumonia. After his first injection of heparin to prevent blood clots, he declines the rest: "Those shots leave bruises all over me." On the morning of day three, Isabel finds four doses marked refused in the medication record and no provider note that mentions them. Read after a clot, those entries would describe refusals nobody heard.

Her note that morning answers them: what he declined and why; that she told him a clot could form in his leg and travel to his lungs while he spends most of the day in bed; what he accepted instead, inflatable compression sleeves and walks in the hall, which she told him protect less; and that she will offer the injections again each morning and record each answer with its time. A refusal written once and never revisited reads as a decision the clinicians stopped thinking about.

What the note must contain

- **The recommendation.** What was proposed, why, who would do it and who was present.

- **The risks in plain words.** The risks of refusing for this patient, the most serious first, and those of the test or treatment itself.
- **The alternatives.** Each option offered, including going without and what would replace it.
- **The patient's own words.** The reason for the refusal and the patient's account of what could happen.
- **The capacity finding.** For this decision at this time, with the answers behind it.
- **The partial yes.** What was accepted, what was refused and what the refusal changed in the plan.
- **The next offer.** That the patient may change their mind, who will ask again and when, and orders updated to match.
- **The form.** Signed or declined, never in place of the conversation.

CHAPTER 17

The Patient Leaving Against Medical Advice

A bag packed at 14:10

At 14:10 on a Wednesday, Dennis Quigley, sixty-six, is dressed, his bag is packed and he wants his IV out. He was admitted on Monday at 22:00 with right lower lobe pneumonia, a temperature of 102.8°F and an oxygen saturation of 89%, and started on IV ceftriaxone and azithromycin. He came off oxygen on Tuesday evening, but at 06:00 this morning his temperature was 100.9°F, and the team plans at least one more day of IV treatment.

His nurse, Silas Nakamura, pages the hospital medicine NP covering the floor, Fiona Strand. Mr. Quigley's wife had a stroke last year and cannot be left alone, and the neighbor staying with her goes home tonight. "I'm not lying in this bed," he tells Silas, "while she's on the floor somewhere."

Fiona sits with him from 14:20 to 14:45, and Dr. Vikram Mehta, the attending, joins them at 14:35. Mr. Quigley signs the hospital's form at 15:00 and walks out at 15:15.

What the record will be asked

A patient leaves **against medical advice**, usually shortened to AMA, when they end care before the treating clinicians judge it safe, after being told so. Leaving refuses everything that would have followed. So the record will first be asked what the chapter on informed refusal asked of any no: whether he understood

what he was refusing and what could follow, whether he heard the alternatives, and whether he could make the decision.

A departure adds a question of its own: once he said no, was he given every reasonable chance to do well anyway, and to come back? Suppose Mr. Quigley returns on Saturday in septic shock. A reviewer who knows that ending will look at the hour before he left to see what he took with him: a treatment, a follow-up and a way back, or a signature and nothing else.

The note as written

Nobody in this case is a real person. At 15:30 Fiona writes:

> 15:30. Pt leaving AMA. Risks explained incl. death. Pt has capacity. AMA form signed. Dr. Mehta aware. IV d/c'd.

The chapter on informed refusal showed what a form cannot record, and the note beside this form says little more. “Risks explained incl. death” names the worst outcome without the path to it: what pneumonia does when treatment stops early, and which signs would mean it was happening. “Pt has capacity” is a conclusion with none of the answers that the chapter on the capacity question puts behind one. His reason is gone, so the departure reads as impatience rather than a husband's choice. And the note ends at the door. Nothing says what treatment he left with, who will see him next or whether anyone told him he could come back. Read on Saturday, it describes care that ended with a signature.

The rewrite

This time the note goes past the signature, to what he left with and how he could come back:

> 15:30. Left hospital against medical advice at 15:15. His reason: “My wife can't be alone tonight. The neighbor goes home at six.” Day 3 of IV ceftriaxone and azithromycin for RLL pneumonia. T 100.9°F at 06:00, 99.8°F at 14:00; HR 96; SpO2 93% RA at rest, 91% walking at 13:30 per RN S. Nakamura.

Recommended: stay at least 24 more hours for IV antibiotics and monitoring until fever resolves.

Discussed 14:20–14:45; Dr. V. Mehta, attending, from 14:35. Explained in plain words: the infection is better but not controlled. If he leaves now, it is more likely to get worse, spread to his blood or lower his oxygen, and any of these can be life-threatening and can happen at home overnight. Offered: social worker to arrange overnight help for his wife so he could stay (declined: "She won't let a stranger in the house"). His account of the risk: "It's not finished, and it could get bad fast. If I can't breathe or I get mixed up, I come right back." Capacity for this decision: restated the risks in his own words, applies them to himself, reason consistent with his values, choice unchanged over 25 minutes. Has capacity at this time to decline further admission.

Leaving with: oral amoxicillin-clavulanate and azithromycin, chosen with Dr. Mehta to continue similar coverage by mouth, sent to his pharmacy 14:55. Told that pills are a step down from IV at this stage but far better than none. IV removed by RN 15:05, catheter intact. Return to the ED or call 911 for breathlessness at rest, confusion, chest pain, blue lips, temperature 102°F or higher, or vomiting that keeps him from taking the pills; written copy given, and he repeated them back. Blood cultures from Monday: no growth to date, final pending. I will follow them and call his cell, number confirmed, if positive. 14:50 called the office of Dr. Grace Ochoa, PCP: visit booked Friday 09:30; discharge summary to her today. Told: "You can come back here any time, tonight included, and we'll pick up where we left off. Nobody will hold it against you." He said: "Good. I might need that." Form signed 15:00, copy given.

Leaving the door open

Nothing on the AMA form, and nothing in the note as written, matches the rewrite's last paragraph. A patient who refuses the hospital has not refused all care. What he still accepts is the partial yes from the chapter on informed refusal, and here that is the outpatient plan. Withholding a prescription or a visit to press him to stay would send a man with pneumonia home untreated, and on the

record that reads as punishment. The oral antibiotics need a reason, as the chapter on prescribing asks of any drug, and a line on what he was told about stepping down from IV.

Write the invitation to return the way you said it, because a reviewer cannot tell "may return PRN" from a door politely closed. The wording matters at the bedside too: a patient who thinks he has burned a bridge has a reason to stay home while his breathing worsens. Leave out warnings you cannot stand behind, such as a claim that insurance will refuse to pay. You rarely know it, the patient may believe it, and a note that records it reads as pressure, not information.

Each part of the plan he leaves with also needs an owner, as the chapter on closing the loop required of every result. Cultures that post after a patient has left belong to someone, and the note names who. The visit has a day and an hour, and the next clinician hears about the stay from the team, not from the patient. Write the departure itself by the rules in the chapter on facts and quotes: "insisted on leaving despite repeated warnings" records a contest, not a conversation.

The patient who is simply gone

If Mr. Quigley had refused to sign, the rewrite would change in one line: "Declined to sign form; witnessed by RN S. Nakamura." Some patients leave before any conversation can happen, as in a second case, imaginary like the first. Tariq Rahimi works nights as an NP in a community emergency department. At 01:00 on a Sunday he sees Troy Underwood, forty-seven, whose chest pressure began at 23:00 and has faded. The ECG and the first high-sensitivity troponin are normal, and a second troponin is due at 03:10. At 01:40 Tariq tells him why: "A heart attack doesn't always show on the first test." At 02:40 his nurse, Nora Vance, finds the room empty.

Nobody discussed leaving with him, so there is no refusal to record, and the note must not invent one. "Left AMA" implies that the risks were explained and a choice was made. Your organization may use another label for a departure nobody witnessed, such as *eloped* or *left before treatment complete*. At 02:55 Tariq writes:

02:55. Not in room at 02:40, when RN N. Vance came to draw repeat troponin; last seen by her 02:05, vitals normal, no pain. Gown on bed; saline lock on bedside table, catheter intact. ED, restrooms and waiting room checked; security checked main exit, did not see him. Workup incomplete: ECG 01:05 and troponin 01:10 normal; repeat troponin due 03:10 not drawn. At 01:40, alert and oriented, told the plan and why the second test is needed. No discussion of leaving took place. 02:50 called cell on file: voicemail asked him to call or come back, no results stated. Dr. Alan Wexler, ED attending, informed 02:55; per department policy, no police welfare check requested: no concern about capacity. Will call again 03:30.

Addendum, 03:40. Reached by phone 03:30, identity confirmed. Speech clear, answers coherent. Left because "the wait was killing me, and I start work at seven." No pain since midnight. Explained that the second test could still show a heart attack the first one missed; invited him back now or at any hour. Declined tonight; will call his PCP for a visit Monday. Told to call 911, not drive, if the pressure returns or comes with sweating or shortness of breath. He said: "So it could still be my heart. If it comes back, I'm calling 911."

The line about the discussion that never took place keeps the first entry honest. The addendum is the informed refusal, held by phone after the fact and labeled the way the chapter on templates and the late addendum describes. Your organization's policy sets who must be told after a departure nobody saw, and whether security or the police are asked to look for the patient. Write which steps it called for, who took them and when. Had Mr. Underwood been confused, or held under a legal order, his leaving would be a safety event, not a choice. What staff may do about it then depends on state law and policy, so ask your attending and risk manager.

What the note must contain

- **The departure.** The time, the patient's reason in their own words, and the care still recommended.
- **The informed refusal.** The risks of leaving now, the alternatives offered (including help with the reason for leaving), and the capacity finding,

written as the chapters on informed refusal and the capacity question describe.

- **The door left open.** The invitation to return as it was said, and the patient's reply.
- **What the patient left with.** Prescriptions accepted or declined, lines removed, instructions for this illness, pending results and their owner.
- **The follow-up.** Who sees the patient next, when, and who arranged it.
- **Who was told.** The attending and anyone else policy names, with times.
- **The form.** Signed or declined, filed beside the note and never in place of it.
- **The unwitnessed departure.** When the patient was last seen and found gone, the search, what they had been told, each attempt to reach them, and who was notified. Never a conversation that did not happen.

CHAPTER 18

The Discharge and the Patient Who Comes Back

A third episode in a month

The pain began at 18:15 on a Wednesday, half an hour after dinner. At 19:50 Bettina Prentiss, forty-four, describes it to Paloma Marchetti, an NP at an urgent care clinic with X-ray and an ECG machine, but no ultrasound and no same-night blood tests. The pain is steady, under her breastbone and right ribs, and reaches around to her right shoulder blade. She has had two episodes like it this month, each gone within an hour or two. The clinic and both women are inventions.

Her temperature is 98.4°F and her heart rate 92. Her right upper abdomen is tender. When Paloma presses under the ribs and asks for a deep breath, the breath does not catch, so Murphy sign, a pointer to an inflamed gallbladder, is absent. Her eyes are not yellow and her ECG is normal. After ketorolac and ondansetron at 20:05, her pain is down to 1 out of 10 by 20:50.

Paloma thinks a gallstone is the likeliest cause, but tonight she can neither see the gallbladder nor check the blood. She offers the emergency department, which can do both. Ms. Prentiss chooses home: “These always go away, and my husband's with me.” She leaves with him at 21:05, carrying a written copy of the signs that should send her to the hospital.

What the record will be asked

A discharge is a disposition, in the sense of the chapter on the provider note, and the record will ask three things of it. Why was home safe enough tonight, for this patient, given what was known and what was not? What was she told to watch for, and did she understand it? And who would see her next, when, and could she actually get there? If Ms. Prentiss comes back, a fourth question comes with her: what changed between the two visits? A reviewer will lay the second note beside the first and read them as one story.

The note as written

Her note goes into the chart at 21:15:

> 44F epigastric/RUQ pain after dinner, vomiting x1, similar episodes x2. RUQ tender. ECG nl. Toradol, Zofran, pain resolved. Dx: biliary colic. Outpt US. F/u PCP. Return to ED for worsening pain, fever or other concerns. Abdominal pain handout given.

The diagnosis reads as settled. That is the false certainty the chapter on the working diagnosis found in Erin Kowalski's first note, and here the discharge pays for it. A settled diagnosis seems to need no precautions of its own, so these could follow any visit. “Worsening pain” leaves a woman whose pain comes and goes to judge alone whether a recurrence counts, and “fever” has no number. “Outpt US” has nobody booking it, and “F/u PCP” has no name or date. The note also drops the emergency department she was offered, so home reads as the only option anyone considered.

The rewrite

With the open questions left in, the same visit reads:

> 44F, steady epigastric and RUQ pain from 18:15, ~30 min after dinner, radiating to R scapula; vomited x1. Two similar episodes this month, each resolved within 1–2 h. Seen 19:50: T 98.4°F, HR 92, BP 136/82. RUQ tender, Murphy sign negative, no jaundice. ECG 19:58 normal. Urine hCG negative.

Ketorolac IM, ondansetron 20:05. At 20:50: pain 1/10, HR 80, RUQ mildly tender, Murphy negative, tolerating water.

Assessment: biliary colic most likely: post-meal RUQ pain radiating to R scapula, third episode, resolved in clinic. Not excluded tonight without labs or US: early acute cholecystitis, common bile duct stone, pancreatitis. Against them now, though none excludes them: afebrile, Murphy negative, no jaundice, pain resolved. Would reconsider for pain that returns and persists, fever, jaundice or dark urine, or pain into the back with vomiting.

Disposition: offered ED tonight for labs and US, likely several hours. Prefers home: "These always go away, and my husband's with me." Husband driving, with her overnight. Told a gallstone is the likeliest cause but cannot be confirmed tonight. Told to go to the hospital ED, not this clinic, for pain that returns and lasts more than 3 h, temperature 100.4°F or higher or shaking chills, yellow eyes or skin or dark urine, or pain through to the back with vomiting. Written copy given. Asked what would send her back: "If it lasts three hours, or I get a fever, or I turn yellow or my pee goes dark." Back pain with vomiting reviewed again; she repeated it.

Follow-up: RUQ US ordered; patient to call imaging center (number on instructions) Thu after 07:00. Front desk to call her Thu 12:00 to confirm a date. PCP Dr. Anjali Varma, family practice: EHR message sent 21:10 with findings and plan; patient to book visit after US. If no US or PCP visit by Fri, return here Fri for recheck; we will help arrange both.

Precautions the assessment wrote

Each instruction in the rewrite takes a diagnosis from the assessment and turns it into something Ms. Prentiss could notice at home. An inflamed gallbladder shows as pain that does not go away, or as fever. A stone in the bile duct shows as yellow eyes or dark urine, and pancreatitis as pain through to the back with vomiting. This is the sentence about what would change your mind, from the chapter on the working diagnosis, carried to the door. Without that sentence the

precautions have nothing to be written from, which is how the first note ended up with a generic list.

The printed handout can still go home with her, but it speaks to any patient with abdominal pain, while the note says what this one was told. “Return precautions given,” inserted at a keystroke as the chapter on templates and the late addendum warned, names none of them. What she understood goes in her own words, together with the question that drew them out, quoted by the rules in the chapter on facts and quotes. Her first answer left out the back pain, and the rewrite records Paloma going over it again. The record now shows an instruction that was checked, not assumed.

A follow-up she can reach

“F/u PCP” names a category, not a plan. A follow-up the record can defend has a who, a when and a way in. The who is a clinician or service by name, ideally one who knows the patient is coming, as Dr. Varma did from Paloma's message that evening. The when is a date, or a deadline tied to an event. The way in is a phone number, the hours it answers and a means of getting there.

One more line covers the appointment that cannot be made. Imaging slots fill, and primary care offices can book weeks ahead. For a patient with no primary care clinician, “follow up with PCP” points nowhere, so the note names the clinic or service that will see them instead. Paloma's last line gives the plan a floor: if neither the scan nor the visit happens by Friday, Ms. Prentiss comes back to the clinic that saw her. The front desk's Thursday call makes the clinic, not only the patient, responsible for noticing if no date comes.

The patient who comes back

The earliest ultrasound was Tuesday, and Dr. Varma's office had nothing before the following week. At 08:20 on Friday Ms. Prentiss returns for the recheck she was offered. Ingrid Tolliver, who has the morning shift, sees her at 08:40.

With the board filling, a second note comes easily:

> 44F known biliary colic, seen here Wed. Pain again today. Afebrile. RUQ tender. To ED for US and labs.

"Known" turns Wednesday's working diagnosis, which no test has confirmed, into a fact. "RUQ tender" was in Wednesday's note too, and "afebrile" hides a number. Side by side, the two notes show the same findings and opposite decisions. A reader who knows the ending will decide that Wednesday's was the mistake. Ingrid's note measures Friday against Wednesday:

> Return visit. Seen here Wed 19:50 by P. Marchetti, NP; that note read before seeing patient. Working diagnosis then biliary colic, unconfirmed: US not done, earliest slot Tue; no PCP visit yet. Since Wed: Thu 22:00 episode like the others, gone by 23:30. Today dull, constant RUQ ache since 06:30, unchanged after ibuprofen 400 mg at 07:00. Nausea, no vomiting, no back pain, urine normal color. Seen 08:40: T 100.1°F (98.4 Wed), HR 98 (80 Wed at discharge). RUQ tender, Murphy sign positive (negative Wed). No jaundice.
>
> Assessment: acute cholecystitis now possible and not excluded, given changes since Wed: constant pain not easing after ibuprofen, new Murphy sign, temperature up. Plan: ED now for labs and US. 08:55 history and findings given by phone to Dr. Felix Romano, ED attending. Husband driving; left 09:05. Told her the gallbladder may now be inflamed and needs testing today. She said: "So this one isn't like the others."

Reading Wednesday's note first gives Ingrid a baseline and the triggers Paloma set. Taking the history again keeps Wednesday's diagnosis from becoming hers by default. The triggers are a place to start, not a checklist. A temperature of 100.1°F is below the threshold Ms. Prentiss was given, and Ingrid records it as a change anyway, because she is comparing Friday's patient with Wednesday's, not with a printed cutoff.

The values in parentheses carry the second note. They show a patient who changed between visits, and that lets each decision be judged on its own terms: home for the woman described on Wednesday, the emergency department for the one described on Friday. Had nothing changed, the note would have to say so, and say why the same answer still fits a patient who came back. "Missed at prior visit" or "should have gone to the ED Wednesday" is a verdict on a colleague, which the chapter on facts and quotes keeps out of the chart. Wednesday's note stays as Paloma wrote it, and Friday's note records Friday.

What the note must contain

- **The disposition and why.** Where the patient went, what made it safe enough, the options offered and the patient's choice.
- **The uncertainty carried out the door.** The working diagnosis labeled as working, and the dangers not yet excluded.
- **Precautions for this diagnosis.** Each open danger as a sign the patient can recognize, with a threshold and a place that will be open.
- **What the patient understood.** Their account in their own words, and the written copy.
- **The follow-up.** Who by name, when, how to reach them, who was told, and what happens if the appointment cannot be made.
- **On a return visit.** The earlier note reviewed, a fresh history and exam, each change with values from both visits, and the decision with its reason.

CHAPTER 19

Handoffs, Cross-Cover, and the Night Call

A pressure of 84 at 02:10

Thirty-eight patients become Claire Yamada's at 19:00 on a Tuesday, when her night shift as a hospital medicine NP begins, and she has never met any of them. One is Leonard Vogel, seventy-two, admitted at 14:30 by the day NP, Deepa Natarajan, with right pyelonephritis.

On this fictional Tuesday, Mr. Vogel's sign-out takes ninety seconds. Ceftriaxone has run since 13:10, and blood cultures from the emergency department are pending. His blood pressure, Deepa says, is "a little soft, 104 over 62 at five, after a liter in the ED. Probably dry. I'm holding his lisinopril." A repeat lactate is due at 21:00; the first, at 12:50, was 2.4. "If his systolic drops under 90, give him 500 of LR and recheck." Claire writes it on her list.

At 21:40 she sees the repeat lactate: 1.6. At 02:10 his nurse, Ruby Whelan, pages her: blood pressure 84/50, temperature 101.3°F, and "he's sleepier than he was at midnight." Claire is at the bedside at 02:20. Before ordering anything, she opens his medication record and finds lisinopril 20 mg, his usual evening dose, given at 21:00; the order to continue it was still active. At 02:45 she calls Dr. Kofi Mensah, the night hospitalist.

What the record will be asked

A clinician on *cross-cover*, caring overnight for patients another team admitted, inherits them without the reasoning behind the plan, and a reviewer knows it.

Suppose that on Thursday Mr. Vogel is in intensive care with septic shock. The night's entries will be read against one question in two parts: what did Claire know, and what did she do with it? Some of what she knew she was told at 19:00, and some she could have found in the chart, and a reviewer will want to know which was which: a sign-out can be wrong, and relying on one without looking is judged differently from checking.

The second part asks whether the decisions after the page were hers: did she see the patient, form her own view of a man who had changed since the day team saw him, and decide whether their plan still fit? Behind both parts sits a question of ownership: who was responsible for each pending item, and when did it pass from one clinician to the next? Most of a handoff is spoken; the chart holds the answers only if someone wrote them down.

The note as written

Her 02:55 note, typed between pages, reads:

> 02:55. Cross-cover. Called for hypotension. 72M w/ R pyelo on ceftriaxone, BCx pending. BP soft since admission, lisinopril held. Pt seen, mentating. Hypotension likely sepsis vs meds. 1 L LR bolus, labs. Dr. Mensah aware. Will monitor. Day team to f/u in AM.

Almost nothing in it is Claire's. “BP soft since admission, lisinopril held” is the sign-out in her voice, with no values and no source. It fits the day team's plan and the order Claire stopped at 02:30, but not the 21:00 dose, and beside a medication record that shows the dose, it reads like a clinician who never looked.

“Pt seen, mentating” has no time and no findings, so the nurse's “sleepier” goes unanswered. “Sepsis vs meds” names two causes without saying which she favored, or why the bolus was twice the size the sign-out planned. “Dr. Mensah aware” does not say when, of what, or whether he saw the patient. “Will monitor” sets no limit, and “day team to f/u” hands nothing to anyone.

The rewrite

Here are the night's entries, written from what Claire was told, checked and did:

21:40. Cross-cover, night NP. Pending item accepted at 19:00 sign-out from D. Natarajan, NP: repeat lactate 1.6 at 21:00 (2.4 at 12:50). BP 110/66 at 21:30. No change to plan.

02:55. Cross-cover. Called 02:10 by RN Ruby Whelan: BP 84/50, T 101.3°F, "he's sleepier than he was at midnight." Per 19:00 sign-out (D. Natarajan, NP): R pyelonephritis, ceftriaxone since 13:10, ED blood cultures pending; BP 104/62 at 17:00 after 1 L in ED, thought volume-depleted; lisinopril to be held; if SBP below 90, LR 500 mL. Seen 02:20: BP 86/52 manual, HR 98, RR 22, SpO2 95% RA. Drowsy, rouses to voice, oriented x3, slow to answer. Lungs clear. R flank tender. Urine 120 mL since 22:00. Checked in chart: MAR shows lisinopril 20 mg given 21:00 (home evening dose; order active). CT 12:40: R pyelonephritis, no hydronephrosis or stone. Hb 12.6 at 12:50; no bleeding per RN.

Assessment: hypotension with new fever and drowsiness. Worsening sepsis from R kidney most likely; lisinopril at 21:00 likely contributing. Obstruction: none on CT. Bleeding: no signs; Hb repeated. Fever and drowsiness new since sign-out, so bolus larger than the 500 mL planned.

Plan: LR 1 L bolus 02:30. Lisinopril order discontinued 02:30. Lactate, CBC, BMP 02:40. 02:45 Dr. Kofi Mensah, night hospitalist, by phone: given vitals, exam, MAR finding and plan; has not seen patient. Agrees; continue ceftriaxone. If SBP below 90 after bolus: rapid response, and he will come to bedside. RN to call me for SBP below 90, HR above 110, more drowsiness or T 102°F or higher. Reassess after bolus.

03:35. Seen 03:25: BP 104/62, HR 90. Awake, answers promptly. Lactate 2.6 at 02:40 (1.6 at 21:00); Hb 12.1; creatinine 1.8 (1.6 at 12:50). 03:30 Dr. Mensah updated: continue plan. Repeat lactate 05:00; reassess then.

What you were told, and what you accepted

The written part of a handoff is the sign-out, a list or an electronic tool that in many hospitals sits outside the medical record, is updated every shift and is sometimes discarded at the end of it. Whether yours is kept, and for how long, is

a question for your risk manager. Either way, do not count on it to show later what you heard.

So when a decision of yours rests on something you were told, put it in your note with its source and its time, as the rewrite does with “Per 19:00 sign-out.” It is the attribution the chapter on facts and quotes asks of a nurse's report. You do not need a note for every patient on the list. You need one when you act, and when what you were told shapes what you do.

Pending items need the same care, because a handoff moves ownership along with information. An item you accept at 19:00, such as the repeat lactate, is yours until you close it or hand it back by name, as a result belonged to its owner in the chapter on closing the loop on the abnormal result. The 21:40 entry is short, and it shows the item received, seen and closed. The same holds in clinic: a result that posts while you cover a colleague's inbox is yours, and the addendum carries your name.

Your assessment, not the sign-out's

The sign-out carries the day team's working diagnosis, and as the chapter on the working diagnosis put it, a working diagnosis has a date on it. “Probably dry” described a man at 17:00. The man Claire saw at 02:20 had a fever, slow answers and a dose of lisinopril nobody had planned. Recorded as what you were told, the day team's view is a fact. Carried into your assessment, “per day team” or not, it is a judgment you did not make about a man who has since changed, with your signature under it.

The if-then plan in the sign-out deserves the same look. “Give him 500 of LR” was advice written before the event, by someone picturing it. When the event comes, decide whether the advice still fits, and write which it was: followed, changed or set aside, with the reason in a few words. The last line of Claire's assessment does that, and it turns a bolus twice the planned size from a deviation into a decision.

Keep the note brief and timed. The background is in the admission note, so point to it rather than retell it, and spend the words on what only you can supply: what you found, what you checked and what you made of it. The alternatives fit the short form from the chapter on the differential. Where the sign-out and the chart disagree, write both as facts, with no word on whose error

it was. If the discrepancy needs review, it goes through your organization's reporting channel, not the chart, as the chapter on the consultant who never comes advised.

Handing the patient back

At 07:05 the handoff runs the other way. What Deepa Natarajan hears is not in the chart unless Claire puts it there, so at 07:10 she adds one entry:

> 07:10. Cross-cover summary for day team. Hypotension 02:10, see 02:55 note. After LR 1 L: SBP 102–112 since 03:25, 108/64 at 06:15, HR 86; alert and oriented at 05:05 reassessment. Lactate 1.5 at 05:00. Changed overnight: lisinopril order discontinued. Pending: ED blood cultures. Open for day team: whether to restart lisinopril, and antibiotic choice when cultures result. RN call limits from 02:55 note still in force. Verbal handoff to D. Natarajan, NP, 07:05.

The entry tells Deepa what changed, what is still waiting and which decisions are now hers, so nothing rests on a conversation in a hallway. A reviewer will look there for the moment each open item passed to a named clinician. Deepa's own note starts from her own exam: copying the night entries forward would put her signature under findings she did not make, the trap described in the chapter on templates and the late addendum.

What the note must contain

- **What you were told.** The source and time of the sign-out, and the items your decisions rest on.
- **The pending items.** Each item accepted, its result and what you did, or the clinician you handed it back to.
- **The call.** Who called, when, and what they reported.
- **Your own look.** When you saw the patient, the findings that bear on the problem, what you checked in the chart, and any disagreement with the sign-out, as facts.
- **Your assessment.** The working diagnosis and alternatives in your words, and whether the inherited plan still fit.

- **What you did and whom you told.** Orders with times; the physician you called, when, whether they saw the patient and what they advised.
- **The limits and the next look.** What the nurse will call you for, and when you will see the patient again.
- **The handback.** What changed overnight, what is pending, which decisions now belong to the day team, the limits still in force, and who took over and when.

CHAPTER 20

The Telehealth Visit and the Portal Message

A peak flow held up to the camera

Thursday afternoons are video visits for Maya Thibodeaux, a primary care NP. At 14:40 she connects with Colleen Pacheco, forty-six, who has had asthma since childhood. A cold began on Saturday; since Monday she has been wheezing and short of breath, and since Tuesday she has needed her albuterol inhaler every four hours. This visit never happened, and neither did the night call later in the chapter.

Maya cannot listen to her lungs, so she asks for what a camera can show. Ms. Pacheco talks in full sentences while she walks to the kitchen and back, lowers her collar, holds her pulse oximeter up to the lens, blows three times into her peak flow meter and turns the laptop toward her calves. At 14:58 Maya decides on home treatment and sets the point at which Ms. Pacheco must be seen in person.

What the record will be asked

Suppose that on Saturday Ms. Pacheco reaches the emergency department with pneumonia. Beyond what is asked of any visit, a reviewer will ask three things of this one. What could Maya assess through a camera, and what could she not? Which findings came from her own eyes, which from the patient's devices and which from the patient's account? And given what the screen could not show, why was remote care reasonable, and what would have brought her in?

The chapter on what a chart becomes after a bad outcome showed that the setting counts toward the standard of care, but only the setting the reader can see. A video visit is part of the circumstances, and it is also a choice. The record has to show that it was chosen for reasons, not inherited from the way the visit was booked.

The note as written

Maya closes her note at 15:10, as the next patient connects:

> Telehealth visit. 46F hx asthma, SOB and wheeze x 3 days after URI, albuterol q4h. Gen: NAD. Resp: no respiratory distress, lungs clear to auscultation bilaterally. SpO2 95%. A: asthma exacerbation. P: prednisone 40 mg daily x 5 days, continue albuterol. ED if worse.

Maya never typed one of these lines. “Lungs clear to auscultation” came from the clinic's default exam. It is the finding nobody could have made, from the chapter on templates and the late addendum, and it costs the rest of the exam its weight, “NAD” included. “Telehealth” does not say whether there was a picture or only a voice, and “SpO2 95%” has no device and no source. “ED if worse” gives a woman with a peak flow meter on her counter no number to watch. Nor does anything say an in-person exam was weighed, so the lungs nobody heard read as an oversight rather than a limit.

The rewrite

How the visit was held goes first, and the numbers that would bring her in go last:

> Video visit, audio and video throughout, 14:40–14:58. Patient at home; address on file confirmed for emergency use. Identity confirmed by name, date of birth and chart photo. Alone. Telehealth consent per clinic process, confirmed at start.
>
> Reported: cold since Sat; wheeze, chest tightness and shortness of breath since Mon; albuterol q4h since Tue, relief ~3 h. Home temp 99.0°F this morning. No

chest pain, leg pain, hormone use or sputum change. Two prior flares, settled at home.

Observed on video: full sentences, also while walking to kitchen and back. RR 22, counted twice over 30 s. No neck muscle use or retractions, collar lowered. Calves symmetric, no visible swelling. Measured on camera, her devices: peak flow 300, best of 3 (personal best 430 per action plan in chart: 70%); pulse oximeter 95%, steady, finger warm. Not assessable by video: lung and heart sounds, temperature, oxygen on exertion.

Assessment: asthma exacerbation after viral URI, like her prior flares. Pneumonia not excluded without auscultation; against it, near-normal temperature, no sputum change. PE: gradual onset with URI, no hormone use, calves symmetric. Remote care reasonable today: full sentences, SpO2 95%, peak flow 70% of best. In-person exam offered today 16:30; she prefers home treatment.

Plan: prednisone 40 mg daily x 5 days; albuterol as before. Peak flow and oximeter at 20:00 and 08:00, readings by portal by 09:00. In-person visit held Fri 10:20, to keep if peak flow below 340, tightness not easing or temperature 100.4°F or higher. ED or 911 for: cannot finish a sentence, blue lips, oximeter below 92%, peak flow below 215, or albuterol relief under 2 h. She said: “Under 340 or a fever, I come in. Under 215, or I can't talk, I go to the ER.”

Observed, measured, reported

The rewrite sorts the remote exam by source. What Maya saw or heard through the connection is *observed*, and it counts as her exam. A number from the patient's own device, read on camera at Maya's request, is *measured*: data with a source, so the note names the device and how the reading was seen. A number the patient only reports is history. And the line on what was not assessable names what the modality ruled out.

That line keeps the assessment honest. Pneumonia stays “not excluded” because nobody heard the lungs, and the plan answers it, as the chapter on the working diagnosis requires: a fever is one of the triggers for the morning visit.

The reasons for remote care point to findings already in the note, so a reviewer can test the decision against them. The thresholds open two doors, the clinic for slow progress and the emergency department for danger, each with numbers Ms. Pacheco can read on her own devices. If the video fails partway, the note says when, and what was seen before it.

Advice by phone and portal

At 08:15 on Friday a message reaches Maya's inbox from Ms. Pacheco's portal account: “Peak flow 320 this morning, feel a little better. Do I still need to come in? I'd have to miss work.” A reply that tells a patient what to do is a clinical decision, by phone or by portal, and the record will ask four things of it: who asked, what they asked, what was advised and how the answer reached them.

Who asked is not always the patient: a daughter writing through her father's proxy access receives the advice, and the note names her. Record the question in the note, since a portal thread may not be filed with the chart; ask health information management what your system does. When a nurse or medical assistant sends the reply, the note says whose advice it carries. The method has to fit the urgency. A portal reply can sit unread until evening, and this answer cannot wait:

> 08:45. Portal message 08:15 from patient: peak flow 320, “a little better,” asks whether to keep visit. Below in-person threshold of 340 set yesterday. Called cell 08:35, reached patient, identity confirmed. Advised visit today for lung exam; offered 12:40 instead of 10:20, to fit work. She chose 12:40. ED thresholds reviewed, unchanged. Portal reply 08:42 confirming 12:40.

The call after hours

The night call belongs to Jonah Soto, an NP who covers the practice's after-hours line from home. At 23:02 on a Wednesday, paged by the answering service, he reaches Gail Rinaldi, thirty-nine, on day three of amoxicillin for strep throat, with an itchy rash since 21:00. The answering service keeps a log of its

own, but the day team will open the chart, so at 23:20 Jonah writes a short note there:

> After-hours call 23:02–23:12. Caller: patient, identity confirmed by name and date of birth. Chart reviewed remotely: amoxicillin started Mon by Dr. Miriam Voss for strep pharyngitis; no allergies listed. Itchy raised rash since 21:00. No swelling of lips, tongue or throat; no trouble breathing or swallowing; no faintness. By phone: full sentences, voice not hoarse. Assessment: hives, likely reaction to amoxicillin; no features of anaphylaxis. Advised: stop amoxicillin; cetirizine 10 mg tonight. 911 for swelling of lips, tongue or throat, trouble breathing or swallowing, or feeling faint. She repeated: "Stop the pills, take the Zyrtec, 911 if my mouth swells or I can't breathe." Amoxicillin (hives) added to allergy list. For Dr. Voss, inbox 23:20: choose replacement antibiotic, call patient Thu by 10:00. Patient told to expect the call.

The threshold turns the danger the assessment named into signs Ms. Rinaldi can notice herself, as in the chapter on the discharge and the patient who comes back. The last lines are the handoff, built like the handback in the chapter on handoffs, cross-cover and the night call: what changed tonight, which decision the day team now owns, and who acts by when. If you take a call without chart access, write that too.

Consent, location and identity

The rewrite, the portal entry and the night note all record who was on the other end, and the video rewrite adds where Ms. Pacheco was and that she agreed to be seen remotely. These lines look administrative, and the rules behind them can differ by state and by organization, so this book leaves them as questions. Ask before your first remote visit, not after a complaint.

Ask your organization whether a remote visit needs its own consent, whether a verbal yes is enough and what the note must say about it; what it accepts as proof of identity for a patient you have never met; and whether others in the room must be named. If the patient may be across a state line during a visit, ask your board, and that state's board, whether you may treat her while she is there,

and on what terms. Whatever the answers, record where the patient is: if she collapses mid-sentence, the address on file may not be where help is needed.

What the note must contain

- **The modality.** Video, phone or portal; start and end times; any switch partway, and when.
- **Who, where and consent.** How identity was confirmed, where the patient was, who else was present, and consent as your organization requires.
- **The remote exam, by source.** What you observed, what the patient's devices measured on camera, what was only reported, and what could not be assessed.
- **Why remote was enough.** The findings that made remote care reasonable, and the in-person option offered.
- **The threshold for in-person care.** The signs or readings that bring the patient in, and those that send them to the emergency department.
- **Phone and portal advice.** Who asked, in their words; what was advised and by whom; how the answer was delivered and whether it was received.
- **The after-hours call.** The time, your chart access, the advice, the escalation threshold with the patient's read-back, and who owns the next step by when.

CHAPTER 21

Missed Visits and Referrals That Go Nowhere

A positive test and two empty slots

At her annual visit on March 4, Marlene Ostrowski, fifty-eight, takes home a fecal immunochemical test, or FIT: a stool test for blood too slight to see, used to screen for colorectal cancer. Nina Sandoval, who ordered it, is an NP at a family practice. On Tuesday, March 18, the result posts: positive.

Ms. Ostrowski is fictitious. At 16:10 Nina reaches her at home. A positive test does not mean cancer, she explains, but the blood has a source, and a colonoscopy is how it is found. Ms. Ostrowski wants to talk it through in person before agreeing to anything, so a visit is booked for March 25 at 10:00. She does not come. The clinic rebooks her for April 8 at 09:20, and she misses that visit too.

What the record will be asked

Suppose the story stops there, and the following January Ms. Ostrowski is found to have colon cancer. On the timeline from the chapter on what a chart becomes after a bad outcome, her record shows a positive screening test in March, two no-show flags and then months of silence. A reviewer will ask three things of it. Was the follow-up necessary, and does a line written before anyone knew the

ending say so? Was she told why it mattered, in words she could act on? And when it did not happen, who noticed, when, and what was done?

The scheduling system answers none of these questions. A no-show flag records an empty slot, not anyone's decision about it. Whether the effort that followed was reasonable is the standard-of-care question again, and the stakes shape the answer: a missed blood pressure recheck and a missed visit after a positive cancer screening do not call for the same pursuit. An argument that the patient chose to stay away will rest on what the record shows she was told.

The note as written

Between patients, Nina writes two entries:

> 3/18. FIT positive. Pt notified, f/u appt scheduled.
>
> 4/11. Pt no-show x2. Letter sent. Noncompliant with f/u.

"Pt notified" does not say what she heard: nothing shows the word colonoscopy, or why it mattered, so the visit reads like a routine recheck she could skip. Nobody appears to have noticed the first no-show: the March 25 flag stands alone, and the rebooking shows up only as a second empty slot. "Letter sent" has no date, no content and no copy. "Noncompliant" is the label the chapter on facts and quotes took apart, a verdict on a woman whose reason for staying away nobody recorded. And the entries stop at the letter, with no next step and no date, so a positive cancer screening settles into the chart as if it had been handled.

The rewrite

Nina's record of those weeks could look like this:

> Tue 3/18, 16:25. Telephone encounter. FIT from 3/4 visit, resulted 09:40 today: positive. Called home 16:10, reached patient, identity confirmed by date of birth. Told in plain words: the test found hidden blood in her stool; it does not mean cancer, but the source must be found, and colonoscopy is how; polyps

and early cancers found this way are easier to treat. She asked to discuss in person before any referral: “I'm not agreeing to a scope over the phone.” Visit booked Tue 3/25 10:00 for that discussion. She said: “So it may be nothing, but only the scope can tell.” Added to problem list: positive FIT, colonoscopy not yet done.

Tue 3/25, 14:45. No-show, 10:00 visit for positive FIT (see 3/18). 10:45 called cell: no answer; voicemail asked her to call clinic, result not stated. 14:30 Keisha Monroe, MA, reached patient; reason given: forgot. Rebooked Tue 4/8 09:20; told the visit is about her test result.

Tue 4/8, 10:15. Second no-show, visit for positive FIT. 09:50 called cell and home: no answer; voicemails as on 3/25. Portal message 10:00: a test result needs follow-up, please call me. If no contact by Fri 4/11: letter per clinic policy. Will review 4/11.

Fri 4/11, 15:40. No call; portal shows message unread. Letter sent by certified mail per clinic policy, copy in chart: a test result needs follow-up; waiting could let a treatable problem become harder to treat; my direct line and clinic hours. Chart alert: positive FIT, colonoscopy not done; route any contact to me.

Wed 4/16, 12:50. Telephone encounter. Patient called 12:30. Why she missed both visits: “I kept putting it off. I was scared of what they'd find.” Explained again why colonoscopy is needed and what it involves; questions answered. Agrees to referral: “Okay. Better to know.” Referral sent 12:45 to Dr. Samuel Tran, gastroenterology, marked positive FIT. Will check scheduling by 5/14.

Outreach the record can count

The first entry, written on March 18 before anyone knew they would be asked, answers the reviewer's first two questions. “Easier to treat” is the reason for the visit in words a patient can weigh against a missed shift; “we need to see you” gives her nothing to weigh. Her read-back shows she understood why it mattered. The problem-list line keeps the open question in front of anyone who opens her chart, including a colleague who sees her for a sore throat in June.

Each outreach entry is built like the call log in the chapter on the consultant who never comes: time, method, what was said or left, what came back. A voicemail or an unopened portal message is an attempt, as the chapter on closing the loop on the abnormal result established, and the note never calls it "notified." Within what your policy lets a message say, it says that something needs attention and how to reach you. The copy in the chart shows what the letter said, not just that it went out.

The number of attempts, the methods and whether the letter goes by certified mail come from your organization's policy, not from this book, as they did for the result in that chapter. Treat policy as the least you do; the stakes set how far beyond it you go. Nina tried the home number as well as the cell and gave the letter a date, because this result could not wait for Ms. Ostrowski to remember it. Each entry ends with the next step and its deadline. Her reason, once she gave it, goes in her own words: fear can be answered, and a label cannot.

Had she never called, the April 11 entry could stand as the last step policy required. It records what was done, that the result is still open and how the chart will catch her next contact. "Lost to follow-up," written after an entry like that, describes where an effort ended instead of replacing one.

The referral that never comes back

The invented case goes on. On May 14, the date Nina set, the referral still reads "sent" in the clinic's system, with no appointment beside it. She calls the gastroenterology practice, then the patient, and writes:

> Wed 5/14, 11:40. Referral check, positive FIT (referral sent 4/16). No appointment in system. 11:05 called GI practice: referral received 4/17; voicemails left for patient 4/18 and 4/25, no reply. 11:20 called patient: "I thought they'd call me again." GI practice's scheduling number given; she will call today.
>
> Addendum, 16:40. GI practice confirms colonoscopy booked Tue 6/10. Will check for report by 6/24.
>
> Tue 6/24, 09:40. Colonoscopy report not in chart. 09:10 called GI practice:

done 6/10; report sent, received 09:30 and reviewed. Two polyps removed; pathology pending. Per GI practice, Dr. Tran's office will give patient the pathology result and set her next colonoscopy; copy to be sent here. Problem list updated: colonoscopy 6/10, pathology pending at GI practice.

A referral marked "sent" records an order, not an appointment, and an appointment is not a result. Nina's two checks sit where referrals stall: the scheduling that never happens and the report that never comes back. Each was dated at the step before it, so neither depended on her memory. Who tracks referrals is your organization's decision: a referral coordinator in some, the ordering clinician in others. Ask who it is where you work, and what happens when one stalls.

Ownership follows the rule from the chapter on closing the loop on the abnormal result: it passes only when the next clinician knows and accepts. Until the colonoscopy was done and its report read, the question raised by the test Nina ordered was hers. On June 24 Dr. Tran's office took on the next step, and the entry names it. Nothing in her entries faults the GI practice or the patient; the unanswered voicemails and the missing report appear as facts with times.

The patient who says no

Suppose that on April 16 Ms. Ostrowski had said she would not have a colonoscopy. What had been silence would have become a refusal, and the note would be the one set out in the chapter on informed refusal: the risk of leaving the bleeding unexplained, in plain words; the alternatives offered, such as a visit with Dr. Tran only to talk; her reason and her account of what could happen; capacity for this decision; and the next offer, at each visit, with each answer timed. The problem-list line stays.

The opposite mistake is writing "declined colonoscopy" for a patient who never answered. It records a conversation that did not happen, the error the chapter on the patient leaving against medical advice ruled out for the departure nobody witnessed. Unanswered calls, an unopened portal message and an unanswered letter are outreach, not a refusal, and the note calls them that.

What the note must contain

- **Why the follow-up matters.** At the call or visit that sets it up, the reason in plain words and what the patient said back.
- **The open item.** Where the chart will show it until it closes, such as the problem list or an alert.
- **Each missed visit, noticed.** Who reviewed it, when, and the decision.
- **Each attempt.** Time, method, what was said or left, and the reply or its absence; a copy of any letter.
- **The next step and its date.** At the end of every entry, within the steps your policy sets.
- **The referral, tracked.** Whether it was received and scheduled, whether the report came back, who checked and when, and who owns the next step until the report has been read.
- **The patient's answer.** Their reason in their own words; a refusal written as an informed refusal, and silence never written as one.
- **Where the effort ended.** What was done, that the item is still open, and how the chart will catch the next contact.

CHAPTER 22

The Adverse Event and the Records Request

A glucose of 48 at 05:50

Edith Nyberg's fingerstick glucose is 48 when the night nurse, Joel Ramsey, checks it at 05:50 on a Wednesday in February; she is sweaty and slow to answer, and he treats her under the hospital's hypoglycemia protocol. Seventy-eight, she was admitted on Monday with a flare of COPD, and the prednisone for it has pushed her glucose above 300. At 16:30 on Tuesday, Hazel Ventura, a hospital medicine NP, ordered insulin glargine, a long-acting insulin, 14 units at 21:00. Every part of this case is imagined.

At 07:05, before rounds, Hazel finds glargine 40 units in the medication record, documented at 21:10. She sees Mrs. Nyberg at 07:15, and at 10:45 she and the attending, Dr. Irene Sato, sit down with Mrs. Nyberg and her daughter, Linda.

What the record will be asked

Most notes in this book were written before anyone knew the ending. This one is written after something has gone wrong, and every reader will know it. They will ask what happened to Mrs. Nyberg and when it was found; what was done, how fast, and how she was watched while the danger lasted; and what she was told. And they will look for any line that reads as defense rather than care: a cause nobody witnessed, a colleague blamed, a pointer to a document outside the chart, or an entry that appears only after a lawyer has written.

The note as written

Hazel's one entry, written at 11:30 between admissions:

> 11:30. Pt received wrong dose of glargine last night (40u instead of 14u) due to RN error. Hypoglycemic overnight, treated by RN. Now stable. D10 started, will monitor BG. Dr. Sato aware. Incident report filed, risk mgmt notified. Pt and daughter informed; daughter very upset. Apologized.

The numbers are right, and little else is. “Due to RN error” names a cause Hazel did not witness. Finding out how 40 units came to be given is the review's job, and the chapter on facts and quotes keeps blame of colleagues out of the chart. The report and the call to risk management appear in the one document that should never mention them. “Now stable” has no values, and “will monitor BG” sets no frequency, limit or end for an insulin that keeps working through the day. “Daughter very upset” is a label, and both “informed” and “apologized” record that a conversation took place, but not a word of it.

The rewrite

Two entries replace the 11:30 note: the findings and plan at 07:55, and the conversation with Mrs. Nyberg and Linda at 11:10.

> 07:55. Hypoglycemia overnight. Per medication record, insulin glargine 40 units given 21:10 Tue; ordered dose 14 units. Per RN Joel Ramsey: at 05:50 diaphoretic, slow to answer, glucose 48; IV dextrose 05:55 per protocol; glucose 132 at 06:10, 96 at 07:00. Seen 07:15: awake, oriented x3, answers promptly, skin dry, no focal deficit. HR 88, BP 138/76. Glucose 84 at 07:30.
>
> Assessment: hypoglycemia after glargine 40 units, nearly three times the ordered dose. Glargine acts for about a day, and can act longer after an overdose or with reduced kidney function, so the risk of further lows lasts at least until tonight; glucose already falling after dextrose. No neurologic deficit.
>
> Plan: dextrose 10% IV at 07:45. Glucose hourly until 22:00, then as the readings dictate; BMP with potassium this morning. RN to call me, or night coverage

after 19:00, for glucose below 70 or any change in alertness. Tonight's glargine on hold; decision by 18:00 from today's readings. 07:35 Dr. Irene Sato, attending, by phone: told findings and plan, agrees; will see patient by 09:00. Pharmacy asked 07:50 to review insulin orders.

11:10. Discussion 10:45–11:05 with patient and daughter Linda Nyberg, present at patient's request; Dr. Sato and this NP. Dr. Sato explained: last night she received more of her long-acting insulin than was ordered, 40 units instead of 14; this caused the low sugar at 05:50, which was treated. Because this insulin lasts about a day, she will have a sugar drip and hourly checks until tonight, and tonight's dose is on hold. The hospital is reviewing how it happened. Patient asked: "Could it have hurt my brain?" I answered: no sign of that on exam; alertness checked with each sugar. Dr. Sato to update patient and daughter by 16:00; daughter's cell confirmed. Patient said: "So it's the extra insulin, and you're watching my sugar till tonight."

Facts without a verdict

The rewrite never uses the word *error*, and it hides nothing. The dose ordered and the dose given, each with its source, say exactly what happened. The clinical cause of the hypoglycemia belongs in the note; how the overdose came about belongs to the review. If the wrong dose had been in Hazel's own order, the rule would hold: the order as written, the dose given, no verdict. The rest is the method of every earlier scenario: values instead of "stable," and an assessment that says how long the danger lasts, which sets the checks.

Disclosure of an adverse event is telling the patient, or the person who decides for them, that something unexpected happened in their care, what it means for them and what is being done. It is not the same as releasing health information under privacy law. The 11:10 entry records the conversation the weak note only named, with the patient's questions in her own words. It states only what was known at 11:05; a guess at the cause, later proven wrong, would stay in the chart. Many organizations have a disclosure policy that sets who leads the conversation and how soon it happens, so read yours before your first event. Laws on apologies differ by state, and so does what they protect; ask your risk manager whether and how an expression of regret is recorded.

What the incident report carries

The chapter on facts and quotes explained why the chart never names the incident report, and the call to risk management stays out for the same reason. The report is written for the organization's safety review, and it carries what the chart leaves out. That means how the event came to light, and anything Hazel saw or was told that may have contributed, such as a look-alike pen or an interruption the night nurse described. It sticks to facts too: depending on how it is handled and on the law where you work, it may travel beyond the safety team.

Three habits keep the two documents apart and consistent. The clinical facts go in the chart even when the report repeats them, because the chart is the record of the patient's care and the report is not. The times and values match, so no reader has to ask which one is true. And the report stays in its own channel, with no copy in the chart; your policy says who files it and how soon. Ask your risk manager what to do with your own copies.

When the request arrives

In June, Hazel's risk manager calls: an attorney for Mrs. Nyberg has asked for her complete record, and a copy is on its way to Hazel. A **records request** is a request for copies of the chart, made by the patient or someone acting for them, often an attorney; health information management usually answers it. Reading her copy, Hazel sees that the bedside recheck she did at 15:30, with Linda in the room, appears nowhere. She remembers it clearly and wants to add one line.

Most organizations would say no. Once a complaint, a claim or an attorney's request has arrived, add nothing to the chart about the care in question and change nothing in it. The duty to leave the record alone can begin earlier, when a claim is reasonably expected, so ask risk management before you add anything about an event like this one. The chapter on templates, copy-forward and the late addendum gives the reasons. The audit trail dates every entry, a line written in June about February reads as a record built for an argument, and an altered record can be treated as spoliation. On the afternoon of the recheck, a late entry, labeled and written the same day, would have been ordinary.

What Hazel remembers goes to risk management or to the attorney handling the matter, in whatever form they ask for. The record is also larger than her notes: the hourly glucose values are already in it. If the patient is still in your care, chart today's care as you always would; the rule covers the past, not the present. And when you need to reread the chart, ask risk management how rather than opening it on your own: in most systems the audit trail records who opened the chart and when.

A complaint to your licensing board arrives as a letter, often with a deadline. A **subpoena** is a legal demand, also with a deadline, to produce records or to appear and testify. Most organizations ask four things of you: ignore neither, answer neither alone, do not hand over records yourself, and do not discuss the complaint or the claim with the patient, the family or their attorney. If you are still caring for the patient, keep answering questions about their care, and ask risk management what to say about the event. Whatever arrives, call your organization's risk manager the same day, and your own professional liability insurer if you carry a policy. Many policies require prompt notice of a claim, and some of an event that may become one; read the notice section of yours now.

What to ask, and whom

None of these calls decides anything, and nobody can promise you how the matter will end. Ask risk management what has arrived and from whom, whether the organization's coverage extends to you, what you may say, to whom, and whether you need your own attorney. Ask your insurer what your policy covers, from a subpoena to a board matter, as the chapter on where claims begin advised. Ask how soon notice is due, whether an attorney will be assigned and whether a settlement would need your consent.

Ask the attorney, first, whom they represent: you, your employer or both. If those interests could diverge, ask whether you need counsel of your own. Then ask about deadlines, what happens next and your part in it, whom you may talk to about the event, and whether and how to write down what you remember. For Hazel, that means asking whether she may talk about the event with Joel Ramsey or Dr. Sato, who both cared for Mrs. Nyberg, and how to set down what she remembers of the 15:30 recheck.

What the note must contain

- **The event as facts.** What happened and when, with the source of anything you did not witness; no guess at how it came about, no blame.
- **The patient's condition.** Findings with times and values, not "stable."
- **What was done.** Treatment and orders, with times.
- **How long the danger lasts.** Monitoring, call limits and an end point.
- **Who was told.** Clinicians involved in the care, with times; never the report or the call to risk management.
- **The disclosure.** When, who was present and who spoke, what was said, the patient's questions in their words with the answers, and who speaks to them next.
- **After an attorney's request, a complaint, a claim or a subpoena.** No new entries about the past care and no changes; today's care charted as usual.

PART IV

The Mid-Shift Reference

CHAPTER 23

Phrasebook of Weak and Defensible Wording

Models, not text to paste

The pairs follow the order of the chapters, and each heading is the title of the chapter that shows the full note and its reasoning. The weak line is what a busy, competent NP might write. The defensible line is shortened from the rewrite of the same encounter, and the reason says in a line why it holds up better. "Defensible" here means the note shows the reasoning; it is not a legal result.

The phrases are models to rewrite, never text to paste. Their times, values, doses, thresholds, names and quotations belong to constructed patients. A line pasted without the care behind it becomes what the chapter on templates and the late addendum warned against: a sentence you sign without having written it. Keep the shape and write it in your own words, about your own patient.

Writing the Differential You Actually Ran

Weak: PE unlikely.

Defensible: PE considered: judged low risk on clinical grounds, then PERC applied, none of 8 criteria present (age 33, HR 84, SpO2 98% RA; no hemoptysis, hormone use, prior VTE, unilateral leg swelling, or recent surgery or trauma). No further testing for PE.

Why: The danger has reasons of its own, and the rule shows its inputs after the clinical judgment it requires.

The Working Diagnosis and What Would Change It

Weak: r/o appendicitis. Clinical correlation recommended.

Defensible: Early appendicitis not excluded: at 6 hrs the exam may not yet show it. Would reconsider if pain localizes to the RLQ, becomes constant or hurts with walking, or if fever develops.

Why: "R/o" can mean open or excluded, and in a provider note the correlating clinician is you; say what would change your mind.

Weak: Risks and benefits discussed, patient agrees with plan.

Defensible: Options discussed: (1) ED now for blood work and imaging, likely several hours, may show nothing; (2) home, recheck here 09:00 tomorrow, accepting that appendicitis, if present, would be found later. Patient prefers 2, to avoid a night in the ED; roommate home overnight, has a car.

Why: The options as offered, what each costs and the patient's reason; the phrase would fit any patient.

Facts, Quotes, and What Never Goes in the Chart

Weak: Hx noncompliance, off metformin for months. Drug-seeking, demanding IV Dilaudid.

Defensible: Has not taken metformin for ~4 months; states it caused stomach upset. Reports L leg pain 8/10, "the pills don't touch it"; asked for IV hydromorphone by name. Last oxycodone 5 mg at 04:10.

Why: A behavior with its reason, and the exact request, replace a label that ends the asking and a motive nobody can observe.

Templates, Copy-Forward, and the Late Addendum

Weak (copied forward): Feels better, breathing easier, slept well. Exam: alert and oriented x3, NAD.

Defensible: Seen 08:45. RN charted new confusion at 05:30. Drowsy, rouses to voice, oriented to self only; oriented x3 on my exam yesterday.

Why: Today's findings, written fresh, with the comparison to yesterday that copy-forward erases.

Weak (signed 09:24): Plan: continue IV furosemide 40 mg BID.

Defensible: Correction, 10:15, to progress note signed 09:24 today. Subjective, exam, assessment and plan were carried forward in error from yesterday's note. IV furosemide held 08:55, not continued.

Why: The error stays readable, and a correction, never a silent edit, says who changed it, when and why.

Weak (written at 15:50): 11:30. Awake, oriented to person and place, not date. Plan unchanged.

Defensible: Late entry, written 15:50, for bedside reassessment at 11:30. Delayed by two admissions. At 11:30: awake, oriented to person and place, not date.

Why: Labeled, with both times and a reason, a late entry is ordinary; disguised as contemporaneous, it is not.

The Test You Order and the Test You Don't

Weak: No indication for CT head.

Defensible: Intracranial injury considered. Canadian CT Head Rule applies (minor head injury with witnessed LOC, age 16 or over, no anticoagulant); none of its 7 criteria present: GCS 15 at 10:20, 2.5 h after injury; no sign of open, depressed or basal skull fracture; no vomiting; age 38; no amnesia before the fall; mechanism a ground-level slip. CT not required by the rule.

Why: The test weighed is named, with the rule, why the patient fell inside it, each criterion with its value and time, and an output stated no larger than it is.

Closing the Loop on the Abnormal Result

Weak: Pt notified, advised ED.

Defensible: 17:20 called home, reached patient, identity confirmed with date of birth. Told him his potassium is high enough to disturb his heart rhythm and he needs the ED now.

Why: The note shows which call reached him, and “advised ED” becomes go now, with the reason he heard.

The Delayed Scan and the Bed That Never Opens

Weak: CT delayed, scanner down.

Defensible: 09:55. CT technologist reports scanner out of service, repair expected by 12:00. Nearest other CT 40 min by ambulance; transfer for imaging would not be faster than the repair estimate.

Why: The delay has a source, a time and an estimate, and the other route was weighed.

Weak: Pt resting comfortably, VSS. Continued to monitor.

Defensible: 17:15. HR 88, pain 2/10. Abdomen softer, less distended. NG 300 mL since placement. Plan unchanged.

Why: A time, the findings that bear on the danger, and a decision a reader can test against them.

The Consultant Who Never Comes

Weak: Vascular paged multiple times without response.

Defensible: 14:20. No callback. Operator confirmed Dr. Kirby on call, same pager; second page, same text.

Why: "No callback" with its time is a fact a reader can check against the log; "without response" is a complaint.

Why This Drug, This Dose, This Patient

Weak: Rx oxycodone 5 mg q6h prn pain #10.

Defensible: Oxycodone 5 mg, 1 tab up to every 6 h, only for severe pain the scheduled drugs do not relieve; #10, no refills, enough until ortho visit. Tramadol not used: serotonin syndrome and seizure risk with sertraline.

Why: The as-needed dose has a threshold, the quantity an end, and the drug set aside a reason.

Weak: PDMP reviewed. NKDA. Risks/benefits discussed.

Defensible: PDMP 17:05: no controlled-substance prescriptions filled in past 12 months. Allergies: penicillin (hives); none to analgesics. Discussed: oxycodone can cause drowsiness and slowed breathing, more so with sleep apnea or alcohol.

Why: A timed result, a reaction instead of a prefilled field, and this patient's risks.

When You Say No to an Antibiotic or a Scan

Weak: Viral URI, abx not indicated. Educated on antibiotic stewardship.

Defensible: Bacterial sinusitis considered, IDSA criteria absent: under 10 days, no worsening after initial improvement, no high fever with severe symptoms at onset. Explained: an antibiotic would not shorten it and can cause diarrhea or a rash.

Why: Each criterion has a finding, and the explanation concerns his illness, not resistance in the community.

Weak: Antibiotics deferred, or patient declined antibiotics.

Defensible: No antibiotic started today.

Why: "Deferred" hides the request and "declined" puts the choice in the wrong hands; "today" keeps it a judgment a later day may revise.

Hostile Encounters in Neutral Words

Weak: Son extremely hostile and aggressive, cursing at staff. Became threatening.

Defensible: Son at bedside, speaking loudly; swore several times. 18:50. Son struck overbed table with his palm and said to me: "If he dies tonight, I'll be waiting for you in the parking lot."

Why: Observed behavior, and the threat once in its exact words, let a reader judge the response.

The Capacity Question, Decision by Decision

Weak: Confused, A&O x1. Lacks capacity.

Defensible: Capacity assessment for one decision: EGD recommended by GI. Understanding: could not say why he is in the hospital; 2 min later asked, "What test?" Assessment: lacks capacity at this time for this decision, due to fluctuating delirium.

Why: Capacity belongs to one decision at one time, and orientation is not one of the four abilities.

Weak: Pt A&O x3 today, has capacity.

Defensible: 07:35. Understanding: "You put a camera down my throat to find the bleeding and maybe stop it." Reasoning: "My wife choked on one of those tubes and was never right after." Has capacity at this time to decline EGD; deficits found at 17:05 not present.

Why: His own answers, set against the evening's deficits, show a patient who changed, not a clinician who changed her mind.

Informed Refusal and the Partial Yes

Weak: LP deferred.

Defensible: Lumbar puncture declined; rest of plan accepted.

Why: "Deferred" makes a refusal look like a plan; naming what he accepted keeps the no from reading as a refusal of care.

Weak: Risks explained, pt verbalized understanding.

Defensible: Explained in plain words: without the LP we could miss an infection that needs a different drug. Asked what could happen without it: "You can't be sure what bug it is, so I'm on the strong stuff longer, and you might miss something else."

Why: The risk of refusing is in the words used, and his own account replaces a conclusion with nothing inside it.

The Patient Leaving Against Medical Advice

Weak: May return PRN.

Defensible: Told: "You can come back here any time, tonight included, and we'll pick up where we left off. Nobody will hold it against you."

Why: The invitation as it was said; "PRN" reads the same as a door politely closed.

Weak (patient found gone): Left AMA.

Defensible: Not in room at 02:40. At 01:40, alert and oriented, told the plan and why the second test is needed. No discussion of leaving took place.

Why: "AMA" implies that risks were explained and a choice made; the note never records a conversation that did not happen.

The Discharge and the Patient Who Comes Back

Weak: Return to ED for worsening pain, fever or other concerns.

Defensible: Told to go to the hospital ED, not this clinic, for pain that returns and lasts more than 3 h, temperature 100.4°F or higher or shaking chills, yellow eyes or skin or dark urine, or pain through to the back with vomiting.

Why: Each danger the assessment left open becomes a sign with a threshold, and the place named will be open.

Handoffs, Cross-Cover, and the Night Call

Weak: BP soft since admission, lisinopril held.

Defensible: Per 19:00 sign-out (D. Natarajan, NP): lisinopril to be held. Checked in chart: MAR shows lisinopril 20 mg given 21:00.

Why: What you were told keeps its source and time, apart from what you checked.

The Telehealth Visit and the Portal Message

Weak: Telehealth visit. Lungs clear to auscultation bilaterally.

Defensible: Video visit, audio and video throughout, 14:40–14:58. Measured on camera, her devices: peak flow 300, best of 3; pulse oximeter 95%. Not assessable by video: lung and heart sounds, temperature, oxygen on exertion.

Why: The exam is sorted by source, and the camera's limit replaces a finding nobody could have made.

Missed Visits and Referrals That Go Nowhere

Weak: Referral sent.

Defensible: Referral sent 12:45 to Dr. Samuel Tran, gastroenterology, marked positive FIT. Will check scheduling by 5/14.

Why: "Sent" records an order, not an appointment; a dated check keeps it from depending on memory.

The Adverse Event and the Records Request

Weak: Pt received wrong dose of glargine last night due to RN error.

Defensible: Per medication record, insulin glargine 40 units given 21:10 Tue; ordered dose 14 units.

Why: The doses ordered and given, each with its source, say what happened; how it happened belongs to the review.

Weak: Pt and daughter informed; daughter very upset.

Defensible: Dr. Sato explained: last night she received more of her long-acting insulin than was ordered, 40 units instead of 14. Patient asked: "Could it have hurt my brain?"

Why: The conversation is recorded, not just named, and a question in her words replaces a label.

CHAPTER 24

Scenario Finder and What Each Note Must Contain

What is happening now

- A dangerous diagnosis to name and set aside, or a score or decision rule you relied on: *Writing the Differential You Actually Ran*
- A working diagnosis you are not sure of, a choice the evidence leaves to the patient, a symptom with no cause yet: *The Working Diagnosis and What Would Change It*
- A behavior to describe, the patient's words to quote, a label or a complaint about staff you are tempted to chart: *Facts, Quotes, and What Never Goes in the Chart*
- A copied or templated note, a signed note to correct or add to, care written up hours later: *Templates, Copy-Forward, and the Late Addendum*
- A test to order, to skip or to leave for later: *The Test You Order and the Test You Don't*
- A critical value, an incidental finding, a result after the patient has left: *Closing the Loop on the Abnormal Result*
- A delayed scan, a bed or transfer that does not open: *The Delayed Scan and the Bed That Never Opens*
- A consultant who does not call back or come: *The Consultant Who Never Comes*

- A prescription, above all for a controlled substance: *Why This Drug, This Dose, This Patient*
- A request for an antibiotic, a scan or a referral you have decided against: *When You Say No to an Antibiotic or a Scan*
- Shouting, a threat, a request that keeps coming back, a disagreement with a colleague: *Hostile Encounters in Neutral Words*
- A patient who may lack capacity to decide, or a surrogate deciding instead: *The Capacity Question, Decision by Decision*
- A patient who declines part of the plan but stays in care: *Informed Refusal and the Partial Yes*
- A patient leaving against advice, or already gone: *The Patient Leaving Against Medical Advice*
- A discharge from emergency or urgent care; a patient who comes back: *The Discharge and the Patient Who Comes Back*
- A sign-out, a cross-cover page at night, the morning handback: *Handoffs, Cross-Cover, and the Night Call*
- A video visit, advice by phone or portal, an after-hours call: *The Telehealth Visit and the Portal Message*
- A missed visit, a patient lost to follow-up, a stalled referral: *Missed Visits and Referrals That Go Nowhere*
- An adverse event; a records request, board complaint or subpoena: *The Adverse Event and the Records Request*

By setting

No scenario belongs to one setting alone; these are the ones that recur in each.

Hospital medicine

The abnormal result, the bed that never opens, the consultant who never comes, hostile encounters, the capacity question, informed refusal, leaving against medical advice, handoffs and cross-cover, the adverse event.

Emergency and urgent care

The test you order and the test you don't, the abnormal result, the delayed scan, the consultant who never comes, the prescription, hostile encounters, informed refusal, leaving against medical advice, the discharge and the patient who comes back.

Primary care

The abnormal result, the prescription, the antibiotic or scan you decline, the telehealth visit and the portal message, missed visits and referrals that go nowhere.

What each note must contain

The Test You Order and the Test You Don't

- **The indication, or the reason not to test.** For a test ordered, the question it was meant to answer and the finding that raised it. For a test weighed and not ordered, the test by name and why it was not needed.
- **The decision rule and its inputs.** The rule by name, the findings that place the patient inside it, each criterion with its value, the times the rule depends on, and the output stated no larger than it is.
- **The patient discussion.** When the evidence left a real choice, the options, what each would cost, and the patient's preference in their own words.
- **The deferred plan.** For a test left for later, who orders it, when it happens, what the patient was told and what they will do if the date slips.

Closing the Loop on the Abnormal Result

- **The result and its reviewer.** The value, when it posted or was phoned in and to whom, and when you saw it.
- **The action.** What you decided about the result, including a decision to change nothing, with its reason.

- **Who was notified.** The person you reached, how you confirmed who they were, what they were told and what they said back; when another clinician takes over, that clinician by name and their reply.
- **The method and the time.** Phone, portal, bedside or letter, each attempt with its time, failed ones included.
- **The plan if the patient cannot be reached.** The next attempt, its method and its deadline, written before you know you will need it.

The Delayed Scan and the Bed That Never Opens

- **The delay as a fact.** What is delayed, who told you and when, the estimate and each change to it, with no comment on the department or facility.
- **The interim plan and its owner.** What you did for the condition you suspected, and who was responsible for the patient while they waited.
- **The limits that end the wait.** The findings that would make you stop waiting, who watches for them and what you will do.
- **Each reassessment.** Its time, the findings that bear on the danger and the decision, including the decision to keep waiting.
- **For a transfer.** The accepting facility and clinician, when they accepted and what it is pending, transport and its estimate, the alternatives considered, what the patient and family were told, and when you will act if the bed does not open; and, if your shift ends first, the handoff: time, the clinician by name, what you told them.

The Consultant Who Never Comes

- **The trigger.** The finding that made the consult urgent, with its time and onset.
- **Each call.** The time, the person or role, the route, and the message or question as sent.
- **Each answer.** The reply or its absence, with the time and with no adjective or guessed reason; a phone recommendation as the consultant's, read back, with whether they had seen the patient.

- **The interim care.** What you did while waiting, and each reassessment with its findings and its decision, including the decision to escalate.
- **Each escalation step.** The physician you work with and each chain-of-command contact: name, role, time, what you told or asked them, and what they did.
- **The handover.** Who took over, when, and with what plan.

Why This Drug, This Dose, This Patient

- **The indication.** The diagnosis and expected course that call for the drug, why this drug over the alternatives, and the dose, frequency and threshold for any as-needed dose.
- **The risks discussed.** The risks for this patient in the words used, the patient's reply, and instructions on alcohol, driving, storage and disposal where they apply.
- **The checks performed.** Allergies with their reactions, interactions with what they changed, the risk factors behind the plan, and for a controlled substance the PDMP result with its time, the relevant history and any agreement your state or policy requires.
- **The duration.** The quantity, the refills and the days they cover.
- **Who reassesses and when.** The clinician and visit that decide what comes next, and the return instructions for what the drug could hide.

When You Say No to an Antibiotic or a Scan

- **The request.** The patient's words and the concern behind them.
- **The decision.** What you did not prescribe or order, written as your clinical choice, with its date.
- **The clinical reason.** The diagnosis the request aimed at, with the findings that set it aside, and the dangerous alternative with its own.
- **The explanation and the reply.** What the patient heard about their own illness, and their answer, disagreement included.
- **What was offered instead.** Symptom relief, a delayed prescription, a referral or recheck, and what was accepted.

- **The signs that change the answer.** Those that mean start the delayed plan, and those that mean be seen today.
- **The way back.** Who reassesses and when, and how the record will learn the delayed plan was used.

Hostile Encounters in Neutral Words

- **The report and your look.** What was raised, by whom, and your exam or review in response, with times.
- **Each request and your answer.** The request in the person's words, and the clinical reason, given once.
- **The behavior, observed.** What was said and done, without adjectives, and a threat in its exact words; the rest of any incident goes in your organization's reports.
- **What changed for the patient.** Security, visiting and updates, and the patient's own wishes.
- **The physician asked for.** Who was called, when, and what they did.
- **For a disagreement.** The option considered, the findings behind it, the decision and its owner, and any escalation; never the conflict.

The Capacity Question, Decision by Decision

- **The question.** The decision assessed, named specifically, and the time of the assessment.
- **The information.** What the patient was told, and any interpreter or sensory aid used.
- **The assessment.** Each of the four abilities, with the patient's own answers, right or wrong.
- **The conclusion.** A finding limited to this decision at this time, with its likely cause.
- **The timing.** Whether the decision could wait; each reassessment, timed, with what changed.
- **The clinicians.** Who assessed, and any psychiatry or ethics consultation: question, answer, action.

- **The surrogate.** For a surrogate: name, relationship, source of authority, what they were told, what they chose and why.

Informed Refusal and the Partial Yes

- **The recommendation.** What was proposed, why, who would do it and who was present.
- **The risks in plain words.** The risks of refusing for this patient, the most serious first, and those of the test or treatment itself.
- **The alternatives.** Each option offered, including going without and what would replace it.
- **The patient's own words.** The reason for the refusal and the patient's account of what could happen.
- **The capacity finding.** For this decision at this time, with the answers behind it.
- **The partial yes.** What was accepted, what was refused and what the refusal changed in the plan.
- **The next offer.** That the patient may change their mind, who will ask again and when, and orders updated to match.
- **The form.** Signed or declined, never in place of the conversation.

The Patient Leaving Against Medical Advice

- **The departure.** The time, the patient's reason in their own words, and the care still recommended.
- **The informed refusal.** The risks of leaving now, the alternatives offered (including help with the reason for leaving), and the capacity finding, written as the chapters on informed refusal and the capacity question describe.
- **The door left open.** The invitation to return as it was said, and the patient's reply.
- **What the patient left with.** Prescriptions accepted or declined, lines removed, instructions for this illness, pending results and their owner.
- **The follow-up.** Who sees the patient next, when, and who arranged it.

- **Who was told.** The attending and anyone else policy names, with times.
- **The form.** Signed or declined, filed beside the note and never in place of it.
- **The unwitnessed departure.** When the patient was last seen and found gone, the search, what they had been told, each attempt to reach them, and who was notified. Never a conversation that did not happen.

The Discharge and the Patient Who Comes Back

- **The disposition and why.** Where the patient went, what made it safe enough, the options offered and the patient's choice.
- **The uncertainty carried out the door.** The working diagnosis labeled as working, and the dangers not yet excluded.
- **Precautions for this diagnosis.** Each open danger as a sign the patient can recognize, with a threshold and a place that will be open.
- **What the patient understood.** Their account in their own words, and the written copy.
- **The follow-up.** Who by name, when, how to reach them, who was told, and what happens if the appointment cannot be made.
- **On a return visit.** The earlier note reviewed, a fresh history and exam, each change with values from both visits, and the decision with its reason.

Handoffs, Cross-Cover, and the Night Call

- **What you were told.** The source and time of the sign-out, and the items your decisions rest on.
- **The pending items.** Each item accepted, its result and what you did, or the clinician you handed it back to.
- **The call.** Who called, when, and what they reported.
- **Your own look.** When you saw the patient, the findings that bear on the problem, what you checked in the chart, and any disagreement with the sign-out, as facts.
- **Your assessment.** The working diagnosis and alternatives in your words, and whether the inherited plan still fit.

- **What you did and whom you told.** Orders with times; the physician you called, when, whether they saw the patient and what they advised.
- **The limits and the next look.** What the nurse will call you for, and when you will see the patient again.
- **The handback.** What changed overnight, what is pending, which decisions now belong to the day team, the limits still in force, and who took over and when.

The Telehealth Visit and the Portal Message

- **The modality.** Video, phone or portal; start and end times; any switch partway, and when.
- **Who, where and consent.** How identity was confirmed, where the patient was, who else was present, and consent as your organization requires.
- **The remote exam, by source.** What you observed, what the patient's devices measured on camera, what was only reported, and what could not be assessed.
- **Why remote was enough.** The findings that made remote care reasonable, and the in-person option offered.
- **The threshold for in-person care.** The signs or readings that bring the patient in, and those that send them to the emergency department.
- **Phone and portal advice.** Who asked, in their words; what was advised and by whom; how the answer was delivered and whether it was received.
- **The after-hours call.** The time, your chart access, the advice, the escalation threshold with the patient's read-back, and who owns the next step by when.

Missed Visits and Referrals That Go Nowhere

- **Why the follow-up matters.** At the call or visit that sets it up, the reason in plain words and what the patient said back.
- **The open item.** Where the chart will show it until it closes, such as the problem list or an alert.

- **Each missed visit, noticed.** Who reviewed it, when, and the decision.
- **Each attempt.** Time, method, what was said or left, and the reply or its absence; a copy of any letter.
- **The next step and its date.** At the end of every entry, within the steps your policy sets.
- **The referral, tracked.** Whether it was received and scheduled, whether the report came back, who checked and when, and who owns the next step until the report has been read.
- **The patient's answer.** Their reason in their own words; a refusal written as an informed refusal, and silence never written as one.
- **Where the effort ended.** What was done, that the item is still open, and how the chart will catch the next contact.

The Adverse Event and the Records Request

- **The event as facts.** What happened and when, with the source of anything you did not witness; no guess at how it came about, no blame.
- **The patient's condition.** Findings with times and values, not "stable."
- **What was done.** Treatment and orders, with times.
- **How long the danger lasts.** Monitoring, call limits and an end point.
- **Who was told.** Clinicians involved in the care, with times; never the report or the call to risk management.
- **The disclosure.** When, who was present and who spoke, what was said, the patient's questions in their words with the answers, and who speaks to them next.
- **After an attorney's request, a complaint, a claim or a subpoena.** No new entries about the past care and no changes; today's care charted as usual.

Before you sign

Read the list for your scenario once the note is drafted, and look for each element in what you wrote; one you cannot find is a sentence to add before you

sign. The lists are not a form to fill in: an element is there when your own sentences show it, not when a heading names it.

CHAPTER 25

Glossary of Legal Terms in Plain English

THESE DEFINITIONS ARE general information, not legal advice, and the precise meaning of a term can change from state to state; for your own state, ask your risk manager, your insurer or your state board. The italics at the end of each entry name the chapter to turn to for that term.

Addendum. Information added to a note that is already signed, such as a result that came back, a call returned or a detail left out. Organizations define addenda and late entries differently, so follow your policy on the label. *Templates, Copy-Forward, and the Late Addendum*.

Adverse event. Unintended harm to a patient that results from care rather than from the underlying illness. Whether a given event must be reported, and to whom, depends on state law and your organization's policy. *What a Chart Becomes After a Bad Outcome*.

Against medical advice (AMA). A patient leaves against medical advice when they end care before the treating clinicians judge it safe, after being told so. A patient who leaves before anyone has discussed leaving with them has not left AMA. *The Patient Leaving Against Medical Advice*.

Attestation. A statement a second clinician adds to someone else's note about their own role: that they saw the patient, reviewed the note or agreed with the plan. *The Provider Note Records a Decision*.

Audit trail. The system's own record of activity in the chart: which account opened which record and when, and what was viewed, created, edited, signed or deleted, often with each earlier version of a changed note. *Templates, Copy-Forward, and the Late Addendum*.

Behavior agreement. A written statement, made with a patient or visitor whose conduct has disrupted care, of what is expected from now on and what follows if it is not met. *Hostile Encounters in Neutral Words*.

Best interests. The standard a surrogate decides by when the patient's own wishes cannot be known: the choice that best serves the patient. Compare substituted judgment. *The Capacity Question, Decision by Decision*.

Board investigator. The person who examines a licensing board complaint, asking whether your practice met the standards your license requires and stayed within your scope of practice. *What a Chart Becomes After a Bad Outcome*.

Capacity. See decision-making capacity.

Causation. The link between a failure to meet the standard of care and the patient's harm. In most states, a claim has to show that doing what should have been done would, more likely than not, have changed what happened. Some states also allow a claim for a lost chance of a better outcome. *What a Chart Becomes After a Bad Outcome*.

Chain of command. The route your organization sets for taking a clinical concern past the people who have not resolved it. *The Consultant Who Never Comes*.

Closed claim. A malpractice claim that has reached its end: withdrawn, dismissed, settled or decided at trial. *Where a Claim, a Complaint, and a Review Begin*.

Collaborative agreement. An arrangement, usually in writing, that sets out how an NP and a physician work together. Whether one applies to you depends on your state and setting. *The Provider Note Records a Decision*.

Competence. A legal status decided by a court, unlike decision-making capacity. *The Capacity Question, Decision by Decision*.

Contemporaneous. Written at the time of the care or soon after, before memory starts filling gaps. *Where a Claim, a Complaint, and a Review Begin*.

Controlled substance. A drug that federal and state law regulate more tightly because it can lead to dependence or misuse. *Why This Drug, This Dose, This Patient*.

Controlled-substance agreement. A written statement of the terms of ongoing prescribing. Whether one is required depends on your state and your organization. *Why This Drug, This Dose, This Patient*.

Cosignature. A second clinician's signature on a note someone else wrote. By itself it does not say whether that clinician examined the patient, read the note line by line or only signed it. *The Provider Note Records a Decision*.

Damages. The money a malpractice claim seeks for the harm it alleges. *Where a Claim, a Complaint, and a Review Begin*.

Decision-making capacity. A clinical judgment, usually made by the treating clinician, that a patient can make one particular decision at one particular time. Clinically it rests on four abilities: understanding, appreciating that the situation applies to oneself, reasoning and expressing a choice. State law may use its own wording, and when the finding activates a health care agent or a surrogate it may say who must make it, so ask whether an NP's finding is enough where you work. *The Capacity Question, Decision by Decision*.

Defense attorney. The lawyer who defends against a claim. The defense attorney may represent you, your employer or both, so ask first whom they represent. *What a Chart Becomes After a Bad Outcome*.

Disclosure of an adverse event. Telling the patient, or the person who decides for them, that something unexpected happened in their care, what it means for them and what is being done. Not the same as releasing health information under privacy law. *The Adverse Event and the Records Request*.

Employer review. The internal review your organization starts when it learns of a bad outcome, often before any claim or complaint. It can end in a conversation, limits on practice or the loss of the job. *Where a Claim, a Complaint, and a Review Begin*.

Expert witness. A clinician hired by one side to explain what the standard of care required and whether your care met it. State rules decide who may serve as an expert in a case against an NP, and it is not always another NP. *What a Chart Becomes After a Bad Outcome*.

Guardian. A person a court appoints, called a conservator in some states, to make some or all decisions for someone it has found incompetent. *The Capacity Question, Decision by Decision*.

Health care agent. The person a patient names in a health care power of attorney to decide for them if they cannot. *The Capacity Question, Decision by Decision*.

Health care power of attorney. A document in which a patient names a health care agent to make decisions if the patient cannot. *The Capacity Question, Decision by Decision*.

Health care proxy. In some states, the name of the health care power of attorney document; in everyday speech, also the person it names. See health care power of attorney and health care agent. *The Capacity Question, Decision by Decision*.

Hindsight bias. The tendency, once you know how something ended, to see the ending as more predictable than it was and the early signs as louder than they sounded at the time. *What a Chart Becomes After a Bad Outcome*.

Incident report. An internal document written for the organization's safety and quality review and kept in its own channel. How far the law protects it varies, and the chart never mentions it. *Facts, Quotes, and What Never Goes in the Chart*.

Informed consent. The process in which a clinician explains a proposed test or treatment, its purpose, its main risks and benefits and the reasonable alternatives, and a patient with capacity agrees. *Informed Refusal and the Partial Yes*.

Informed refusal. The informed-consent process ending in no, with the risks of refusing at its center. A signed refusal form records a signature; the note records the conversation. *Informed Refusal and the Partial Yes*.

Judgment. A court's decision that ends a case, including any money one side must pay. *Where a Claim, a Complaint, and a Review Begin*.

Late entry. An entry documenting care given earlier but not written at the time. It is labeled as late and gives both the time of the care and the reason for the delay. *Templates, Copy-Forward, and the Late Addendum*.

Licensing board complaint. A complaint to the state board that licenses you. Anyone can file one, it needs no injury, and what is at stake is chiefly the license, though a board can also fine you and its actions can be reported to the National Practitioner Data Bank. *Where a Claim, a Complaint, and a Review Begin*.

Malpractice claim. A claim of negligence against a clinician, seeking money damages for harm to a patient. *Where a Claim, a Complaint, and a Review Begin*.

National Practitioner Data Bank. A federal repository that hospitals, licensing boards and some other health care organizations can query. Malpractice payments made on a clinician's behalf, by settlement or judgment and whatever the amount, are reported to it, and so are licensing board actions and some actions on hospital privileges. *Where a Claim, a Complaint, and a Review Begin*.

Negligence. In a malpractice claim, the allegation that a clinician who owed the patient care failed to meet the standard of care, and that this failure harmed the patient. *What a Chart Becomes After a Bad Outcome*.

Peer review. An internal examination of a clinician's care by colleagues. Whether it can later be used outside the organization depends on state law. *Where a Claim, a Complaint, and a Review Begin*.

Plaintiff. The patient or family bringing a claim. *What a Chart Becomes After a Bad Outcome*.

Prescription drug monitoring program (PDMP). A state database of dispensed prescriptions for controlled drugs. When a search is required depends on your state, and the note records the time and result of each search, even an empty one. *Why This Drug, This Dose, This Patient*.

Records request. A request for copies of the chart, from the patient, from someone acting for them such as an attorney, or from another party with a right to it. Most are routine. One that comes from an attorney, or with a complaint or a claim, means: add nothing to the chart about the care in question and change nothing in it. *The Adverse Event and the Records Request*.

Risk manager. The person in your organization whose job includes finding out what happened after an adverse event and how exposed the organization is. *What a Chart Becomes After a Bad Outcome*.

Scope of practice. The care your license allows you to provide, as set by state law and your state board. *Introduction*.

Settlement. An agreement that ends a claim without a court's decision, usually with a payment. *Where a Claim, a Complaint, and a Review Begin*.

Spoliation. The destruction, alteration or failure to preserve evidence. Altering a record once a claim is pending or reasonably expected can be treated as spoliation. *Templates, Copy-Forward, and the Late Addendum*.

Standard of care. In most states, what a reasonably prudent NP, with similar training and in similar circumstances, would have done with the same information at that moment. It is not perfection, and often several choices would have met it. Some courts hold an NP who does what a physician would do to the physician's standard; ask your risk manager which applies where you work. *What a Chart Becomes After a Bad Outcome*.

Subpoena. A legal demand, with a deadline, to produce records or to appear and testify. Call your risk manager the same day, and your own professional liability insurer if you carry a policy. *The Adverse Event and the Records Request*.

Substituted judgment. The standard a surrogate is generally expected to use: deciding as the patient would have, from the patient's known wishes and values. Compare best interests. *The Capacity Question, Decision by Decision*.

Supervision. An arrangement, depending on your state and setting, in which a physician holds a defined oversight role in an NP's practice. *The Provider Note Records a Decision*.

Surrogate decision-maker. The person who decides for a patient who lacks capacity. Their authority may come from a health care power of attorney or a court-appointed guardian or conservator; if no one was named, state law and your organization's policy set who may decide. *The Capacity Question, Decision by Decision*.

Sources for the tools in this book

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About the Author

Dana Ellery is the author of this plain-English casebook on clinical documentation for nurse practitioners. The author is not a clinician, and this book is not legal or clinical advice. It builds on published decision rules and guidelines, listed at the end of the book, and sets every principle on a concrete note; its scenarios are constructed for teaching. For the rules where you practice, ask your risk manager, your professional liability insurer and your state board of nursing.

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